

PERFECT TIMING - RECOLLECTIONS OF COPING WITH CANCER DURING A PANDEMIC



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Perfect Timing - Recollections of coping with cancer during a pandemic

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Dedication

This book is dedicated to **my twin sister Susanne**, with all my love and gratitude.



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Preface

Dear reader,

This cheeky limerick captures, in a nutshell, my rationale for writing this book:

There once was a university prof in Saskatchewan

Who needed to learn that — all along —

It was important to share her own journey

With others (without sounding corny)

And do it before she herself was gone.

Alternatively, you could describe it as an educational, entertaining, and highly personal memoir that provides an insightful snapshot of the occasionally bumpy yet spiritually transformative cancer journey which a middle-aged, immigrant, and non-partnered academic living in a sunny Canadian prairie province undertook during a global pandemic.

If you were able to understand this admittedly complex sentence the first time, “Dr. Barb” (which is what I call my inner scholar) will be impressed.

If you did not, don’t worry; this book is not meant to turn your brain into a pretzel. On the contrary, I tried my best to keep my target audiences in mind throughout the writing process.

If you are currently (that is, during or after a pandemic) or have ever been on the cancer continuum as a patient, caregiver, family member, or friend, this story is for you.

If you are or strive to be a health professional (oncologist, GP, nurse, social worker, pharmacist, physio- or exercise therapist, etc.), this story is for you, too.

If you are an administrator, instructor, teaching assistant, or student at a post-secondary institution, then this story should resonate with you on a variety of levels as well. Why?

Because it not only touches upon topics of interest in health science classes, but also qualifies as a piece of literature that could be examined in English courses focusing on, for instance, memoir writing, creative non-fiction, and narratives of illness. Certain parts of my story are meant to draw in readers from other disciplines, including Women’s and Gender Studies, Spirituality Studies, Religious Studies, and the Fine Arts. Heads up: “Dr. Barb” has included a list of “Leading Reading Questions” in an Appendix to help you get the most out of your reading experience (and lighten the load of fellow university professors). Enjoy!

If none of these categories are a good fit for you — perhaps, you are an author yourself or a reader who likes to give writers from the Canadian prairies a chance — I hope that my recollections and reflections will manage to touch you, nonetheless.

If nothing else, thanks to the [Open Educational Resources](#) program of the University of Regina, you did not have to spend any money on purchasing this book.

I would, therefore, be grateful if you helped me **raise funds for cancer research**. It will help ensure that more patients, including women, can receive the care they need before, during, and after a cancer diagnosis changes their lives forever.

To find out more about how to give to the **Cancer Foundation of Saskatchewan**, visit their [website](#).

Alternatively, you could support a charity of your own choice. Thank you!

Barbara Reul (she/her)

PS: This book was originally published on the University of Regina, Canada, Open Education Resource Platform in December 2021 (see the cover below). Its contents have been updated at regular intervals, including the section “Back Matter”, specifically “Publications about and by this author”.



PERFECT TIMING:
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cancer during a pandemic



Authored by Barbara Reed

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Acknowledgments

I thank everyone who supported me throughout this journey back to health from the bottom of my heart. Even though you shall remain nameless to protect your privacy and mine, you know who you are and what you did (or did not do). I will forever be filled with love and gratitude for **your acts of kindness**, and I will do my very best to **pay them forward to honour you**.

My “Canadian parents,” **Leonore and Jim**, and my “Canadian sister,” **Pauline**, left us in 2019 and 2020, respectively, to watch this part of my life story unfold from the other side. I owe them a lot, and I miss them even more.

I am grateful to the **Saskatchewan Cancer Agency** to whom I have presented my patient story twice to date (spring 2024): to its Board members on October 7, 2021, and to its Senior Leadership Team on July 11, 2023. On November 27, 2021, I was also the featured speaker of the [Ovarian Cancer Canada](#) Saskatchewan support group’s Christmas event in Regina, Saskatchewan.

I am a **proud supporter** of the **Saskatchewan Cancer Foundation**.

Proud Supporter of



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1: “This is who I am” — Introduction

What do you need to know about me before you begin reading this story of personal rediscovery, dear reader?

Everything you have heard about me is true

This has been my favourite way of introducing myself to unsuspecting students at our local university who have signed up for (or are required to take) a class with me. In other words, I am a somewhat accidental academic blessed with a sunny, cheerful personality, and a wicked sense of humour. I hope it will shine through frequently as you join me on this journey of transformation.

Otherwise, I strive for excellence in most everything I do — only to be reminded on a regular basis that I am a human being who struggles with her weight often and her age occasionally.

I was born and grew up in a small town in southern Germany in a family of four girls; our parents both passed away before my younger twin sister and I finished high school. In the mid-1980s, the two of us immigrated to Canada to join our oldest sister, who was raising a family in Victoria, B.C. We began attending university immediately and over the course of a decade I finished — very much to everyone’s satisfaction, including my own — three degrees, including a doctorate in music history.

I returned to Germany for two and a half years in the late 1990s to work for a not-for-profit musical organization that promotes the life and works of a German contemporary of [J. S. Bach](#) and [G. F. Handel](#), [J. F. Fasch](#) (1688–1758), on whom I had chosen to write my doctoral thesis. Upon my return to Canada, I applied for jobs in higher academia while supporting myself with teaching continuing education courses at my alma mater and local senior centres, playing the organ for worship services at various churches, and cleaning houses.



Prairie winter scene

In the early 2000s, I moved from the [capital of British Columbia](#) on Vancouver Island to the [capital of Saskatchewan](#), one of the coldest parts of Canada, to pursue a career as a university professor.

“I am going to like it here,” I decided after only a few short months, much to my (and my family’s) surprise. Granted, I struggled greatly with the long winter season which typically runs from October to April as well as the dry air which made me spend a fortune on hand creams, body lotions, and eye drops.

But my new colleagues and a local church community looking for an organist had welcomed me with open arms. Who knew that prairie folks had such warm hearts and always looked out for one another? Their repeated attempts to find me a “nice man” to settle down with amused me to no end. After all, I had been (and continue to be) in a committed relationship with a male since the early 1990s — the fact that he has been dead for two and a half centuries has always been an asset, not a liability, as far as I am concerned.

Why? Because [Fasch’s music](#) is as cheerful as it is profound and has been a source of excitement and comfort to me ever since we met on a blind date in graduate school. Spending quality time in German archives (specifically in the state of [Saxony-Anhalt](#) where Fasch was based for most of his career) and examining countless old manuscripts to get to the bottom of what eighteenth-century life viewed through a musical lens was really like, gives me great joy. In fact, it makes me feel like a cross between [Indiana Jones](#), [Sherlock Holmes](#), and Dr. Temperance Brennan, the forensic anthropologist from the hit TV series [Bones](#). Engaging passionately with the past has also turned out to be a perfect antidote to teaching sleepy-eyed students day in, day out in window-less university lecture halls.

Not surprisingly, the arrival of [Covid-19](#) in the early spring 2020 turned my highly scheduled (but never boring) life upside down. Besides facing unexpected changes at work (“You have four days to move all your courses online!”) and at my church (“We’ll be

doing services in the ‘Fireside Room’ next to the sanctuary via [Zoom](#) from now on”), I was stunned to hear about the imminent closure of our local downtown [YMCA](#)’s fitness facilities. It meant that I could no longer hop in the pool to de-stress or volunteer to teach classes as a certified group aquatic instructor. Fortunately, my twin sister had just decided to switch careers and become a certified fitness professional. We spontaneously resolved that we were going to get into the best shape of our lives together, starting immediately.

Working (it) out

After two months of exercising six times a week, I felt and looked better than ever, confident that I would not gain back (m)any of the 75 pounds I had intentionally lost in 2018/19 to combat [perimenopausal symptoms](#). At the same time, I was missing visits with my family out west and my annual research trip to archives in Germany that had to be cancelled because of Covid-19 restrictions. To compensate, I focused on my work, specifically a long overdue scholarly book manuscript on my favourite composer (“Mr. Fasch”). I also spent a lot of time in nature; I live near a beautiful park with a big lake. And I was on the phone/social media a lot, staying in close touch with family and friends all over the world. On [July 1, Canada Day](#), I remember toasting to the second half of 2020, determined that it was going to be better than the first half.

Little did I know at the time that my life would get a whole lot worse first.

And before you scroll down to the next chapter, I want to you to keep in mind that even though my story contains some tough parts (spoiler alert), it ends well. My experiences as a cancer patient are also unique to me. My hope is that you will count your blessings as you join me on this trip down memory lane.

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2: “I look pregnant” — Noticing Symptoms

Have you ever looked in a mirror and noticed that part(s) of your body have changed, usually to the worse and seemingly overnight?

Something is happening

That happened to me on a sunny morning in mid-June 2020. I remember looking at my tummy sticking out and thinking “Great ... now I am finally turning into my mom!” That afternoon I complained bitterly to my twin during our exercise session. Granted, as a decidedly single, post-menopausal woman nearing her 53rd birthday, being pregnant was out of the question. Instead, my abdominal muscles seemed to have a mind of their own, and old age was evidently getting the better of me.



Selfie from June 2020

My trusted scale also revealed that I had gained ten pounds, a fact I quickly attributed to “relaxing” my eating routine in response to coping with Covid-19 stressors. What about the occasional abdominal pain and having to run to the toilet more often than usual? In hindsight, I wish I had admitted to myself that my body was sending me important messages which I chose to ignore.

Instead, I added more rest to my daily routines and watched my diet more carefully for two weeks. That strategy paid off: I was not only full of energy all day long, but also down eight pounds! As a committed “breakfast person,” I did notice, however, that I was suddenly no longer hungry in the morning (“Great ... now I am turning into my twin sister!”) and felt very full after eating very small meals. But reporting any or all of these symptoms to my doctor seemed like a waste of time to me, as he had other, more important things to do during a pandemic.

It is probably nothing

During a yoga session with my twin sister on July 18, 2020, I noticed a small, hard mass in my lower left abdomen. “We probably did too many abdominal crunches yesterday,” I remember thinking. It was gone when I checked for it again the next day.

Nevertheless, I brought it to the attention of my trusted massage therapist a few days later. She could feel it as well and advised me to call my doctor to have it checked out.

“I will,” I said, quick to offer a self-diagnosis that would haunt me for months to come — “because, after all, we both know I don’t have cancer!”

Due to [Covid-19 restrictions](#), my GP (general practitioner) had to telephone me first.

I promptly apologized for bothering him. “I feel perfectly fine — it’s probably nothing,” I said.

“I will decide whether you are fine or not! I’ll see you tomorrow afternoon,” he replied.

A risk worth taking

Before I continue with my story, I have a favour to ask. If you or someone close to you experiences symptoms that cause feelings of unease (or even dis-ease), consult a physician sooner rather than later. Do not let arguments such as “I know my body best,” “I just had a check-up,” or “Dr. Google says not to worry” deter you.

If it turns out you were wrong, you will likely be forgiven.

If it turns out you were right, you could potentially be held responsible for changing that person's and/or your own life trajectory forever.

If that scares you, do it anyway. In the end, it is a risk worth taking.

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3: “A sinister mass” — Being Diagnosed

One more thing you need to know about me is that prior to this health journey, I always preferred folks with PhDs to medical doctors. They do not tend to ask annoying questions (“Could you be pregnant?”) or conclude in record time that I — despite being in excellent health — would die young like my parents.

I knew I had found the right physician in spring 2017 when in response to my admittedly cheeky statement he asked, grinning from ear to ear, “What happens when health professionals have dual degrees...?” I had limped into my university’s walk-in clinic to have someone assess an old injury that refused to heal.

“It clearly states in the [radiologist](#)’s report from fall 2016 that you probably broke a small bone in your right foot,” he noted. “You had fallen down some steep church stairs?”

“Yes, but your colleague just told me to go home to rest and ice it,” I replied, stunned.

This physician did not waste any time to get to the bottom of my limp. The broken bone had healed (“good”), but I had developed a heel spur in the other foot (“not good”). To get me back on my feet again, I was to see a physiotherapist immediately. “Wow! This doctor really knows his stuff,” I concluded at the end of our visit.

When I came to see my GP on Thursday, July 23, 2020, he was impressed that I had successfully dropped so much weight. I was impressed that my blood pressure was perfect that day, as I have long suffered from “[white coat syndrome](#).” My blood pressure tends to shoot up when a nurse or a medical doctor check it.

“What are you thinking?” I asked my GP had after he had examined my belly.

“Large [fibroids](#); you will definitely need surgery, and soon,” he replied. His answer was unexpected, but not anything I could not handle; a dear friend with the same diagnosis had recovered quickly and was back at work in no time. A [transvaginal ultrasound](#) and a [pelvic exam](#) were scheduled for the following Tuesday.

“Call me if there’s any pain,” my GP said. Since there had been none for weeks, I assumed that everything was going to be fine on the weekend.

I was wrong.

Drop everything and go now!

The very next day, a Friday, I suddenly felt sharp pains around my belly button and called my GP immediately.

“I had been expecting that,” he said. “I have already made an emergency ultrasound appointment at the hospital for you, so drop everything and go now! I’ll call you tomorrow with the results.”

I was stunned — and then promptly panicked, but not because of his tone: I had just washed my one and only cotton mask, and I needed it to enter the hospital — darn pandemic! So, I spent 10 minutes blow-drying it, donned a pretty summer dress and a hat, and in 30-degree heat walked 12 minutes to the hospital.

An hour later, when the [ultrasound](#) technician had outlined whatever was growing in my tummy seemingly a hundred times, I did not know what to think anymore. I also dreaded having to tell my relatives and trusted friends about this sudden turn of events, because I knew they would be worried.

As a level-headed individual who has never been prone to emotional outbursts, I had expected everyone to be shocked to the core (I was!) and then offer their full support (because I would have!). I was not prepared for the deafening silence my difficult news elicited at times, however. Thankfully, nobody ever hung up on me.

When I woke up the next morning — pain-free and full of energy — I was confident that it was all going to work out.

A call from my GP signalled the opposite, however. The ultrasound had shown a large “sinister mass” in my belly, and a [CT](#) (computer topography) scan appointment on Monday morning (“that fast — during Covid?”) would tell us more.

“But I feel better than ever! Can I at least play for the church service tomorrow?” I said.

“Yes, but if the pain comes back and increases, please go to the hospital right away,” he replied to my amazement and delight.

“It won’t,” I thought.

Wrong again.

When getting ready the next morning, I noticed a spot of blood in my panty liner which made me sigh out loud. Being [post-menopausal](#), I knew what it meant — an impending cancer diagnosis. Should I call my pastors and arrange for a substitute? I decided against it, given that praying and singing with others always helps me feel grounded and, most importantly, useful.

By noon, the pain returned and increased to the point that I asked a trusted neighbour to drive me to the hospital (no walking this time!). I waited for two hours at emergency, trying to distract myself by reading a historical romance novel with a predictable plot and outcome (“Marriage-resistant duke falls in love with beautiful and incredibly smart commoner”). Finally, I was told by the attending physician that because of my upcoming CT scan, he was not going to do anything but send me home to take painkillers.

“So, I am not going to die tonight?” I asked him, trying to put on a humorous front.

“No, you won’t, and now get out of here before I change my mind,” he replied with a smile on his face. My heart, however, was heavy.

Later that evening, I talked at length to my sisters, several trusted friends, and my pastors. I asked them to keep me in their prayers (“You got it, Barb”); I had a feeling I would need them.

The CT scan itself was a quick affair. Feeling pain-free and full of energy again on that sunny, warm Monday morning, I not only walked home from the hospital, but also sat down to put the final touches on a research project that was due soon: an informative entry on the life and musical works of my favourite composer (“Mr. Fasch”) to be included in the prestigious [Grove Music Online Dictionary](#).

Then the phone rang. It was my GP’s administrative assistant, who asked me to come in to see him in the early afternoon. When she said, “You may bring a support person with you,” I braced myself for the worst. My best friend, a long-time high school teacher and fellow musician, immediately offered to accompany me.

The news was grim. My ovaries had grown all the way up to my belly button, but there was also a lot of free fluid that needed to be drained — immediately. My physical reaction to that bit of news was immediate as well: I felt completely numb.

“I need you to have blood drawn first before you go to the hospital; certain [tumour markers](#) need to be checked right away as well,” my GP continued.

“This is serious, Barb,” my best friend, who had miraculously survived a heart attack and a stroke in 2011, commented on our way to the lab. “I have never seen a doctor speak and act with such a sense of urgency.” I agreed.

“Did you also see the look on his face when *you* started coughing?” I asked her.



Fluids drained after CT scan

I then pleaded with her to have it checked out, as it had been going on for months, and she had often complained about fatigue as well. Neither of us knew at that time that this conversation would be the beginning of her very own cancer journey.

My best friend dropped me off at the hospital, where I spent the next three hours watching, in both horror and relief, as a total of 4.5 litres of [fluid](#) were drained. Suddenly, my belly was flat again — and I felt about ten pounds lighter overall, having emptied my bladder completely at the hospital for the first time in months.

A visit from the other side

At the same time, I was still rattled from what had transpired earlier on when I was waiting to have the procedure, and the nurse had stepped away for a few moments.

An ethereal being had appeared at the end of my hospital bed.

“Are you ready to come with me?” it asked.

In hindsight, he looked remarkably like [Andrew](#), the Angel of Death in the TV series [Touched by an Angel](#), on whom I had had a secret crush as a university student.

I was stunned, to say the least.

The closest I had ever come to pondering my own demise had been during a beach holiday in Mexico many years ago. I am an excellent swimmer, yet a sudden cramp in my leg prompted the headline “Canadian graduate student tragically killed while belly-boarding near [Cabo San Lucas](#)” to flash through my head before making it to shore safely.

I answered the question posed by “Andrew” with a resounding “No!” I also remember feeling both angry and scared that the universe would propose such a ridiculous notion.

I was not ready to die.

Given the fact that my health had been excellent over the years (save for [appendicitis](#) in Grade 10 and a couple of broken wrists), there was no reason for me to follow in my parents’ footsteps. Both had died before their respective 60th birthday: my dad of [blood cancer](#) at age 52 after having been ill for 13 years, and my mom very suddenly at age 58, after suffering three [strokes](#) on three consecutive days despite taking high blood pressure medication for years.

I immediately turned to my intuition or inner guidance system for advice. Ever since I had become interested in [energy healing](#) in the mid-2000s, it has contacted me in the form of a distinguished-looking male guide wearing a top hat. He told me not to worry, and a wonderful feeling of a host of angels spreading their wings around me came over me. I knew then that I had nothing to fear.

When I told my twin sister and a long-time clergy friend, a fellow cancer survivor based in Alberta, about this unexpected and initially frightening incident, neither was surprised. The universe was trying to get my attention, they said.

They were right.

The next morning my body sent a message of its own: I felt like 50 tractors were running over me for what seemed like several hours. Thankfully, by the time my GP called in the afternoon via video chat, I was tired but pain-free and had asked my twin sister to join me via telephone to listen to his news. Tumour markers for ovaries, breast, and pancreas were all up, he reported, and it would take another 24 hours to find out whether the fluid they had drained contained malignant cells.

“Would it be a fair assessment, given this news, to conclude that I am not fine?” I asked.

I had thought very carefully about how to word my question ahead of time, given that he was so hesitant to say the word *cancer* out loud.

“Yes,” he replied simply.

My GP’s short, emotionally charged answer was as devastating to me as his advice not to have any of my family or friends travel 2000 kilometers east or cross the Atlantic to support me in the coming weeks and months. The risk of contracting Covid-19 and passing it on to me was simply too great.

My sisters and I were heartbroken. Who would take their place?

Small miracles

“Of course, I will look after you when you come home from the hospital,” said a dear friend who always stayed with me when she was in town. I knew that her schedule as a fellow academic would be flexible because she was on a research sabbatical until the end of the year.

Nevertheless, her heartfelt offer made me — who never cried — burst into tears.

“You have paid into the friendship account all your life, and now it is your time to accept blessings,” she reminded me.

I was possibly even more touched when a trusted neighbour friend who had lost her husband to cancer a long time ago, informed me and my worried relatives that from now on she was going to be my “go-to, in-house person.” She vowed to be there for me — “I will be checking up on you daily now, Barb!” — and to chauffeur me to the hospital, get groceries, etc.

Having lost my own mother at age 17, I felt that she had just gifted me with the most wonderful substitute — or had my trusted neighbour been sent by my “Canadian mom,” who had passed away in October 2019? A fellow German immigrant to Canada, she had, in part, been responsible for me pursuing an academic career (“You are a clever girl, Barb — go for it!”).

I also knew I could rely on her daughter, a close friend since university days and psychiatrist by training, as well as my clergy friend in Alberta for mental health support in the coming months. “We will get you through this,” these “sisters from another mister” told me, and I had every reason to believe them.

The following day my GP called with excellent news, for once — there were no malignant cells in the fluid! I felt relieved beyond words, as did my twin sister who I had asked to be part of the phone conversation. He had also arranged for a gynecological oncologist to see me as soon as possible. Incidentally, the word “cancer” still had not crossed his lips, even when I asked him when and what to tell my employer about having to go on medical leave. I was to wait until I had received a proper diagnosis, he advised.

“I was supposed to teach three classes in the fall,” I pointed out to him.

“Could you move one of them to next year?” he wondered.

“That’s a great idea,” I said and began putting together possible replacement scenarios that evening. Letting someone else teach the “little darlings” did not bother me. I was used to handing over my students to my colleagues whenever I went on research sabbaticals.

However, an e-mail from my academic dean as my immediate supervisor about her upcoming vacation made me disclose my news to her, her assistant, and my new department head the very next day. I will never forget their shocked faces — in gallery view, courtesy of Zoom — as I shared my “this is what’s been going on and what I think we should do next” plans with them. The little girl inside me was shaking, however.

“What are we going to tell the students?” I asked. In response, my department head and I prepared a letter to let them know about my impending surgery and medical leave. “Let’s say something like ‘Dr. Reul looks forward to torturing you again soon’,” I suggested, making my department head chuckle despite the seriousness of the situation.

Finally, we decided that I would contact those faculty members close to me on my own and otherwise say as little as possible for now, also not to worry anyone (“What you don’t know, can’t hurt you”).

Then, another, most welcome “small miracle” occurred. An instructor who was qualified, able, and willing to teach my fall semester classes was found within days. I was incredibly relieved, to say the least.

At the same time, I was wondering whether what was happening to me was, in fact, real. To that end, my clever top-hatted guide suggested that I would let my adult self (“Dr. Barb”) handle all the difficult tasks. This, in turn, would help calm my [inner child](#). She was scared beyond belief by now and ready to throw a nasty temper tantrum at a moment’s notice.

“Make sure you are super patient with yourself and brutally honest as far as your innermost feelings are concerned from now on,” my guide advised. “I will try,” I replied.

Nevertheless, I wondered whether the road ahead would be filled with emotional traffic lights (“Red means ‘Stop!’”) or scary physical potholes (“Caution – Construction ahead!”) that I would need to circumvent carefully, if not avoid altogether. Turning to my guide for answers, I was greeted with silence. I knew from experience that this meant “trouble ahead.”

If nothing else, when the [local cancer agency](#) called to assign an ID number to me (“Please quote it every time you call!”) and told me to come in for a [biopsy](#) on July 31, my diagnosis had become official. But how could I have cancer, given that I looked like a million bucks and continued to be pain-free with energy to burn?

Maybe it was all a big mistake, and “Dr. Barb” and the little girl inside her would wake up tomorrow from this bizarre dream.

Take a deep breath!

I felt unexpectedly calm and even cheerful before the biopsy appointment. It took place at the cancer clinic located inside the other hospital, an easy 10-minute car ride from my place. I was relieved to be able to bring a support person, a welcome exception to the otherwise very restrictive Covid-19 hospital rules. A beloved colleague-friend accompanied me, and together we waited for 45 agonizing minutes and caught up on our respective lives (“Do you miss teaching [aquafit](#) classes at the pool, Barb?”) until my name was called.

Much to the merriment of the nurse who took my vitals, we kept cracking jokes in the treatment room. “Who knew that two middle-aged female university profs could be this entertaining, eh?” I concluded with a big grin on my face. I finally sobered up when the medical [oncologist](#) entered the room.

“So, who is that GP of yours who managed to get you on my table within one week during a pandemic?” she asked, smiling. It hit me that I owed a huge debt of gratitude to two special “miracle workers” — my fantastic doctor for turning into [Superman](#) when I really needed him to, and my wonderful massage therapist for urging me to go and see him in the first place.

I was also thankful beyond measure when the oncologist and her resident finished their examination (“take a deep breath!”). Since the pain had been excruciating — they had, after all, cut a tiny bit of flesh out of my uterus! — and nobody offered me a painkiller, I promptly threw up a few minutes later.

Everyone in the room but me assumed that it was a physical reaction on my part to finally having been told the ugly truth: I had at least [endometrial cancer](#) which begins in the uterus, if not two cancers — endometrial as well as [ovarian](#) — wreaking havoc inside me. In hindsight, I realized that if vomiting had been my body’s way of acknowledging my illness for the first time, it certainly had taken a long time for it to catch up to my mind.

I was surprised to hear that the specific type(s) of cancer and [stage\(s\)](#) could only be determined during, if not after, surgery.

“Would Monday, August 10, work for you?”

“Then I will spend my 53rd birthday in the hospital.”

“Do you want me to move it by a week?”

“No.”

“Given that you are in excellent physical shape, you might be able to return home that evening to celebrate!”

We were both wrong.

“Do you want me to stick around for a bit?” my colleague-friend asked on our drive home.

“No, I’ll be fine,” I answered.

Using a professional, matter-of-fact tone that had served me well in previous crisis situations, I (or, more precisely, “Dr. Barb”) proceeded to tell my relatives and closest friends about my experiences at the oncologist’s office and the upcoming surgery. They assured me that they would be with me every step of the way on my cancer journey — and that they would keep the information private.

It is not your story to tell

Let me fast forward a bit to explain what I mean. I will never forget when a well-meaning friend cancelled a meeting for me in late August (“She is not well”). It prompted a phone call by a concerned acquaintance.

“What is really going on? You are never sick,” she asked.

“I had major abdominal surgery,” I replied. It felt positively bizarre to share my diagnosis and treatment plan with people who were not close to me — it was none of their business. Others had the nerve to scold me for not being in touch, arguing that they would have prayed for me if nothing else. “You can always start now,” was my cheeky response.

In hindsight, I should have anticipated, however, that certain comments would indeed upset me. My academic self was used to being critiqued by her peers (“Thesis statement lacks focus”) and had grown a thick skin over the years. My heart, however, was

unsure how to interpret some of the feedback I received. “Then make it a teaching moment,” my wise clergy friend from Alberta suggested after listening to me rant about it repeatedly.

What to say when you don't know what to say

So, how should you talk to individuals who share their diagnosis with you and/or are about to begin or already undergoing cancer treatments?

“Show sincere [empathy](#) in your response” and “refrain from giving advice of any kind” are, in my experience, the two most important things to remember in this regard. If you struggle with the first suggestion (“How do I do that?”) and/or resent the second (“That is what I always do because it works!”), you are probably not alone. At the same time, you will arguably risk hurting the other person who likely lacks the energy, if not the confidence, to point out your unintentional [faux pas](#) (if not to you, then to others).

Replies such as “my heart goes out to you” or “that really sucks!” always made me feel better. I also appreciated direct offers of assistance — “what can I do to help today — tomorrow — this week?” — more than having to reach out to others (“call me if you need anything”). Why? Because I was used to looking after myself and did not want to be a bother to anyone, especially during a pandemic that kept everyone on their toes already.

Reactions to my news that implied — even unintentionally — that the illness was somehow my fault because of my busy lifestyle, were never welcome because they made me feel like I needed to defend the way I had chosen to live my life so far. Listening to depressing or plain wrong cancer survival statistics (“I am certain you will be amongst the 43% of women who make it”) was similarly annoying. Several people also surprised me by bursting into tears when I did not expect them to and others by staying remarkably calm (you know who you are!).

My least favourite response was to have someone grieve my future for me (“You could die!”) — did this person have access to a special crystal ball or had they acquired a medical degree overnight to make such truly scary predictions with such confidence? I also tired quickly of other, undoubtedly well-intended comments such as “you’ll be just fine,” “you are strong and have nothing to worry about,” and especially “the worst is (soon to be) over.” These comments always made me turn to my favourite, admittedly unhealthy, coping mechanism — Nutella!

“I have a suggestion,” my twin sister said when I told her that I had no interest in watching my waistline expand while simultaneously wasting precious energy on feeling upset, if not angry.

Incidentally, she herself had found my “Do not tell anyone!” command tough to follow.

“I feel like I am pretending everything is fine when the opposite is true,” she said.

“I know,” I replied. “But I want to protect you of all people from being overwhelmed by others wanting to know how I am doing.”

“Fair enough,” she responded. “Why don’t you tell repeat offenders to think of your medical journey as a pregnancy of sorts, with every day being different?”

“That’s a brilliant idea.” I put it at the top of my own “what to say when you don’t know what to say” list. I also requested from my relatives and friends who checked in regularly — bless their hearts! — to open the conversation with the question “How are you doing *today*?” It allowed me to share as much as I wanted when I felt ready.

The most helpful comments were offered by my fellow cancer survivors (“There’s a pill for that, Barb!”). They also taught me to call my doctor or oncology nurse if I had questions or if something did not feel right (“That’s their job!”). And I was not to ask the internet for advice (“You’ll regret it every time”). In hindsight, they were right, of course.

In fact, “Dr. Google” was not helpful either as I pondered certain big-picture questions that worried “Dr. Barb” the most. The majority focused on the future. What was taking an extended break from both teaching and research going to feel like? Granted, I was a full professor with [tenure](#). Would a lengthy medical leave seriously hurt my career, financial goals, and long-term retirement plans? Would I be devalued, if not stigmatized, in the highly political university environment that valued productivity over all? And what impact, if any, would the pandemic in general and Covid-19 restrictions specifically have on me returning to work, whenever that would be?

One question focused on the recent past. I was the third female faculty member at my College to have been diagnosed with cancer in the past five years. Granted, our types of cancer were all different. But our three offices were hidden away in the northwest corner of the building, far away from our colleagues. Was the [shared location](#) a mere coincidence or an important signal that something beyond our control had made us more susceptible to getting sick with cancer?

“Stop worrying,” my main guide said with a stern voice every time “Dr. Barb” pondered the past or the future. “Focus on your little girl inside and the present instead.”

The Power of Now

To everyone’s delight, including my own, I felt fabulous during the week that preceded the surgery, probably because of a new and most welcome addition to my living room. Dear friends of mine had asked me a couple of days after the biopsy whether there was something I had always wanted to buy myself that would make a difference in the coming months. I promptly said, “A new, smart TV!” A few minutes later, I hopped into their van to go shopping. A couple of hours later, I happily cued my favourite show on German television ([Der Bergdoktor](#), a series set in the Austrian Alps focusing on a handsome medical doctor on whom my inner child had a big crush) to watch on my new, big screen television.

“Why didn’t I make this purchase much sooner?” I asked my friends, grateful for their help.

“Now is perfect,” they said with a smile.

Staying grounded also helped me manage endless phone calls, get through multiple pre-surgery appointments, and endure being poked in both arms incessantly (“You have good veins, Barb”). Filling out a detailed “[Living Will](#)” which communicates my wishes for end-of-life medical care to my medical team as per the hospital’s request, did not scare me too much either. It did, however, necessitate another “which one of us will go first?” conversation with my twin sister.

We had last addressed this admittedly tough topic in January 2020 when I updated my will and had given her a copy as my executor. Even though neither of us was expecting me to not wake up from the surgery, preparing for the worst-case scenario while hoping for the best outcome now made complete sense to both of us. In hindsight, I realized how surreal it must have been for my twin sister to imagine a world without me in it — we had been together ever since conception and exceptionally close ever since our mother’s untimely death.

Weekend woes and weekend joys

“Hello, I am calling to remind you about your operation on Monday at 8 am,” said a pleasant female voice three days prior to the surgery, a Friday afternoon.

“How could I forget?” I thought. I had been wearing a hospital wrist band for several days already.

“Do you have any questions about the bowel preparation medication you are to take on Saturday and Sunday?” she asked.

“I was told I did not need to do that,” I replied.

“You do,” the nurse insisted.

Her colleague had apparently not read the doctor’s instructions all the way to the end! (I promptly wondered what else had been missed.)

“Drink apple juice after having taken the medication to keep up your electrolytes,” she said.

“No worries,” I replied. I had just bought some because the oncologist had recommended that I drink it the night before the surgery.

On Saturday morning, my bowels were not at all amused. In the afternoon, my throat suddenly became very sore and half an hour later, my chest was on fire. I checked my temperature: it was normal, which meant that it was probably a cold rather than Covid-19. Should I call the clinic to report my symptoms and risk being told that my surgery would be postponed?

“No way!” I thought. Instead, I consulted “Dr. Google” who diagnosed instantly that [acid reflux](#), specifically the apple juice, was to blame. He suggested switching to a clear carbonated drink which made me run to the corner store for some ginger ale.

By the evening I felt so well again that I walked all the way around the large lake near my house and enjoyed the fantastic weather and gorgeous sunset.



Sunset at the lake

“Hello Barbara — how are you doing?” said a familiar voice behind me. It was my favourite yoga instructor, with whom I had lost touch after starting to exercise with my twin sister online. She noticed my wrist band, and when I told her my news promptly offered to drop off food after I returned from the hospital.

“You are on!” I responded, not at all surprised by her generosity. She was surprised, however, when I told her about my favourite part of her wonderful yoga sessions which had kept me sane during stressful work weeks.

“You always reminded us at the very end of class that we are all connected,” I said.

“It has meant that much to you?” she asked, surprised.

“It has meant everything and even more now,” I replied. And then I thanked the universe for reconnecting me with one of the most positive, smiley people I had ever met at a time when I needed her the most.

“I am ready for tomorrow,” I concluded on Sunday evening. My bag was packed, and my twin sister had kindly agreed to update everyone who knew afterwards. My own pre-surgery agenda had included both praying (it pays to be friends with clergy!) and reading, including the classic [When Things Fall Apart](#) by Pema Chödrön, as suggested by my trusted massage therapist. Moreover, creative visualization exercises had informed my hour-long meditative walks that I (still!) went on almost every day after having been diagnosed. *I am strong, I am healthy, I am cancer-free* was my chosen mantra, followed by *This or something even better now manifests for me, in totally satisfying and harmonious ways, to the highest good of all concerned*. This statement — courtesy of [Shakti Gawain](#), whose book on creative visualization is another classic — was perfect for my inner optimist because it focused on the best outcome scenario. When my MD friend’s husband (whom I love dearly) implied that the operation would be “a bumpy ride,” but that in the end I would be fine, I decided to just ignore him.

After all, what could go wrong?

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4: “Treat the patient, not the monitor” — The Hospital Stay

What you are going to read now is a chapter that has a regular beginning, an awful middle, and, thankfully, a happy ending. In hindsight, some parts also offered a “foretaste of the feast to come,” but most of it felt like being on a reality TV show without even trying!

On Monday, August 10, 2020, the day before my 53rd birthday, the same friend and colleague who had accompanied me on the day of the biopsy took me to the hospital in the morning. We arrived a few minutes early, checked in, and went up to the surgery ward. I put on an ugly hospital gown and even uglier housecoat, had my vitals checked, and got a Covid-19 test done. Then we chatted about work-related matters for the most part, waiting patiently for my anesthesiologist and oncologist to meet us.

Upon hearing that I could not have a family member with me, the nurses decided to let my friend stay with me until about five minutes before they rolled me into the operating room. My anesthesiologist then administered an [epidural](#), which hurt more than I had expected. I remember thinking, “I feel sorry for all those women in labour!”

I closed my eyes, asked the universe to look after me during the operation, and went to sleep.

When I opened my eyes six hours later, the nurses asked me for my full name and, according to my twin sister, whom they called after I had been taken to the recovery room, I was joking with them. While that sounds just like me, I have no recollection of it at all. In fact, all I remember is that for the next seven hours, I had my eyes closed (for the most part), but was, in fact, fully awake and listening to every word that was said; this is also what happens when I am on plane rides, regardless of the length.

As a result, I got more and more worried: my blood pressure was apparently tanking, especially the lower number, which refused to rise above 40. The nurses soon called my [anesthesiologist](#), who said I had lost a lot of blood during the surgery (“What?!” I thought). Then she rattled off a list of drugs to give me and left for the day.

When my condition failed to improve within the hour, the nurses contacted the anesthesiologist on call for a second opinion. She checked me out, got really upset with her colleague for bombarding my body with (unnecessary?) medicine, and shouted “Let’s treat the patient, not the monitor” before she stomped out of the recovery room.

I did not know what to make of what had just happened, and neither did my nurse, who then decided to call another doctor for a third opinion. This white-haired gentleman patted me on the shoulder and then announced that both of his colleagues were wrong.

“She’s not sick enough to go to [ICU](#),” he declared. “But she is not well enough to go to the ward either.”

Consequently, I was going to be sent to the [medical surveillance unit](#) for observation first.

At that time, I opened my eyes and said to the nurse, “Please call my twin sister and tell her about this change of plan.” She hesitated at first, but when I noted that it was close to midnight and this was a birthday wish, she readily agreed. I was stunned and angry, however, when she described my current state to my sister as “just fine” (!), but there was nothing I could do about it either, so I let it go.

Another nurse and an aide then started pushing me out of the recovery room. I asked them for the time.

“It’s 11:55 pm — why do you want to know?”

“Because I am going to be 53 years old at midnight.”

They promptly launched into an off-key version of a [tune](#) I knew all too well.

Happy birthday to me

Hooked up to a multitude of machines, I decided there was absolutely nothing to celebrate at the hospital. My left arm was being squeezed every 15 minutes by a blood pressure cuff, a patient across the hall kept screaming “Help me, help me!”, and a nurse came in twice to take blood from my feet because there were no veins left in either of my arms (!). I was also getting increasingly annoyed with being asked to [rate my pain](#) by the night nurse.

“Still 10 out of 10,” I said. “Which part of that do you not understand?” I snapped at her. Had they run out of painkillers in this place of healing, or what was going on...?

When sunlight flooded my room in the morning, I realized that someone had written a congratulatory note on a board on the wall across from my bed. “It’s going to be one hell of a birthday,” I thought, wincing when a new nurse came in and lifted the flat sheet on top of me to check out my belly.

Heavy fatigue set in, which felt like 500 tractors slowly rolling up and down my body. When the oncologist resident came by to see me early in the morning — with Starbucks cake pops in hand as a present — I put up a brave front. In reality, I was ready to lose my mind. Were there no physicians in this hospital who were authorized to give me something for the pain?

“Hello, my dear,” a doctor with a booming and annoyingly cheerful voice said. “How are you feeling?”

“Awful,” I said, cussing under my breath in German first. Then I waited for him to offer a medical rationale as to what was going on. He did: part of my epidural had not worked.

“It happens,” he said matter-of-factly.

Upon finding out that I was a music history professor, the doctor dashed away, only to return with an iPad in hand. Soon a “[Best of Baroque](#)” recording was playing on YouTube. “Maybe he knows what he is doing after all,” I had to admit, smiling for the first time on my birthday.

But there was a price to pay — he wanted me to have some breakfast. Specifically, he wanted me to drink a half a bottle of [Ensure](#). “I’ll try,” I said.

Then, I promptly threw up all over him.

“Oh @\$%,” I thought, feeling too weak to apologize.

“Oh dear,” he commented, asking the nurse to increase my nausea medication before he left.

I decided to pass the time by responding, albeit briefly, to the many “Happy Birthday” wishes I had received on my phone as texts and as messages on social media. One wish was even delivered in person that afternoon by one of my pastors, together with a beautiful prayer of hope.

It worked, as the night was (mostly) bearable. The colleague-friend who had dropped me off at the hospital visited the next morning; what a gift during Covid-19 times! I promised to try a bit of the opera cake (“How appropriate!” I thought) she had brought with her once I had been moved up to the “Short Stay” Ward. Two new “friends” were going to come along — a [catheter](#) and a [plastic bulb](#) which captured other bodily fluids, both of which needed draining on a regular basis. “I am never going to get any sleep,” I remember thinking.

You will be going home soon

My new room had the most depressing lack of natural light and came with a roommate I could not see as curtains were drawn all around my bed. Then, an unexpected gift arrived: a former colleague and fellow cancer survivor, who had moved to Montreal several years ago, had sent a huge box of delectable chocolates! They lifted my otherwise foul mood immediately, and it felt good to gift them to the nurses to show my appreciation for their hard work.

At lunch time, the emergency doctor with the booming voice wanted to watch me eat solid food — a decision he regretted shortly after when I threw up all over him again.

My oncologist and her resident also dropped by later that afternoon to inform me that they had found only one cancer (endometrial, i.e., uterine) which had spread to the ovaries; all lymph nodes had also been removed as a precaution. I would receive a call in about two weeks to tell me about the stage of the cancer. “That’s good to know,” I said.

Then, the oncologist confidently announced that I was going to go home the next day.

“Is she crazy?” I thought.

Surely, the nurses had told her that they had tried to get me to stand up four times, only for me to get super-dizzy and practically faint on the bed every single time. “We are all praying for you,” my sisters in B.C. and Germany as well as my pastor friend in Alberta said on the phone. They were all worried that I was not improving more quickly.

I finally started feeling better in the evening when my hidden roommate asked whether I wanted to listen to some hymns that had been recorded by musicians at her church. To our mutual delight, it turned out that I knew the pianist, a current student. She was

also acquainted with a retired colleague of mine who contacted me that evening and offered to accompany me to my first chemotherapy treatment. Small world!

Having been awakened several times during the night to have my two “friends,” the bulb and the catheter, administered to, I was back to feeling awful the next morning.

“Now 699 tractors are running over my body!” I told my twin sister on the phone.

“Be patient,” my oncologist was quick to say when she visited.

She also reminded me that I had just undergone major surgery, lost a lot of blood, and needed to give my body time to heal. That was, incidentally, exactly what my oldest sister had told me the night before.

“Fair enough,” I thought, only to be stunned when the oncologist proceeded to tell me that I was going to be discharged soon.

“She *is* crazy,” I concluded.

“Why don’t you take a page out of the anesthesiologist’s book and treat me, the patient, not the monitor?” I snapped at her and her resident (and apologized for it the next day).

Whining to my family and friends at different times and across various time zones did little to lift my spirits either. Being moved over to the window spot after my roommate was discharged after lunch finally brought some relief.

How I had missed seeing the sun and blue skies!

Getting up to get out

“Change your attitude, or you *will* stay here forever” was the message my main guide — the one with the top hat — sent me seemingly out of the blue later that afternoon. I promptly pressed the red button to get the staff’s attention.



At the hospital

“I’d like to get up,” I said.

“I’ll help you,” the nurse replied. Gritting my teeth and using sheer will power, I did!

“Excellent; you keep that up,” she encouraged me.

“I will!”

I knew I had turned another corner when I put on music by my favourite composer a little later and began to relax despite of the uncomfortable hospital bed. Then, I asked whether it was possible to wash my hair. It involved a wet shower cap with shampoo on the inside and felt heavenly.

The next day, I amazed everyone — including two nursing students who had done a great job earlier cleaning my huge scar — first, by getting out of bed to sit in the chair next to the window, and then, by using a walker to circle the ward twice. A physiotherapist who introduced himself in the hallway, followed me to my room. “You were clearly in excellent shape before you had surgery,” he said.

“I plan to bounce back in record time,” I replied.

It still took several days to be well enough to leave the hospital. On the weekend, I kept pestering the nurses about being discharged, but not before they had removed my catheter and bulb “friends,” as I was not willing to take them home with me. In turn, the nurses kept bugging me to eat more and get my digestion going.

“I would love to — if only I could prepare the food myself,” I said. To prove my point, I sent my twin sister a picture of what had been sent up for lunch; I had ordered it from a list provided by a [dietician](#) the day before. My sister was unable to identify any of the dishes, much to my delight.

“According to the printout, the yellowish bit is supposed to be chicken pot pie, and the green liquid is listed as turkey soup,” I informed her. Thankfully, another colleague-friend — a fellow German who, incidentally, one of the nursing students recognized as her biology professor — visited on Saturday and Sunday. She brought two bags of sweet treats with her, one of which I shared with the staff, much to their delight.

Finally, on the morning of Tuesday, August 18, 2020, over a week after I had undergone surgery, I was told that I would be discharged after lunch. Hallelujah! Waiting for my oncologist to make it official, I circled the ward again and again, quietly sending love and gratitude to everyone who had been praying for my speedy recovery.

Then, a nurse told me what to expect over the course of the next month or so. My oncologist would call me about the stage of my cancer in two weeks and do a follow-up exam three weeks later; my first of six [chemotherapy](#) appointments had been booked for September 25, 2020. Until then, I was to take it easy and try to gain some of the weight back that I had lost because of the surgery. “I’ll do my very best,” I promised.

Handing thank-you cards to my oncologist, her [resident](#), and the staff, I grabbed my bag as well as the medication and blood thinner I was to take home with me and walked (!) down to the main entrance, where my colleague-friend was waiting.

“I expected you to be in a wheelchair,” she said.

“Not needed,” I replied.

However, the pillow on my belly during the ride home was a visual reminder that I had a lengthy recovery period ahead of me.

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5: “Walk 5 — Rest 5 — Walk 5” — Recovery, Part 1

I will never forget what it felt like to walk into my “sanctuary,” as I like to call my bright, west-facing apartment.

The energy in the hospital had been dark (for lack of a better word), while here, at home, it was light and sparkly, the perfect environment to heal from what had admittedly been an ordeal. I was also exceedingly grateful to have my very own “butler-nurse” (as one of my sisters would later call my friend who had travelled 260 km south to join me) at my disposal for an entire week. To sleep in my own bed with approximately ten pillows to keep me comfortable was another welcome gift, in contrast to waking up with a belly that felt like I had swallowed a ten-pound dumbbell!

At the same time, I was not really surprised. The bulb — which had drained the fluid my body was producing post-surgery — and the catheter had been removed prior to my departure from the hospital. My oncologist’s nurse assured me that massaging my abdomen would bring relief, as would bowel movements. She was right, and by the afternoon I had enough energy to get into my friend’s car and go to the pharmacy, followed by a much-needed trip to the grocery store.

That evening, I also stepped on a scale, stunned to have lost ten pounds in a week. As if by magic, food began to be dropped off at my house (“You eat meat, right?”) to help me gain it back quickly. The biggest culinary treats that first week were a roasted chicken expertly prepared by my Greek colleague-friend, a huge chocolate cheesecake baked to perfection by my former department head, and a yummy vegetarian “[Buddha bowl](#)” cooked and delivered by my favourite yoga instructor.



Getting well wishes

I was also very grateful for the multitude of presents and cards sent by family members and friends from out of town, being prayed for by my church community, and receiving countless, often daily texts and e-mails of encouragement on my phone. The cutest surprise was a picture sent via [snail mail](#) that featured my seven-month-old grandniece from B.C. holding up “get well” signs her mom and aunt had specially made. “Bless their hearts,” I kept saying, highly motivated to please everyone, including myself, with a speedy recovery.

It's a marathon, not a sprint

Every activity, big or small (like taking a shower or washing my hair) now required careful preparation as well as resting beforehand and afterwards. As a high energy person, having little to no physical strength was a real challenge. My “butler-nurse” friend suggested I keep a detailed diary to track my diet, energy level, and physical activity. “Good idea,” I replied.

The following morning, she urged me strongly to try and walk for five minutes to visit my friends who had helped me with setting up my TV and lived close by, rest for a while, and then walk back. “Bad idea,” I replied.

Handing me her partner’s [walking poles](#) which she said I could borrow for the week, she would not take no for an answer and gently pushed me out the door. “This feels like a marathon from hell,” I muttered. It would be impossible to increase my activity level by a whopping 10 minutes every week, as suggested by my oncologist.

“One day and one walk at a time,” my friend said, when I shared my concerns with her. To make it fun, she chose increasingly exciting locations in and out of town for us to be active together. To my amazement and amusement, she used a stopwatch to track my progress (“walk 5 — rest 5 — walk 5”). Had she perhaps been a German drill sergeant in a past life?



Looking “cool”!

“Plan your walks according to the number of rest opportunities such as benches and steps to sit on from now on,” she suggested.

“Excellent idea,” I replied. The new walking poles which I had been gifted by the friends we had visited shortly after I got back from the hospital also helped me feel secure and look “cool” when venturing outside.

Six days after returning from the hospital, I had 35 staples removed at a medical treatment centre nearby. It did not hurt at all, unlike my friend’s departure that same afternoon. When was I going to see her again in person, I wondered, given the pandemic? And what would it feel like to return to an empty apartment? I should not have worried.

My friend had already lined up my trusted neighbour, who checked on me daily, and the friends who had helped me with setting up my new TV (“We are making supper for you tonight!”) to keep me company whenever I craved it. Using technology, family members and close friends also kept a close eye on me, and with good reason: I still needed a lot of rest, and my body promptly complained (“why is my belly hurting again?”) if I failed to comply.

Getting my work-focused brain to relax was a different matter. It was late August after all, and the fall semester was about to begin. But I soon realized that spending quality time on my balcony every day to connect with nature – were Canadian prairie skies always this blue? – helped me heal on the inside and the outside.

The diagnosis

On Tuesday, August 25, 2020, my oncologist called with the most fantastic news. Much to her surprise, the cancer — which she was able to remove completely — had not spread to the lymph nodes before the surgery. This meant only a [Stage 3A diagnosis](#), rather than Stage 4, which indicates that the cancer has metastasized to other, distant organs of the body and may be terminal.

“Thank you to everyone who has prayed and looked out for me,” I thought, convinced that my daily creative visualization exercises had made and would continue to make a difference as well. However, since the cancer had spread so aggressively from the uterus to the ovaries, it still meant six rounds of chemotherapy, which were to begin in a month, followed by radiation therapy in the new year.

Determined to be as healthy as possible for my follow-up appointment in September, I adopted an admittedly slower and gentler rhythm during the next four weeks in recovery mode. Regular “walking dates,” as suggested by my “butler-nurse” friend on her last day in town, turned out to be absolute highlights. Each involved my beloved walking poles, special friends as well as colleagues from work (and often their dogs, if not spouses), and long, meaningful chats.

My mental health also benefitted from taking my car for a spin a couple of times a week, always carefully adjusting my seatbelt around a pillow on my belly. The drives were short, though, because sitting for long periods of time continued to be a problem. I was also not allowed to carry, push, or lift anything for six weeks after the surgery either.

Gentle stretching routines were now possible again — and most welcome. On September 1, 2020, my twin sister and I resumed our online physical activity sessions, starting with 10 to 15 minutes a day, minus ab work and anything else that would stress my body too much too soon. What a blessing to share this special time with her!

I was admittedly angry and frustrated, however, that a pandemic of all things made care visits impossible. It would have meant the world to me to welcome anyone who had offered to stay with me, specifically my three sisters, two nieces, my oldest friend from university days, and my former colleague and fellow cancer survivor who had moved to Montreal.

Instead, I was to ask my “local family” for assistance. I am happy to say that they exceeded all expectations. “How lucky am I to have so many wonderful people right here in town to help me,” I kept saying to myself, truly grateful for their continued support.

What are the odds?

Prior to the surgery, I had relied much on the support of my best friend, a local high school teacher and fellow musician. Unable to travel due to Covid-19 restrictions, we had spent a lot of quality time together in the spring and early summer, both indoors and outdoors. When she first shared her cancer diagnosis with me — I was still at the hospital — and that the doctors were working on a treatment plan for her (“I am complicated”), my heart sank. Was she going to die soon?

A Positron Emission Tomography ([PET](#)) scan finally brought clarity. It was Stage 4 [Hodgkins’ Lymphoma](#) requiring IV and oral chemotherapy; major surgery and radiation were impossible because of her pre-existing conditions.

“What are the odds,” I asked myself — and the universe — “that two best friends embark on a cancer journey at roughly the same time, and what are they to learn from it?”

In practical terms it meant that neither of us was going back to teaching in early September 2020. Never having been on medical leave before, I continued to be quite anxious about being forced to fill my time with mindful non-work-related pursuits for the next six months, if not longer. Granted, part of me was relieved about not having to run an online classroom; only one week into the semester many of my colleagues were ready to lose their minds (“I had 160 e-mails in my inbox after returning from a one-hour meeting!”).

At the same time, I feared that the insurance would not approve my long-term disability application. Over the years, I had heard my fair share of [horror stories](#) about cancer patients being forced to work throughout their treatments and return to duties too early in order to avoid losing their jobs. I also could not shake the horrible feeling that I *would* be penalized or stigmatized intentionally or inadvertently, now and/or in the future, for not being productive and/or active in the academic community.

And would the global pandemic be over by Christmas or linger on?

Feeling loved

In hindsight, I should have been more trusting regarding the tremendous, continuing support my colleagues would provide, regardless of location and time zone (“It’s eight hours later in Sweden, so if you can’t sleep and want to talk, give me a shout!”). My wonderful, loving church community had also been keeping me in their prayers since my surgery in August. Anticipating that playing for services was going to be out of the question for months to come, I focused on the two Sundays that were left before chemotherapy was to begin in late September.

My pastors promptly worried about me contracting Covid-19 if I rejoined their small bubble (“Will wearing masks and physical distancing be enough?”). I worried about not being able to sit on the piano bench long enough to make it through playing a 10-minute-long prelude.

During my first practice session, I realized that if I strapped a small pillow around my belly and rested before, during, and after the service, I could do it! One of my trusted substitutes had also indicated that she was willing to cover for me during my absence, for which I was incredibly grateful. Still, not playing at all felt like a minor death sentence. Could I perhaps come in on “good days” when there was nobody at the church to avoid exposure to the coronavirus?

The pastors agreed immediately, which meant more to me than can be expressed in words. Granted, there would not be a congregation and/or choir to play for. But I could still sing along to my favourite hymns — “[Abide with me](#)” and “[Jesus Christ is risen today](#)” come to mind immediately — and entertain myself with fun music for pipe organ by J. S. Bach and the many wonderful composers who have followed in his footsteps and kept organists literally on their toes.



Hello, snazzy Audi convertible!

My mental health also benefitted tremendously from being whisked off to fun locations by friends and colleagues in September. One of my favourite outings involved posing in front of the [local Harley Davidson](#) dealer — I have been known to ride motorcycles, albeit as a passenger — with my former department head who was on sabbatical; another was being chauffeured to a lovely prairie lake by my favourite yoga teacher in her snazzy Audi convertible. Life was great!

Everything changed on September 17, 2020, when my retired colleague from Montreal texted me from the hospital. “The cancer is back,” her message read.

It shocked me to the core, as I had been the first person whom she had called to share her cancer diagnosis with over a decade ago. She had been wonderfully supportive ever since I had received my diagnosis (“My favourite Barb, we will get through this together!”). The recurrence meant that there were now three of us battling cancer at the same time.

“It feels like the universe has pushed the ‘pause,’ if not ‘stop,’ button for all of us,” I said to my twin sister that evening. “I have no idea for how long.”

Treatment (and other) plans

My own life was going to get busy soon. I felt (and looked) great on September 22, 2020, when I saw my oncologist for the follow-up appointment. Due to pandemic restrictions, I could not bring anyone with me. My trusted neighbour was, however, allowed to join us on the phone and even volunteered to take minutes of everything that was said.

“How have you been?” the oncologist asked me. She was pleased with my post-surgery recovery. “That scar of yours is healing nicely!” (Maybe I would be comfortable showing it off to someone not related to me a year from now?)

The doctor’s most shocking news, though, was that only 60 to 70 percent of women with Stage 3A endometrial cancer were still alive five years later, and those who did would be susceptible to breast and colon cancer forever afterwards. My [treatment plan](#) would have to be quite aggressive as a result, and I should be prepared to deal with a multitude of side effects. That was hard to hear, especially given my excellent health prior to the cancer diagnosis.

“We will give you a lot of paperwork to go through carefully before and during your treatments, Barb,” I was told.

“No problem,” I replied, even though I secretly dreaded the idea.

The oncologist’s advice to take an entire year off (“that much?”) to focus on my health scared me; her recommendation to do what I enjoyed during that time gave me hope.

My “homework” as a patient was simple enough: no dieting, no alcohol, and no supplements besides Vitamin D. Most importantly, with a pandemic going on, my bubble needed to be super-small, and I was to wear a mask whenever I left my apartment, including when going for walks outdoors. Finally, blood work needed to be done 48 to 72 hours before each treatment, rather than on a weekly basis as had been the case with my cancer survivor friends in pre-pandemic times.

“Your wish is my command,” I said to the oncologist, and I meant it. She was the expert, and I knew instinctively that I could trust her, not unlike my students trust me.

The most memorable part of our conversation was her answer to the following question: “If I start on Friday and there are three weeks between each cycle, my last chemotherapy round would be in early January 2021, yes?”

She laughed out loud.

“Your body, not your head, will decide when it is time to move on to the next phase of your treatment, my dear,” the oncologist advised.

“Whatever that means,” I thought, rolling my eyes, albeit only in my head, and feeling sick to my stomach.

In hindsight, it turned out that she was right.

But I was still in charge until the first chemotherapy date. I remember intentionally cramming my agenda full the day before to distract myself. My top-hatted guide insisted early in the morning that I listen to a 30-minute guided [\(pre-\)chemotherapy meditation](#) on YouTube. The speaker sounded most convincing but nonetheless frightened me. I wept incessantly while wondering whether the chemotherapy drugs were really “healing energy medicine.” I was also skeptical that my body would indeed cooperate and instinctively know what to do to get me to wellness.

Change is in the (h)air



Change is in the (h)air

There were more tears when I went to see my long-time, trusted hairdresser for a super-short and super-stylish haircut before lunch. After much deliberation I had opted against investing in a hair piece. “Too expensive, too Halloweenish, and I won’t be going anywhere anyway because of the pandemic,” was my rationale. I planned to wear turbans instead, like the pretty bamboo one I had bought at the wig store the day before. The clerk had shown me several hats as well which featured buttons on either side onto which to hook one’s protective face mask. “What a great idea,” I remember thinking.

I mentioned it to my Greek colleague, one of the other female faculty members who had faced a cancer diagnosis in the past five years. A dear friend, she promptly dropped off not only multiple containers with food like she always did (“I love cooking, and you need to eat, Barb!”), but also a bag of hats she had worn while undergoing chemo. My immediate favourite was a charming, dark blue hat that would be perfect for walking outside. I made a mental note to sew buttons on it as well as on a couple of lined [toques](#) I had bought last year by the end of fall.

“You know, I covered my head every day so as not to frighten my children, who were not used to seeing me without hair,” she said. I had never thought of how my relatives and close friends would react if I embraced my baldness. Would they be shocked, or amused, or both?

“I better have a hat or scarf at the ready all the time,” I decided.

Then, I told her about how hard I had cried at the hair salon the day before my first treatment. “Who would have guessed that I was so attached to my chin-long, coloured hair?” I had said. I was also embarrassed about emptying my hairdresser’s tissue box in under 15 minutes and dreading the idea of returning any time soon for a head shave.

I eventually pulled myself together during a massage appointment in the afternoon. It was still a highly emotional visit, as the last one — during which I had declared that I did not have cancer — kept flashing through my mind. I grounded myself by shopping for groceries afterwards and distracted myself by going through insurance forms when I got home. “Thinking about the distant future is much easier than pondering the next day,” I concluded.

Finally, I met friends who were visiting from B.C. for a chat at a nearby restaurant with an outdoor patio. “That new haircut really suits you, Barb,” they commented. “You sound like you are more than ready for tomorrow.”

I would find out soon enough whether that was true or not.

5: “Walk 5 — Rest 5 — Walk 5” — Recovery, Part 1 is shared under a [CC BY-NC-ND 4.0](#) license and was authored, remixed, and/or curated by LibreTexts.

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6: “There is a first time for everything” — Chemo, Round 1

If you have never had chemotherapy, what do you think happens at the hospital during treatments and once you get home?

I only had a vague idea and reading up on it on the internet was more confusing than helpful. According to my breast cancer survivor friends, the lengths of treatments could differ considerably, and I would be sleepy for much of it (“The drugs will knock me out? Excellent,” I thought). Hopefully, I would not have to quit after the first round because of a severe allergic reaction like my beloved pastor friend from Alberta.

Each of my six “cycles” — spaced at least three weeks apart — required, at the very least three things from me: a) preparation (wear comfortable clothing; bring your own food and entertainment device and/or something to read); b) accepting help (from my trusted neighbour, who would give me a ride to and from the hospital); and c) stamina (lie in a hospital chair with an IV in my arm for six to seven hours straight).

“I am going to pretend I am taking trips to ‘the land of healing’ far, far away,” I remember telling everyone who had wished me good luck. Prior to the pandemic, patients had been allowed to bring along a support person each time. Now, only the first visit included that privilege.

Chair 19 adventures

I was glad that my retired colleague, our former College registrar, had volunteered to come with me to the cancer clinic on Friday, September 25, 2020. An unexpectedly business-like nurse first checked my vitals, including blood pressure, weight, pulse rate, and oxygen levels. It turned out that this was standard procedure, as it allowed my medical team to flag potential issues.

“You will be in chair 19 today,” another, much more caring nurse said and led me down a short hallway. The rather uninspired piece of hospital furniture featured a remote control that allowed me to put my feet up and lean back. I decided to pretend it was the chemo ward’s “hospital business class” section and sat down.

“See the smiley face hanging off the monitor? That is to let everyone know that today is your first time, and to give you a little extra love,” the nurse explained.

“Impressive,” I thought.

“You will need to unplug the monitor every time you want to use the toilet, which is around the corner,” she continued.

“So much for privacy,” I thought.

“We will also ask you for your full name and birthday every time the medication is changed,” she pointed out.

“Understood,” I replied.

When I was told that some patients had [adverse reactions](#) to one, or both, of the two chemotherapy drugs I would be given, my “inner commentator” finally shut up and let me focus on the task ahead. My right arm was warmed up with a blanket and an IV needle inserted.

Then, the nurse looked me straight in the eye. “If anything feels weird until you get out of this chair to go home several hours from now, you let us know right away, okay?” She was pleased to see me nod in agreement.

Shortly after the first chemotherapy drug (“we call it *agent*,” the nurse had told me), Paclitaxel, was being administered, something went wrong. Within several minutes, I felt hot and flushed, had trouble breathing, and my ears started ringing.

“I am going to faint,” I said to my friend, who alerted the nurses.

They knew exactly what to do, having called my oncologist as well to ask about which medication I should be given to counteract the allergic reaction, and at which — that is slower — speed. I felt better soon, but an extra dose of [Benadryl](#) made it tough to carry on conversations for the remainder of my inaugural trip to the “land of healing.”

In fact, had there been a quiz on any of the instructions given by the pharmacist later that morning, I would have failed it. When constipation was mentioned as a likely side effect, I pointed out that I had [Metamucil](#) at home. “That should work,” the pharmacist said, and there was always [Senokot](#), “if it gets worse.” As a “diarrhea girl” I had never heard of this over-the-counter medication and was, therefore, confident I would not need it.

I was wrong.

In fact, I did not know at all what to expect when I returned home after my first chemotherapy treatment. Would all, some, or none of the side effects I had been told about by my oncologist and cancer survivor friends — and read about online — occur when I was “on chemo watch,” i.e., during the three weeks immediately following a treatment?

Better safe than sorry

Symptoms that are commonly associated with chemotherapy treatments, such as hair loss, nausea, and vomiting, also applied to my type of cancer.

“I have highlighted them in pink for you,” the oncology nurse had said at the follow-up appointment.

“How kind,” I thought, while the colour drained from my face.

My older sister in Germany had already pleaded with me in early September to ask someone to stay with me for the first couple of nights after the initial treatment. “I’ll be fine — my wonderful neighbour is only a text away, and she’s given me a plastic bucket already...!” I joked, much to my sister’s dismay. I promised her and everyone else who expressed their concern that I would ask for help if I needed it.

I was confident, however, that I could handle the other likely symptoms — fatigue, insomnia, mild joint pain, and a low blood count — just fine by myself. The thought of numb fingers and toes as well as changes in my hearing (“Great...! I’ll be turning into [Beethoven!](#)”) frightened my musician-self, however.

My oncologist had also warned me that chills and a fever over 38.1 degrees Celsius were signs of an infection; if ignored, they could potentially lead to death. Consequently, every time I wanted to take a painkiller (to alleviate belly pain or a headache, for example), I needed to check my temperature first.

“Do you even own a thermometer, girl?” my trusted neighbour wanted to know.

“Nope,” I admitted, unable to remember the last time I had been ill. She went to the pharmacy that afternoon and bought one for me.

“Better safe than sorry, Barb,” she said. I had to agree with her.

Overall, I was most worried about my immune system tanking after treatments, especially during the second week, as had been emphasized in the paperwork I had been given.

Most importantly, I could not afford to contract Covid-19, so I followed all the guidelines as far as hygiene and exposure to others were concerned. At the same time, I agreed with my best friend, who was receiving chemotherapy as well, that going into complete hiding was the wrong thing to do, as it would likely be detrimental to my mental health.

“Most of my patients find side effects quite manageable,” my oncologist had assured me at the follow-up appointment. “Some symptoms will get worse over time, while others will only occur after several cycles,” she said. Therefore, I was to track them every day during the first cycle (“Day 1 is the day you received chemo”) and share them with her. This would be done over the phone rather than in person, due to Covid-19 restrictions, and scheduled shortly before the next treatment.

“Sounds easy enough,” I thought.

Wrong again.

Side effects of side effects

For the first week after my first treatment, I found it difficult to figure out which of my symptoms were entirely, somewhat, or not at all caused by the chemotherapy drugs I had been given, and/or related to the medication I was to take for two days following the treatment. Moreover, I did not want to ignore or brush off potential side effects. That had happened when I experienced bumpy heartbeat episodes in August and early September, which turned out to be caused by my blood thinner medication.

The first night after receiving chemotherapy drugs for the first time was a sleepless one. Nowhere in the paperwork had it said anything about severe jetlag (“could it be the extra Benadryl?”) and hot flashes of the finest (“I am back in [menopause](#) land!”). I

concluded that it was my body's way to flush the cancer drugs out of my system in record time. To that end, I drank even more water than usual and remembered to press the toilet handle twice for two days after the treatment (who knew?).

By the next morning, a Saturday, I felt better, much to the delight of my relatives and close friends. One of them was my retired colleague from Montreal who laughed out loud about my first "experiences in the chemo chair." I promised her to take chocolates with me to my chemotherapy treatments. "Buy the good stuff, Barb," she advised. "You got it!" I replied with a smile on my face.

Little did I know that this would be our last conversation ever.

My weekend walking-buddies were thrilled that I had so much energy. Both were special colleague-friends who brought along their beloved dogs.



Resting with Winston

Winston, the [miniature Schnauzer](#), would get very excited every time his academic-advisor mom took him to see Auntie Barb ("He knows the way to your place!"). During the post-surgery recovery period, whenever we rested on a bench, Winston also could not wait to sit on my lap ("Careful, Winnie, she's got a belly scar") and consume some [Cheerios](#), his favourite treat. Freya, the [English Springer Spaniel](#), was much more interested in goose droppings, much to the dismay of her dad, a religious studies professor. Unlike Winston, Freya also did not like taking frequent breaks. I will never forget the "What is wrong with you, lady?" look on her face during our first walk together around the big lake near my house. It did not take her long to figure out that a bench inevitably meant snuggle time with dad and Auntie Barb.

What I did not share with anyone at the time was my ongoing paranoia about having to take a multitude of pills at specific times. The reason? I had forgotten that the oncology pharmacist had said to take one of the pills before 1 pm. Following the instructions on the container instead, I had taken it at supper time. Was God going to penalize this "[little sin immediately](#)," as we say in German, and make me sick? Thankfully, nothing happened. It reminded me, however, to be extra careful with timing my meals as well, so the prescription nausea medication ("take 30 minutes before you eat") could do its job.

Sudden belly pain and vaginal discomfort ailed me throughout the next day. Was the chemo the likely culprit, or was I still healing from the surgery? Or was it a combination of both? I decided to adopt a "wait and see" attitude.

Then, things took a turn for the worse (or so I thought).

I woke up the next morning, a Monday, with a nosebleed and an awful rash on my calves. Could these be side effects as well?

"No," said my oncologist when I mentioned it to her during our pre-treatment telephone call which the nurses referred to as "chemo review." The oncologist explained that these symptoms were more likely to have been caused by the air being too dry in my bedroom.

"So, turn on your humidifier," she advised.

"Consider it done," I replied.

Getting up that morning I had also felt awfully wobbly on my feet, and my knees were screaming — had I perhaps worn the wrong shoes on the weekend when walking with Winston or Freya? No, balance issues and joint pain were indeed side effects, I read, as was my sore mouth; gargling with a homemade mouthwash soon brought relief. As the day progressed, I felt increasingly like someone (or something?) had "unplugged me," as I would call it, and was steadily draining my body's energy battery — for no apparent reason.

When this inexplicable tiredness got worse the next day, I checked with my cancer survivor friends. They confirmed that I was dealing with the famous “[chemo fatigue](#),” best described as an energy war of sorts going on inside me. In fact, the nurses had given me an entire booklet with information on the topic which I found difficult to read, and not just because of the heavy content (“these sections could use some copy-editing”).

My trusted neighbour suggested — and my cancer survivor friends concurred — that I should “rally my external troops.” In other words, I was to combat my symptoms with lots of rest, gentle exercise, a balanced diet, and a cheerful, optimistic attitude. My top-hatted guide agreed.

According to my diary entries this strategy worked: two “good” days followed. In practical terms this meant that I had enough energy to do stretching exercises with my twin sister; go for a 30-minute walk each day; take my car for a short spin on the highway; and play the organ at church for half an hour for the first time after my surgery.

“Aren’t you overdoing it?” my worried relatives asked.

“You don’t understand,” I said. “This first week has taught me that a [Carpe Diem](#) way of life is the way to go,” I explained, and I meant it.

The next night I had a dream in which I acknowledged my cancer diagnosis to others. When I shared that with my best friend, she was not surprised.

“Your subconscious has finally also come to terms with your new reality,” she commented. “That’s real progress, Barb!”

Crankypation

What had evidently not sunk in yet was the fact that I did not have to be what I like to call a *stubborn hero*. The very next morning, I woke up with bad post-surgery pain which warranted an extra-strength Tylenol. It brought relief until sudden, excruciating constipation pains made me wonder what I had done wrong in a past life. “Is this chemo karma?” I wondered.

“Increase your fibre intake — drink plenty of water — exercise” were “Dr. Google’s” top three suggestions online. I had turned to him for advice because I was not ready to do anything weird (“stand on your head and count your wrinkles”) or be judged for my post-chemo eating habits (“if you eat crap, you will feel like crap”). Following his instructions carefully, I consumed every high-fibre food in my pantry, drank like a fish, and even went on two walks instead of my usual one.

Sadly, my bowels were not moved at all by my activities. When the picture of a huge whale which had mistakenly swallowed a year’s supply of soaked beans appeared in my mind’s eye, I was not amused.

“Have you considered taking Metamucil?” my twin sister asked when I started whining to her during our daily stretching routine.

“I already did, first thing in the morning,” I replied, but otherwise kept my mouth shut. I was not going to admit how frightened I had been when my ears would not stop ringing for what seemed like 15 minutes straight after lunch. “Is this what Beethoven had gone through?” I wondered, feeling closer than ever to this musical genius. He had been stricken with permanent hearing loss and even written a suicide note of sorts at age 32 because of it. Thankfully, he did not give up on life and nor would I!

Nevertheless, I was at my wit’s end by supper time and considered praying to the patron saint of bowel disorders (according to the internet, it is [St. Bonaventure](#)). At that point, my trusted guide with the top hat could not contain himself any longer. “Check your diary — the solution is right there,” he said.

Of course! The pharmacist at the clinic had mentioned an over-the-counter medication, Senokot. I quickly looked it up online before heading to the drug store. It was an age-old, all-natural product that contained a stool softener. “Whatever,” I thought, horribly grumpy after having read the instructions.

“Seriously, you are going to make me wait until tomorrow for this stuff to begin working?” I complained to my guide as if he had anything to do with it.

“Patience is a virtue, Barb,” he replied. To my great relief, and that of my bowels, the medication worked — except that I could not shake the strange feeling that Senokot had transformed me overnight into an overweight rabbit which left little gifts behind at the most inopportune times. In any case, I had learned my lesson: get rid of “old @#\$\$” before it catches up with you.

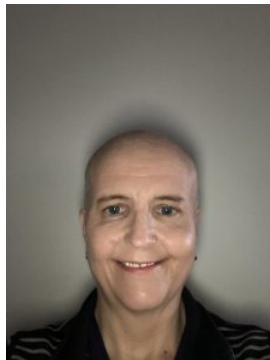
The highlight of the following week was a drive in the countryside. My friends who had helped me set up my new TV told me that there were beautiful early fall colours to be admired. “We’ll pick you up at 1:30 pm and wear masks,” they said. Thinking ahead, I

brought a comfy pillow with me to put over my belly to protect my scar. I was not prepared to sit in the car for over an hour, however, and even less to take a bumpy shortcut gravel road to get to the other side of the [Qu'Appelle Valley](#). To distract myself, I kept commenting on the rugged landscape ("I love the yellow and red leaves on the trees") and my new TV ("I have been overdosing on [Chopped](#) because the *Food Channel* is free in October"). I finally felt better when we walked alongside a creek for 30 minutes ("I can finally breathe," I remember thinking) before heading back into the city. My favourite part of the outing was the end — enjoying a burger and fries from a drive-through at a picnic table. Life was good!

Gains and losses

The next weekend was [Canadian Thanksgiving](#), which is celebrated on the second Monday in October. Under normal circumstances, I would have thanked the universe for looking after me while playing for a church service, and then marked assignments and possibly taught an aquafit class at the pool. This year, my trusted neighbour treated me to a festive Thanksgiving meal (roasted Brussels sprouts with walnuts, yum!). I also watched my very first Thanksgiving service online and cried when my church community told me in the virtual coffee hour afterwards how much they had missed me — the feeling was mutual.

I shed even more tears while counting the many blessings I had received ever since my surgery. Most importantly, I had gained a new appreciation of how many friends considered me part of their extended family. Every single ride to the hospital and grocery shopping trip, every fun walk in the neighbourhood, and every container with food prepared for me made me feel beyond cherished, as did the online concert tickets and the hand-crocheted prayer shawl I had been gifted as well. Life was good!



Bad hair day or chemo?

I was not at all thankful, however, for the gift of hair loss. "What evil crap is in these chemo drugs to make me shed like a dog preparing for winter?" I asked myself every time I found more hair in the sink and on the couch that weekend.

It started to dawn on me why so many women invested in a quality hair piece early on in their health journey. It helps with mirror shock and prevents awkward questions that some folks will ask jokingly ("Bad hair day or chemo?"). Nevertheless, I was going to have my hairdresser shave it all off. That meant not only wearing turbans and scarves in front of others from now on, but also acknowledging, or, more precisely, no longer hiding or even denying that something sinister was going on in my life.

A meditative walk brought much-needed clarity as to how I was going to deal with channelling my inner "[Professor X](#)" (or [Captain Picard](#), if not [Telly Savalas](#); take your pick) on the outside. My strategy was to be like a strong, grounded tree which sheds its leaves in the fall and regrows them in the spring. It was tougher than expected, however, to watch body hair disappear one by one over the course of the next three months. Granted, I did not miss my mustache and goatee ("thank you, menopause") at all.

But I grieved for every single eyelash and — wait for it — all the hairs inside my nose. It is the weirdest feeling when the latter starts running like a waterfall without warning, especially during or after outdoor walks in the cold (which is the case for six months of the year where I live). As a result, I now carry tissues in every jacket and purse.

I'll be missing you

Offering my body hair to the chemo gods as a sacrifice for continued good health was, however, not the worst part of the Thanksgiving long weekend.

"Hi, Barb. I am calling from Montreal to let you know that my mom is in palliative care and will pass away sometime today." It was still early on Thanksgiving Monday.

“She would want you to know that before I post something on Facebook because the two of you were so close,” said my friend’s son.

I had been increasingly worried when she had failed to reply to my texts. I was shocked to hear that she was taken to hospital the day after we had last talked.

“This cannot be happening! She was cancer-free for over a decade,” I remember thinking.

“Do you know how proud your mom has been of you?” I asked her son.

“Not really,” he answered. I proceeded to share many of the wonderful things she had told me about him over the years.

“To hear this while she is still alive means so much to me, Barb,” he replied, crying.

“I am going to keep you and your dad in my prayers,” I told him before I said goodbye, beside myself. I spent the rest of the day reliving favourite memories, including how she helped me overcome a fear of – wait for it – taking my vehicle through the car wash (*autoplénophobia*). “Just pretend you are watching a snowstorm through the windows,” she advised, sitting in the passenger seat and holding my hand the entire time. Then I contemplated what life without my wonderful friend, a fellow music historian and professional clarinetist, would look like.

She passed away that evening. Having shared the devastating news with my family, friends, and colleagues, I could not stop sobbing while listening to her favourite piece of music, [Mozart’s clarinet concerto](#). “It mimics life,” she had once said to me, “because it is fiendishly difficult to play, yet you must make it sound effortless.”

Would a relative of mine have to call my closest friends a decade from now, if not earlier, and deliver a similar message while listening to my favourite piece by “Mr. Fasch”?

Fortunately, I knew that in times of crises like these, I could always turn to my amazing, long-time clergy friend from Alberta for comfort. She reminded me that there was a time for every season, including death. “Welcome the grief,” she said. “It will come in waves; just ride them out the best you can.”

A week later, my oncology nurse phoned.

“We just got the results of your pre-treatment blood work back, Barb.”

“Are they okay?”

“Your white blood cell count is really low, which means that your next treatment may have to be rescheduled.”

I was stunned.

“Did I do anything wrong?” (Maybe I had overdosed on Nutella?)

“No, it’s a side effect, Barb.”

Then, it hit me. This was what my oncologist had meant by “your body will decide,” rather than the calendar. To my amazement, she contacted me the next day and decided to gamble. “I am confident that your blood count will be fine by Friday,” the oncologist said. I was to come in a half hour earlier, repeat the blood work, and then hopefully stay for the day.

I cannot tell you how much I prayed for my second trip to the “land of healing” not to be delayed. As far as I was concerned, the sooner this journey was going to be over, the better, even if the first round had been manageable overall.

Was my body going to cooperate?

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7: “Could it be back?” — Chemo, Round 2

Judging from the ominous title of this chapter you may think that the answer to the question I posed above is a resounding “no.”

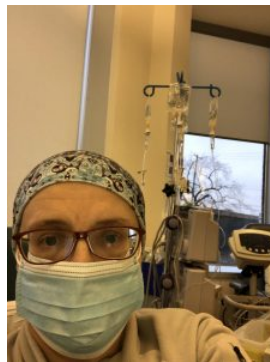
I am happy to report that my white blood cell count had indeed improved to the point that I received chemotherapy for the second time on Friday, October 16, 2020, exactly three weeks after my first cycle.

How did it differ from the first treatment? I had to take three steroid pills the night before to help prevent another allergic reaction; the oncologist’s prediction that I was going to be up all night as a result did not come true. Unlike before, I could not bring a support person with me, courtesy of Covid-19 restrictions.

I was not really going to the cancer clinic by myself, however. I had requested a special “chemo angel” (or [guide](#)) to join my own as well as the hosts of other angels that had been sent by my clergy friend specifically; only I could visualize and feel these ethereal beings around me. When the nurses announced about one hour into the treatment that I would be given the first of my two chemotherapy drugs, I distinctly remember feeling my main guide — the one with the top hat — tap me on the shoulder and angel wings surrounding me.

As if by magic, the newly requested guide, a female, showed up at the end of my chemo chair — and promptly made me smile. She looked remarkably like [Bellini](#)’s powerful opera character [Norma](#), the high priestess of a Druid temple, which I remembered from a recent production by the [Metropolitan Opera](#) in New York. I was waiting for her to sing her famous [Casta diva](#) aria for me, but she never did.

Beep, beep, beep



Ready for the next round?

Instead, the monitor that measured my blood pressure at regular intervals (because of my adverse reaction in round 1) went off and refused to cooperate.

“Let’s try your other arm,” said the nurse who checked on me.

“Fine,” I said.

“I cannot get a reading here either,” she replied.

“Let’s get another monitor,” her colleague decided.

“Works for me,” I replied. The new monitor disagreed, however (“beep, beep, beep”).

“I have a better one,” said yet another nurse.

“Maybe the new guide is trying to get our attention?” I wondered silently. Eventually, the frustrated staff checked my blood pressure manually; it was fine.

Since repeating such an “[old school](#)” approach every half hour or so was too time consuming (“we are really busy today,” I was told), the nurses proceeded to put the blood pressure cuff around my right calf! I will never ever forget the feeling of having it squeezed so hard that I thought I was going to pass out on the spot. Thankfully, I remembered a special mantra my clergy friend had taught me to use in tough situations. So, I recited “I will not die from *that* today” for hours, which seemed to amuse my invisible company much more than it did me.

Incidentally, the hospital chair that had been reserved for me for round 2 provided solid entertainment as well. It was an older, economy-class (or second-hand?) model with levers instead of a fancy remote control. Not only did it squeak badly, but every time I pulled the lever, I fell backwards and had to call a nurse when I wanted to get out of it to use the washroom.

“This chemo show today is reality TV of the finest,” I chuckled, expecting a camera crew and producer to appear at any moment to offer me a lucrative multi-episode deal. Sadly, they never did.

Ode to Joy

The three weeks that followed this treatment resembled, at least initially, those of round 1. Thanks to evidently very expensive steroid pills (“don’t lose them,” the nurses had said) and effective anti-nausea medication, my energy was good on the weekend. As a result, I got to spend quality time with Winston, Freya, and their human parents as we slowly walked around the big lake near my house, taking frequent breaks to admire the fall colours.

My energy dipped badly on Monday, however, and stayed low for seven days afterwards. The side effects I had experienced last time — including fatigue, sore mouth pain, insomnia, and ear noises — had all increased in severity, and I also struggled with new symptoms, specifically headaches.

“How are you doing with yours?” I asked my best friend during one of our frequent phone calls. She had been experiencing intense fatigue prior to her diagnosis and, given her medical history, was terrified that the doctors were going to send her straight to palliative care. After starting chemotherapy treatments, she had turned into [Wonder Woman](#) (her words, not mine) practically overnight, with seemingly limitless energy and comparatively few side effects. “I am so thrilled for you,” I said to her. She was not going to die!

During our cherished weekly visits, we would often watch online concerts — we both missed going to live events so much — on my new smart TV. To our delight, the couple who had helped me with setting up my new TV arranged for complimentary access to online concerts which had been recorded exclusively for a local concert series. My best friend and I were also excited that the performers had chosen music by Beethoven to commemorate the [250th anniversary of his birth](#) in 2020, an occasion which had been completely overshadowed by the pandemic.

We often pondered a multitude of philosophical issues that focused primarily on our shared past (“Remember when...?”) and the present (“Is this really happening?”). After all, thanks to a global pandemic, it was impossible to predict what our respective lives would look like two weeks or even two months from now. We were, therefore, exceedingly grateful that we could support each other in this time of crisis in a variety of meaningful ways.

There was, incidentally, one daily and non-food related activity that involved one other person and always made me feel better. If your imagination just led you astray and conjured up soft pillows, silky bedsheets, and your favourite movie star in a sexy outfit, you are on the wrong track.

Showing up is half the battle

The correct answer is, of course, [exercise](#), specifically indoors during a pandemic.

I cannot stress enough how much it helped me cope with, and manage effectively, the vicious cycle of fatigue that plagued me throughout my journey back to health. Unless I could not physically get off the couch or out of bed, I would meet with my twin sister daily online for gentle stretch routines. She had carefully tailored them to allow me to heal properly from my surgery and had taken side effects into consideration as well. Together, we worked our way up from 15 minutes in bed to an hour on an exercise mat over the course of several months.

I admit that there were days on which showing up at the appointed hour required every ounce of my willpower.

“I won’t last long today,” I remember saying to my favourite personal trainer more than once.

“No worries,” my twin sister replied. “We will just start low and go slow, and you will let me know when it is time to quit.”

“Perfect,” I said. I had learned quickly that I would always feel better physically and mentally when I moved my body, even for a short time.

In hindsight, I am not sure to whom I would have turned to for exercise if there had been no pandemic. All I could find online was a local yoga studio that offered free classes for cancer patients and those in recovery; the local cancer clinic advertised only classes for breast cancer patients. I wondered why I could find no certified cancer exercise specialists in the city where I live, given its population of over 200,000.

And even though cancer is not contagious, I know I would not have been comfortable returning to my regular fitness classes at the local YMCA or teaching aquafit at the pool.

Most importantly, the risk of catching an infection along the way would have been too great, in my opinion, even without Covid-19 keeping the entire world in check. Fortunately, my twin sister accommodated every single exercise and rescheduling wish of mine, as did my walking buddies (and their dogs), who never complained when I cancelled, often at short notice.

On Saturday, October 31, 2020, Halloween — or [Reformation Day](#), take your pick — nasty winds prevented my regular weekend stroll around the big lake near my house. That was fine with me, as I had woken to awful belly pain and spots of bright red blood in my panty liner (“not again”). The symptoms had subsided by the afternoon, which meant that I could walk over to see the couple who had helped me with setting up my new TV and enjoy an evening of live music with them, albeit online. Wearing masks and sitting two metres apart, we first watched a spooky Halloween organ concert that my twin sister, an organist and choral director by training, had helped organize, followed by a wonderful symphony orchestra concert. Life was good!

Actions and reactions

That cheerful feeling changed on Monday when I called the cancer clinic to tell them about my symptoms. “Come in tomorrow at 10 am,” I was told and successfully distracted myself for the remainder of the day by watching TV ([My Lottery Dream Home](#)) and chatting with friends (“Mom and dad send their love”).

When I walked into the clinic’s waiting room, an eerie voice in my head suddenly piped up. “What if the [cancer is back](#) — what if the cancer is back — what if the cancer is back...?”

My stern “Go away!” instructions did not prevent tears streaming down my face when a kind nurse led me to a treatment room and took my vitals.

“Are you okay?” she asked, clearly concerned.

“Not at all,” I answered truthfully. She gently patted me on the shoulder.

“Don’t worry; the doctor and her resident will be here soon.” Then she left.

They got delayed, and I ended up waiting for 45 minutes by myself on an uncomfortable stretcher. My overactive imagination unfortunately got the better of me and conjured up 1,000 different scenarios, ranging from “it’s going to be fine” (my main guide’s preferred one) to “I am going to be dead before Christmas”, courtesy of the terrified little girl inside me.

When my medical team finally entered the room, I was beside myself. The question “What if the cancer is back?” was still echoing inside my head. “Just try and relax,” the oncologist said patiently.

After a thorough exam, she concluded that there was no indication that the cancer had returned, but that a biopsy would be done to be sure. There was also no reason I could not have my third treatment on Friday. “Take it easy until then, Barb,” the oncologist said. I promised to try my very best.

When I arrived at home, I went straight to my fridge and ate everything in sight, including leftover chili, peanut butter straight from the jar, and half a bag of chips I found in my freezer.

In hindsight, I should have called someone to talk about how I, an emotional eater since childhood, felt about what had just happened. If I had, I would have hopefully admitted to being angry at the oncologist for not warning me in late September about the intense food cravings and the increased appetite that had tormented me since the first treatment. Who knew that they were typical side effects of endometrial cancer and true for ovarian and breast cancer as well?

My post-weight loss persona, or “[thin me](#),” also blamed the doctor’s “don’t diet during treatments” instructions for my current — and future — weight gain. Why could I no longer restrain myself when I shopped for groceries and/or others brought me food like I did prior to the surgery? The line *My body needs the extra calories to help me keep up my strength* had been a most welcome excuse whenever it happened.

Curiously, I seemed to be the only one worried that my clothes would soon no longer fit. “If that is your biggest problem these days, you are laughing,” my clergy friend in Alberta said perceptively. She reminded me once again to be more patient with myself. “I’ll try,” I said.

I worried in vain about having to step on a scale when I saw my GP a few days later for my annual flu shot. “Get it the day before your next treatment,” the oncologist had said, explaining that my immune system would be strong at the time. To my surprise, my GP did not agree and promptly got on the phone. “She calls the shots, so I guess I will be giving you one today, Barb,” he announced a few minutes later. I laughed out loud. Then, he asked me to roll up my sleeve. “I owe him so much,” I thought, truly impressed with this detail-oriented and caring physician.

“I’ll make sure to keep you posted on everything that is going on as far as my journey back to health is concerned,” I promised him before I left the treatment room.

“Call me anytime,” he replied. He was clearly smiling behind his surgical mask and plastic face shield. What a gem!

Heads up: I will describe some of my least favourite memories in the next few chapters. A [line](#) my favourite roommate from graduate school had often used — *I laughed; I cried; it became a part of me* — perfectly captures what I went through. In other words, I lived to tell the tale.

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8: “Expect the unexpected” – Chemo, Round 3

Have you ever had a complete change of heart seemingly overnight?

That is what happened to me on the morning of the third treatment on November 6, 2020. Initially, everything had gone according to plan. First, I listened to the pre-chemo meditation before breakfast, crying only once, and then checked the contents of a larger purse that now doubled as what I would refer to as my “[chemo bag](#).” Beverages and food? Yoga shoes and lip balm? Cell phone and charger? Check, check, and check. I was going to watch the news and rest on the sofa until it was time to meet my trusted neighbour and go to the hospital.

Freaking out

Suddenly, I could feel tears welling up inside me — and then the little girl inside me sobbed uncontrollably for almost 30 minutes. She announced to God, the universe, and my stunned guides and angels that, in contrast to last time, she did not want to sit in “that chair” for close to seven hours today. Instead, she and I would stay home.

Why? Because we had been fretting about the biopsy results almost daily, albeit only in our head. My inner child also had “Dr. Barb” convinced by now that they would come back positive, despite the oncologist’s post-examination diagnosis.

In other words, I was tired of putting up a brave front and pretending to be fine when I was anything but. Was my adult self slowly becoming unglued, or what was going on?

“I know this is hard, but you can do this, and we are all rooting for you,” said a quiet female voice with a distinct British accent.

“O dear Lord,” I thought. It had sounded just like my friend from Montreal who had recently passed away.

“Calm down, put on some music by your favourite composer, and then get in touch with your trusted neighbour; she will know what to say to make you feel better,” she instructed with authority.

I did, scaring my neighbour silly in the process as she had never seen me in such a terrible state. Neither had the nurse who took my vitals at the cancer clinic — my blood pressure was sky high.

Things began to finally shift after the big IV needle had gone into the correct arm. “It’s best to alternate each time,” the nurses taught me. My mood improved further when I found out that the dreaded blood pressure monitor was not going to be turned on today.

About 30 minutes into the treatment, I thanked my wonderful guides and angels for surrounding me once again with their loving presence for the next six hours, which I spent meditating and praying. I knew I was back to my usual cheerful self when I began teasing the nurses mercilessly about still not having fixed my hospital chair’s levers since my last visit. “I have a feeling that this round will be the easiest to date, given the awful beginning,” I said to myself on my way out.

I was wrong.

Horrible [insomnia](#) — another common, but little-known side effect during chemotherapy — plagued me for an entire week in addition to the usual symptoms, most importantly fatigue, joint pain, and a sore mouth. I had figured out by now how to deal with them, in contrast to the blood and the brown discharge that reappeared on my panty liner.

“Not again,” I moaned and called the clinic. I was told to get in touch immediately if things took a turn for the worse.

Forty-eight hours later they did: the colour of the discharge had now changed to dark grey.

“Am I dying on the inside?” I asked the nurse, close to tears.

She laughed at me.

“No, Barb; but you do have an infection. The oncologist wants you to take two types of antibiotics for seven days, starting today,” the nurse explained.

“Dr. Google” agreed. I had consulted him before heading to the pharmacy to find out more about what I had been prescribed for bacterial vaginitis. “I’ll do anything to get rid of these unwelcome guests,” I promised the universe. They eventually left, only for another part of my body to start giving me trouble.

"I cannot hear anything in my left ear," I announced to my twin sister the day after I had finished the antibiotics.

"Maybe it is just ear wax," she wondered.

"I don't think so — [hearing loss](#) is one of the side effects of the chemotherapy drugs I am being given." In a panic, I called the clinic and was told to go see my GP right away to have it checked.

"How have you been, Barb?" my doctor asked.

"Fine, except for dealing with an unpleasant infection that required antibiotics," I answered.

"I did not know," he replied.

I made a mental note of calling the cancer clinic to ensure my GP was copied on *everything* that was going on. Then he examined my ear.

"Could this hearing loss be permanent?" I asked, frightened to my core.

"No, because there is a lot of ear wax buildup," he said, much to my relief. I was to put mineral oil into my ear using a dropper for a few days and to return if it was still clogged.

"I told you so," was my twin sister's cheeky response when I passed on the diagnosis.

Six days later I went back to have the ear wax removed. What a difference! Now Beethoven's music would sound better than ever — except that I had other things on my mind that day: the oncology nurse had called earlier in the day to tell me that my [white blood count](#) was very low.

My life is a sob story

"Do you think that means that my fourth cycle on Friday won't happen?" I asked my GP. He proceeded to check my blood work results online.

"Very likely, because it takes your immune system longer each time to bounce back," he replied. I did not like that answer.

"Why did it tank so badly? Should I be even more germophobic than I already am because of Covid-19?" I wanted to know. He smiled.

"Listen to me, Barb — it is one of the side effects, and not your fault." My best bet was to wait for the oncologist's call.

"If the next round is, in fact, rescheduled, I want you to isolate and rest as much as possible until then, Barb," my GP advised.

The treatment was indeed delayed by an entire week.

If that were not reason enough to be annoyed, I had also cut my finger while preparing lunch. It would not stop bleeding, thanks to my low platelet count, yet another side effect of chemotherapy, and I soon ran out of band aids. To my horror, the "multi-coloured panty liner guests," as I had begun calling my vaginal discharge, returned that evening.

"Don't panic, Barb!" the nurse told me when I called the next morning. "The doctor will examine you a few days after the treatment to figure out what is going on."

"My life is a sob story," was my frustrated response, albeit only in my head.

When I told my sisters and close friends that I did not have to sit in a hospital chair for another seven days, they all concluded, "You must be relieved to get an extra week off."

"My immune system sure is," I replied. I was unwilling to acknowledge that I secretly enjoyed playing the "cancer victim" whose body was in control, rather than her mind.

At the same time, I had to admit that the universe was trying its best to cheer me up. When my oldest sister celebrated her birthday that week, my gift to her was to experience no side effects or pain whatsoever that day. Then, a surprise parcel arrived from Germany, courtesy of my other older sister. It contained not only my favourite [German wax ear plugs](#) which I had specifically requested, but so much more. I unpacked a beautiful tablecloth, two gorgeous scissor-cut silhouette Christmas calendars, a lovely new pair of socks knitted by my niece's mother-in-law, and even assorted [Lebkuchen](#), my favourite traditional German Christmas

cookies. My sister was happy when I called her to express my gratitude, as was my trusted neighbour with whom I shared some of the cookies.



Santa Barb

Both were amused about my new favourite piece of head gear, a cute Santa hat, which I donned enthusiastically to hide my increasingly bald head. I also loved the new Santa cat earrings that my best friend had given me as an early Christmas present.

“It’s great to see you smiling,” she said. Her comment reminded me that a positive attitude was one of the most effective mental health gifts I could give myself at any time.

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9: “Three nights on a stretcher” — Chemo, Round 4

I remember only two things about the chemotherapy treatment on Friday, December 4, 2020, [St. Barbara Day](#) — an obnoxious fellow patient and an early Christmas gift.

An elderly woman had dragged herself into the cancer clinic that morning.

“I feel awful,” she had said to the staff, who asked her to take the chair next to mine.

“The water pills my doctor gave me last time made me drop 12 pounds in the past 14 days,” she shouted. The nurses decided to check with her oncologist about whether to go ahead with the treatment. “Good idea,” I thought. Why had she had not called the clinic at the end of the first week, if not sooner? I certainly would have.

Long story, short: after 30 agonizing minutes, they sent her home. Visibly relieved, yet in a foul mood, she called her husband to ask him to come get her — and then went into a loud and lengthy rant about online banking security issues, and how she would tear a strip off her financial advisor later today.

“Seriously?” I thought. Despite realizing that she was clearly not feeling well, I still found it difficult to show empathy. Was she perhaps this nasty all the time, not just as a cancer patient? In any case, the other patients who were not happy with her rude behaviour either. We breathed a collective sigh of relief when she departed. That made me wonder whether my doctor and nurses found me difficult to deal with; if so, they had never let on.

“It takes a special kind of person to deal with oncology patients day in, day out,” I concluded and shared that observation with the nurse who was changing my medication. “I really admire what you do; thank you so much!” I said before I closed my eyes again.

Several hours later, another nurse touched my arm.

“You are almost done, Barb,” she said when I opened my eyes.

“Will my next appointment be moved because of the Christmas holidays?”

“It says here that you are to return on December 29.”

“I’ll be there.” I was truly thankful that I would not have to struggle (much) with side effects on the Christmas weekend. My immune system would also have several more days to recover from this treatment.

“Only two more rounds to go!” my family and close friends had been quick to point out when I checked in with them at home. The next two days were once again regulated by steroids and nausea medication. As a result, I had enough energy to join my two walking-buddies and their four-legged companions for our cherished weekend afternoon walks, despite cold temperatures and snow on the ground.

On Sunday night, however, things took a turn for the worse.

I experienced the most challenging hot flashes since I had finished my period in late November 2018. Literally dripping all over, I changed my pyjamas and the bedding. I also did my best to cope with crippling fatigue and terrible joint pain for the rest of the day. Was I going to feel well enough to see my oncologist the next morning for the promised check-up?

Body matters

Looking back, I should have rescheduled that appointment, even though there had been more discharge and belly pains since Friday. I felt horrible but still dragged myself in to see my oncologist. The diagnosis? I was not healing properly on the inside, most likely because the recovery time between surgery and chemotherapy had not been long enough. I had wondered about that myself.

Then, the oncologist warned me. “You could be stuck with these post-surgery symptoms until you are finished with chemotherapy.” I was to take daily [sitz baths](#) with unscented Epsom salt to help me heal.

For about a week or so afterwards, I spent quality time in my bathtub filled with lukewarm water. This made me think of all the wonderful times I had spent at the local YMCA pool and fitness centre over the years. The facility had been closed for good in late November 2020, a terrible loss as far as I was concerned. My fellow aquafit instructors and other fitness enthusiasts were an important part of my life outside of work. I missed them as much as I missed putting together and demonstrating both [on-deck and](#)

[in-the-water routines](#) that typically involved aqua weights, plastic balls, and pool noodles. When some participants asked whether I stayed up all night to dream up suitable exercises, I replied, “You bet – it beats counting sheep and marking papers!”

On the following Sunday, much to my surprise and frustration, I had another lengthy weeping episode. “I have no reason to cry,” I told myself (or, more precisely, my [inner child](#)), only to open another box with tissues to wipe off the tears. I could not explain why I was so overcome with emotion that morning. It had been a good week, thanks in part to feeling energetic enough to record some organ music for an online carolling event soon to be hosted by my church.

“I have missed playing so much,” I said out loud when putting on my [organ shoes](#) — yes, special footwear is required, in case you did not know. I sat down on the organ bench, turned on the power switch, and stepped on the organ pedals. “Oh no,” I moaned. The upper half of my left foot was completely numb. I promptly started cursing in German to cover up how terrified I felt.

Then, I remembered that the oncologist had asked me a couple of check-ups ago whether I had felt any [tingling and/or numbness](#) in my hands or feet yet. “No — should I?” was my surprised answer. These symptoms apparently affect over half the patients who are given Paclitaxel. In my case, they had set on late and would linger for months. Thankfully, what musicians refer to as “[body memory](#)” came to my rescue, and as a result my “organ feet and hands” knew exactly what to do on the king of the instruments that afternoon.

I found it difficult to focus and concentrate, however.

Brain matters

As a post-menopausal woman over 50, the concept of “[brain fog](#)” was nothing new to me (“why can I not remember my access code to the faculty printer all of a sudden?”). To keep my mental faculties sharp throughout treatments, I played [Bananagrams](#), a [Scrabble](#)-like game, as often as possible and continued to listen to a lot of Classical music, especially works written by my favourite early 18th-century composers: Fasch, Bach, Handel, [Telemann](#), and [Vivaldi](#). In case you missed it — [listening to Classical music](#) has been proven to have beneficial side effects on all ages, including improved memory, reduced stress, and better sleep.

But what exactly was [chemo brain](#), a phenomenon I had read about online? At first, I noticed only that answering texts and e-mails took much longer. Then, my excellent proofreading skills seemed to have disappeared seemingly overnight. Eventually, it became increasingly difficult for me to get through more than a paragraph at a time, regardless of literary genre and difficulty level. “So much for reading any historical romances any time soon,” I concluded regretfully. On the inside, I was starting to freak out.

Since daily news now comes in bit-sized pieces online, and my best friend always brought magazines when she came to visit, I ended up reading a lot, but much more slowly and taking many breaks. Curiously, handwriting cards and envelopes to go with Christmas presents also seemed to take me forever now.

“What is happening to me?” I wondered when words in German and English began to elude me. As a [bilingual](#) person, I am used to the occasional translation issue, but this was decidedly different. Carrying on a conversation in either language occasionally felt like trying to stir a newly opened jar of crunchy peanut butter with oil at the top. The bottom line: I could no longer rely on my brain like I did before — and that scared me.

The other reason for crying so hard on that Sunday morning was likely a philosophical one. I was remembering — or, more precisely, grieving — a future I would never have; the Danish philosopher [Søren Kierkegaard](#) had already figured this out in the 19th century. Most importantly, for the first time in over five decades, I would have to spend the Christmas holidays by myself, courtesy of Covid-19. My brain and my body agreed fully with the pandemic travel restrictions that had been put in place by the government to keep me and others safe. I also very much appreciated technology that allowed me to stay in touch.

The little girl inside me, however, was devastated that she could not leave town to be with loved ones in person.

As if by magic, Christmas cards and presents began showing up in my mailbox and at my doorstep to help me deal with this new reality. Twenty-four different tea samples, a giant poinsettia, and more chocolates, jams, and soaps than a single person would ever need in a year soon graced my kitchen table. Moreover, my niece’s mother-in-law had sent a ton of *Lebkuchen* cookies from Germany after learning from my sister how much I liked them. “Feel free to share them with others,” she told me when I called to thank her (“I will”). Another parcel arrived all the way from Newfoundland, courtesy of my MD friend and her husband. It

contained locally sourced jams, chocolates, candles, and lip balm as well, plus the softest, most gorgeous wool scarf, perfect for wintry days. Life was good!

I was overwhelmed by everyone's generosity and unsure how to reciprocate. My friends' standard reply — "You do not owe us anything, Barb, on the contrary" — signalled that I seemed to have made a difference in people's lives over the years. That, in turn, made me cry many more tears of gratitude and happily acknowledge that life was good indeed.

My nurse will tell you what to expect

"Will you come with me on December 17 to the [radiation oncologist](#)'s assessment appointment?" I had asked my trusted neighbour when a nurse called and told me that I could bring a support person with me. We waited patiently for 45 minutes that morning until a frazzled physician appeared. He skipped an apology for the delay in favour of detailing my treatment plan.

"I want you to have a CT scan prior to your last round of chemotherapy," he said. "I want to know exactly what is going on inside your body. And I also want you to have a PET scan." The Positron Emission Tomography would have to be done in a different hospital, which was 260 km north of where I lived.

"What about the radiation therapy treatments?" I asked.

"There will be 28 in total," the radiologist answered. They would take place at the local cancer clinic ("oh, good — no travel required") and would begin four weeks after I had finished chemotherapy.

"My nurse will now tell you more about what to expect," he said, and left.

The conversation that followed was bizarre and hilarious at the same time. First, I would need to undergo a so-called "[CT scan simulation](#)" procedure; no radiation patient can begin treatments without one. It would help determine, amongst other things, where three radiation beam tattoos on my belly would go.

"They are tiny and won't hurt," the nurse said.

"The 25 [external treatments](#) require a full bladder, will take about 10 to 15 minutes each, and be scheduled on consecutive weekdays," she explained further.

"What about the three [internal treatments](#)?" I asked.

"Brachytherapy sessions require a vaginal sizing appointment first. A plastic radiation device will be inserted each time," she answered. "It comes in various sizes — did you want me to show you some samples?"

"Um, sure," I replied. When she left the room, my neighbour and I promptly started laughing, since we had no idea what to expect.

The nurse returned with several pieces of clear plastic that ranged in size and shape from a certain male appendage to an extra-large tampon with an applicator.

"Um, I'll take that one," I said. (I leave it up to you to decide at which one I had pointed.)

"One of the side effects of pelvic radiation therapy is that the vaginal walls can close, which is very serious," the nurse went on to explain. Patients who had finished radiation treatments therefore needed to stretch them on a regular basis, also to facilitate pelvic exams. Specifically, I was to start using this device one week after finishing treatments three to five times a week, for three to five minutes.

"And you want me to do this for how many months?" I asked.

"Twelve," she answered.

"You are kidding me, right?" I blurted out, stunned but with a big smile on my face.

The nurse was dead-serious — unlike me, who on our drive home went straight into a stand-up comedy routine. "Who knew that at my advanced age, and as a confirmed single, I would be asked to put a decidedly unattractive sex toy of sorts to good use every other day for an entire year?"

"Too bad this thing does not come with an on-switch or will be delivered by my favourite movie boyfriend," I added.

But it was my twin sister who brought down the house when I told her about what I had promptly dubbed “Barb’s pelvic homework.”

“Is this how nuns die?” she asked with a worried tone in her voice.

“Only those nuns who received radiation therapy and failed to follow instructions,” I responded. We both laughed out loud, realizing that humour will get you through pretty much everything.

Two days later, I did not want to laugh anymore.

Saturday Night Fever

I had felt very tired after the oncologist’s visit and had gone to bed early that evening, waking up in the middle of the night to severe hot flashes and chills. The following morning, I also suffered from a bad headache that warranted a pain killer. As always, I checked my temperature first. It had risen to 37.6 degrees Celsius; typical for me is between 36.1 and 36.5 degrees Celsius. “Odd,” I thought. “I better take my temperature again in a couple of hours when I am back from my massage appointment,” I decided.

To my relief, it had gone down to 37.1, so I made myself a tuna sandwich for lunch, watched TV ([Caribbean Life](#)), and even had a little nap. I woke up at around 5 pm to severe chills, and my forehead was suddenly on fire. “Oh no,” I thought, grabbing the thermometer — it read 38.1 at 5:35 pm. “You must contact us immediately if your temperature rises above 38 degrees Celsius,” it had said in the paperwork from the cancer clinic. “Do not try to figure out why you are running a fever — just call us!”

That is exactly what I did at 5:38 pm; there was a special number given for weekend emergencies.

“Come in as soon as possible,” said the resident on call. I remembered her well from the last check-up.

“Should I bring an overnight bag?”

“Yes, Barb.”

Her reply did not come as a surprise. My Greek colleague had told me early on in my treatment journey about her two hospital stays while undergoing chemotherapy. “Be ready at a moment’s notice and expect to be there for about three days,” she had advised. My trusted neighbour immediately offered to drive me to the hospital when I called her with my news.

After showing a small card that read “I am currently receiving chemotherapy” to the triage nurse at 6:30 pm (I had been told to always carry the card with me), I was immediately led to the acute care treatment area.

“Please put a gown on and lie down on the stretcher,” the nurse said, closing the curtains around me. I followed his instructions and then grabbed my phone to get in touch with my twin sister about being in the hospital. Replying via text with a simple “OK”, she was clearly taken aback but trying to stay calm, as were our oldest sister and other close friends whom I contacted over the next half hour or so.

I found out only much later that my cheerful “Hi! I am in the hospital, but don’t worry” texts scared them all profoundly.

I, on the other hand, was calm — and oddly proud because I had jumped into action rather than questioning or, worse, ignoring explicit directions. I also had not argued with my inner guidance system, specifically my main guide with the top hat. He had already predicted during the drive to the hospital that I would have to stay there to heal properly.

“We will let the doctors decide,” I thought, while having blood drawn from both arms. A quick Covid-19 nose swab, a urine sample, and a chest x-ray followed. Then, a first-year neurologist checked me thoroughly from top to bottom. Since he apparently played the cello and his favourite composer was J. S. Bach, I figured I was in good hands.

“You are fine,” he determined 20 minutes later.

“I can go home then?” I asked.

“If all your tests come back negative, yes, you will be able to,” he replied.

He was wrong. About an hour later a nurse informed me that I was being admitted.

“So, they found something?”

“They are running another test that takes 72 hours, so we will put you in an isolation room, let the antibiotics do their job, and wait for the results.”

By the time the oncology resident on call came to see me the next morning, my temperature was normal, but I felt weak despite receiving antibiotics every six hours.

“The nurse woke me up at 4 am to change the IV bag, and I could not fall sleep because it was so loud outside,” I complained while the resident performed yet another pelvic exam.

“I know,” she responded. “I’ll do everything in my power to get you moved to a different part of the emergency department. Unfortunately, we do not have a bed for you in a ward because of Covid-19 patients.”

Code Burgundy

As a result, I spent the next two days and nights by myself in the ambulatory care unit. Was this perhaps the universe’s twisted way of sending me on an all-expenses-paid trip to the “land of healing”? Regardless of the answer, I was going to make the best of it.

On my way back from the shared washroom I asked one of the nurses what [Code Burgundy](#) stood for.

“It was first used when hospitals put sick patients in the morgue because they had run out of space elsewhere,” she said.

“Seriously?”

She laughed. “It just means that it is our job to look after the overflow patients.”

“Are there lots?”

“It is not too bad right now — you should have been here a couple of months ago when the second Covid-19 wave hit.” In other words, I was lucky to have been given my own room instead of twiddling my thumbs on a stretcher in the hallway.

“I need to get out of here as fast as humanly possible,” I decided right there and then.

To that end, I would make sure to annoy the medical staff with a “I feel fine and want to go home now” mantra, including the resident and my oncologist. They dropped by on Sunday afternoon and made me chuckle when they first admitted to each other that neither of them had been to this part of the hospital before and then realized that they had both gotten lost on their way to find my room.

“So, what’s the verdict?” I wanted to know.

“Well, the tests we ran yesterday all came back negative, Barb,” the resident said.

“That’s great,” I thought, winking invisibly at my main guide.

“In fact, that neurologist wanted me to send you home last night,” the resident reported.

“So, isn’t keeping me here overkill?” I wondered.

“No.” She smiled.

“I taught him a lesson,” the resident stated, much to the oncologist’s delight and my own. “When chemotherapy patients run a fever, we admit them, because if we don’t, they could potentially die from an infection,” she explained. The oncologist nodded in agreement.

“I will stay here as long as you want me to, ladies,” I replied.

In practical terms, that meant making do with a stretcher instead of a proper bed, coping with wildly fluctuating room temperatures, and tasting my way through decidedly non-gourmet hospital food once again.



Lunch is served!

“Pureed fruit salad for dessert? You’ve got to be kidding me,” I said to my twin sister, having sent her a picture of my less-than-appetizing supper. “I guess until I get out of here, I will have to live on antibiotics and black tea,” I joked.

My plan was flawed, however, as there was no kettle to be found anywhere in the ambulatory care unit. The nurses had to ask their colleagues in the nearby emergency department to heat water for me in their microwave.

“I am very sorry to be such a bother,” I apologized to the nurses on Monday morning when the hospital kitchen had sent coffee by mistake (I am a “tea girl”). Having been awake since 4 am when my bag of IV-antibiotics had been changed, I was in desperate need of caffeine — and still chuckling about what had happened the evening before.

I was already half-asleep with my eyes closed when I heard the night nurse fiddling with the monitor.

“Shut the @\$% up,” she said.

Stunned, I opened my eyes and looked straight at her.

“I didn’t say anything,” I replied. She immediately apologized, and we both laughed heartily.

A lot of rest to combat belly pain and long-lasting chemotherapy side effects as well as chatting with friends from as far away as New Zealand and Australia made the rest of the day go by quickly and another night on a stretcher bearable.

On Tuesday morning, December 22, 2020, I was told I could go home (“finally”), a prescription for an oral antibiotic in hand. “They ran out at the hospital,” I quipped, much to the delight of my three sisters, my MD friend, and my clergy friend from Alberta. They had anxiously been awaiting my return.

So had my GP, it turned out. He called before lunch. I promptly began to pester him with questions.

“Do you have a sense as to why did the tests all came back negative?”

“Likely because the fever was not caused by an infection. You probably caught a virus, Barb, but not Covid-19.”

“A virus? How? I have been so careful.”

“You could have picked it up anywhere and from anyone. I want you to go into absolute isolation until further notice.”

I was not impressed.

“You do not want to go back to the hospital or your chemotherapy treatments to be delayed again, do you?” he asked.

“No way,” I thought.

Much of the Christmas weekend was therefore spent on the couch. My favourite memories include watching my church’s beautiful Zoom worship service on Christmas Eve and taking my car for a spin on Christmas morning. As had become my custom, I prayed while driving. “Thank you, universe,” I said. I was grateful not only for the beautiful weather and my miraculous recovery, but also for the yummy Christmas dinners I would be served today and tomorrow, courtesy of my trusted neighbour and the couple who had helped me with setting up my new TV. On [Boxing Day](#), I video chatted with my elderly aunt and uncle, their children, and grandchildren in Germany for the first time ever. They were thrilled to see me and complimented me on my Santa hat. Life was good!

I had no idea that the worst chemotherapy round ever was just around the corner.

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10: “Feeling dizzy” — Chemo, Round 5

Have you ever been informed about something important after the fact and then had to deal with the consequences?

On Tuesday, December 29, 2020, I spent only five and a half hours in a hospital chair at the cancer clinic. I would be out of here sooner, according to the nurse.

“How come?”

“We will not be giving you that big bag of saline like before, nor the diuretic pills.”

“Why not?”

“Your medication line-up was recently reviewed by a new team of doctors, and they decided to drop them.”

“Fine.”

And fine was also how I felt for the rest of 2020. “I cannot wait for this year to be over,” I told my twin sister on New Year’s Eve Day. “I remember sitting on my balcony on July 1, 2020, thinking how tough the first six months had been because of the pandemic and predicting that the second half would be much better,” I reminisced.

“Well, we both know what happened,” she commented dryly.

“Indeed,” I said and assured her that, for a change, I would stay up until midnight to usher in the New Year in style.

“I’ll see you then,” she answered.

At 11:30 pm, when I could no longer keep my eyes open, I decided that a quick nap was in order. Of course, I should have set an alarm because I woke up at 10 minutes past midnight and then had to apologize to my twin sister who had wondered what happened to our online date. “Happy New Year — let’s hope it will be a healthy one!” I said. Then, I enjoyed the non-alcoholic peach cooler my trusted neighbour had gifted me for the occasion.

Most of New Year’s Day was spent on the couch trying not to worry about the future (too much). To help ground myself, I took the time to express my sincere thanks to every single person who had accompanied me on my cancer journey in 2020. “My goal for 2021 is to be the healthiest and most cheerful, positive person I have ever been,” I said in my message — and for added emphasis I sent along a recording of me playing an delightful arrangement of the well-known Christmas carol [Joy to the World](#) for pipe organ. They loved it!

Happy New Year

The next morning, I woke up to severe tingling in my hands, horrible numbness in both of my feet and my right thumb, excruciating joint pain, and crippling fatigue. “What a crummy way to start 2021,” I moaned, not interested in getting dressed.

When I eventually got up to have breakfast, I felt incredibly dizzy. “[Vertigo](#)? That’s a new one,” mentally adding it to my list of side effects. “Thankfully, I don’t have to go anywhere today,” I told myself, dragging myself to the couch to alternate short naps with longer naps before and after lunch.

Suddenly, there was a knock at the door. “Who might that be?” I wondered. I got up — very slowly — and put on a mask to investigate. It was a beloved colleague who had been let into the building by a neighbour.



A very special Christmas treat

“Happy New Year, Barb! I brought you one of my mom’s famous [boozy Christmas cakes](#) to celebrate how far you have come,” she said. “There’s so much alcohol in it, you will be able to nibble on it for all of 2021,” she added with a smile.

“That is so kind of you,” I replied. Then, the hallway started spinning.

“I am so sorry; today is not a good day,” I apologized to her.

“Get well soon, Barb,” she said, and waved goodbye before I closed the door.

Then, I prayed I would not faint prior to making it to the couch. “I will need to stay put here for the rest of the day,” I decided.

“Do you want me to bring up some supper?” my trusted neighbour texted a while later.

“Yes, but please use your key to let yourself in.”

“Are you all right, Barb?” she asked upon entering, and for good reason: I had never made such a request before.

“Not really,” I replied and told her about the dizzy spells.

“Take it easy and remember to eat something; it will give you strength,” she recommended. Since the food she had brought smelled heavenly, I followed her instructions, albeit an hour or so later, and went to bed soon after.

“How are you feeling today?” my twin sister asked the next morning.

“Marginally better,” I answered. Another bad vertigo attack hit after lunch, making me feel tired, weak, and frustrated. “I will have to cancel our stretching routine today,” I informed her.

“No problem,” she said. It was her standard reply when I did not feel up to joining her for even the gentlest of exercise sessions.

Another headache, loud ear noises, and body temperature issues (“hot or cold — which one is it?”) bothered me the following day; an entire week had gone by since the last treatment. Arguably, my body was taking longer than ever to bounce back. As far as I was concerned, the change in medication was the main culprit, and I was angry that my oncologist had not kept me in the loop. What else was she going to change without my knowledge next time – the patient, perhaps?

To promote the healing process in general, not just on a physical level, I added a daily meditation practice back into my life. It had been an effective tool to help quiet my chatty mind; much to my regret, I had let it slide after the pandemic hit. A quick look online led me to a free, 21-day [meditation](#) commitment online, courtesy of [Oprah Winfrey](#) and [Deepak Chopra](#). It focused on inviting abundance into one’s life and included meditations that were not too long.

“This is perfect for my ‘chemo brain’,” I remember thinking. I knew I would stick with it when I read the meditation’s “centering thought,” as Chopra calls it, to be pondered on [Day 1](#): “Today, I behold all the abundance that surrounds me.” How true and profound!

During my first outdoor walk of 2021, on January 9, I had a spiritual epiphany of sorts after posing the question “What was the best thing about 2020 for me?”

The answer was immediate — and unexpected: “I did not die!” said my heart to my head. Neither the little girl inside me nor the adult charged with taking care of her had been prepared for such an honest, “face your own mortality” reply.

It also reminded me of a dream I had in June 2020 prior to noticing symptoms. At the time, I had viewed it entirely through a pandemic lens; now, I considered it somewhat prophetic. Specifically, I had been on a plane (representing my life path?) that was going to Germany (representing my career?) when turbulences (the cancer?) made the pilot (“Dr. Barb”?) land in a long, dark [Autobahn](#)-like tunnel (the cancer treatments?) instead of the tarmac. Much to my relief, we landed safely, and I walked off the plane — fine, but visibly shaken (my recovery?).

“I feel so lucky to be alive,” I told everyone who listened, including the nurses at the hospital located near my house who got me ready for the CT scan on January 14, 2021. To my surprise, I had been quite emotional in the waiting room, recalling my appointment in late July 2020 when I still had a full head of hair, unlike now. Like clockwork, whatever little bit managed to grow back between rounds would fall out three weeks later, including my eye lashes, one by one.

Thankfully, hair was not required to play the organ, I told myself on January 16, 2021. I had enough energy that morning to record prelude music for an upcoming church service. I dedicated my repertoire choices to my mother, who had passed away on that day, 36 years before. I have always admired her and my grandmother for making the best of life despite terrible odds.



Genetic role models

In her mid-40s, my [Oma](#) had worried about the safe return of my grandfather, who had been sent to a concentration camp after speaking out against Hitler at a local pub; he was released when [the Americans freed the camp](#). The picture I have included here shows my Oma in the middle, with her sister Marie to her left and my grandfather to the right. My grandmother had also buried five of their six children, including my mom, who had taken care of my sick dad for 13 years while raising four girls. “Thank you, universe, for having these incredibly strong women pass their DNA to me and serve as wonderful role models during my formative years,” I said out loud. I could swear both were proudly winking back at me from the other side.

Ms. Piggy on steroids

I was weepy a couple of days later when it was time for blood work (“one last time,” I said to my twin sister) in preparation for my last cycle of chemotherapy. Was I afraid of the treatment being delayed again? Or was I relieved about getting to the end of the dark “chemo tunnel,” an image that, in hindsight, perfectly captured this long and arduous part of my health journey?

I suspect that my tears were related to something different entirely, however. My diet had been less than stellar in December, thanks to craving sweet and savoury treats non-stop. Many years ago, a colleague had once jokingly described me as “[Miss Piggy](#) on steroids.” My inner child promptly threw a temper tantrum the size of Saskatchewan, while “Dr. Barb” tried to shrug it off.

As a person who has struggled with her weight and body image for a long time, this thoughtless comment began to haunt me now — “I am back,” the most famous of all female Muppet characters seemed to say every time I put food into my mouth, looked in the mirror, and turned on the video camera icon on my phone or computer. Maybe the oncology nurse would judge me when I stepped on a scale tomorrow morning. Perhaps others had already commented on my chubby cheeks behind my back.

In short, I was not in good shape prior to my last chemotherapy treatment.

Out of control

A scheduled telephone call with the oncologist the day before had done little to cheer me up either. First, I begged her — successfully — to change back my medication, which I believed was to blame for me feeling beyond crappy. Then, I shared my

concerns about the PET scan which needed to take place before I started radiation a month from now and could only be done in a bigger, far away hospital.

To my frustration and likely that of many other cancer patients living on their own in Saskatchewan, neither [buses](#) nor trains were an option. This meant that I would need to either beg or hire someone outside my tiny bubble — my trusted neighbour had already declined in December — to drive over 500 km on the same day in unpredictable Canadian winter weather.

“That’s a tall order, especially during Covid-19 times,” I told the oncologist.

“You still need to go,” she insisted.

“I have also lost a lot of sleep over my ‘panty liner guests’ and continued belly pains,” I replied. “Will these symptoms worsen during six consecutive weeks of radiation treatments with multiple side effects?”

“You have done really well so far — you have no reason to be anxious, Barb,” the oncologist said.

Of course, that was exactly what happened when she told me about the results of the CT scan.

“We found a cyst on your bladder, but it is small and will likely not rupture,” she explained.

“Great,” I thought sarcastically, not at all inspired by her confidence.

Her final piece of news — she was going on maternity leave as of March 1, 2021 — freaked me out completely. “I am so happy for you,” I said to her. And I meant it: my oncologist had been wonderfully caring and kind ever since the day of my biopsy almost six months ago. But who would look after me (and my inner child) in the meantime?

“The hospital has hired a replacement,” she said.

“That is good to know.” I was trying to hide my fear of having to fend for myself until she returned to work.

“Have you joined a cancer support group yet?” was the oncologist’s next, arguably perceptive question.

“That is what the [social worker](#) has been suggesting for a while,” I replied.

“Please do it, and soon, Barb!” she said, adding, “Let’s chat again in late February to see how you are doing.”

“I’ll talk to you then,” I said before I hung up, once again in tears.

To calm myself down, I began preparing supper but became so distracted that I cut myself in my left thumb. “Oh, for God’s sake,” I blurted out when it started bleeding like crazy. My main guide watched me put on a band aid in a bit of a panic and then insisted I call my oldest sister. She was a retired high school language teacher with a life-long passion for alternative, non-invasive healing methods.

“Put pressure on it, elevate it, and then call me back in half an hour,” she instructed. Right around the 25-minute mark my thumb stopped pulsating. Five minutes later the bleeding had stopped completely.

“Wow — you are amazing; why didn’t I call you when this happened back in October?” I said to her on the phone.

“I am glad you did today,” she replied.

“Thank you from the bottom of my heart for all the healing energy you have sent me throughout this journey,” I said.

My oldest sister was only one of the many wonderful people with awesome superpowers of their own — you know who you are! — who always had my back.

“We have absolutely nothing to worry about, now and in the future,” I said to my inner child, and then we both fell asleep.

Looking back, this profound realization was also a signal that the trajectory of my story was finally about to change.

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11: “Closing rituals” — Chemo, Round 6

Have you ever had huge expectations from an event, only for them to be crushed?

That is what I felt like at the end of my last chemotherapy treatment on January 20, 2021. The day had begun in the usual fashion. I had listened — one last time — to the pre-chemo meditation (and managed to get through it without crying), eaten breakfast, and checked the contents of my bag to take along to the hospital. Once there, I stepped on the scale and took a deep breath when I saw the number. It was 15 pounds above what I had weighed in late September 2020. “Oh well,” I thought. If nothing else, my blood pressure, pulse, and body temperature were perfectly normal for which I was grateful.

“We have got a ‘business class chair’ for you today,” the nurse announced as we walked down the hall and entered, for the last time in 2021, the treatment centre part of the cancer clinic.

“That’s perfect,” I said, happy to be using a remote control instead of struggling with levers. Taking a deep breath, I took off my winter jacket, sat down, and put my feet up.

“Is it true that you get to [ring a bell](#) when you have finished chemotherapy appointments?” I asked the nurse. I had read about that tradition on the internet. Apparently, it had started in 1996 in the United States, when a navy officer brought a brass bell with them and rang it at the end for a job well done.

“I have never heard a bell on the days I have been here,” I remarked.

“It gets rung,” the nurse assured me. “Would you like to do it today?”

“If it’s not too much of a bother, yes, I would,” I replied.

“Not at all,” she said.

She began to insert the IV needle, going higher up on my arm to give the back of my hands a break; they had bruised badly the last two times. That was a mistake: my veins refused to cooperate. After four failed attempts, she tried the other arm and was immediately successful. “Finally,” I thought and closed my eyes. Surrounded by a multitude of guides and angels, I travelled to “the land of healing” one last time, albeit only in my head.

No rest for the weary

Five minutes later, my cell phone, which I had put on silent, started buzzing. It was the oncology radiologist. According to his voice mail, his nurses would call me with assorted details tomorrow. “Fine,” I thought and closed my eyes.

Ten minutes later, the phone buzzed again. My GP’s office wanted me to call back. “Will do,” I thought and shut my eyes.

Fifteen minutes later the nurse asked me for my full name and birthday date — for the 500th time since I had first taken a seat in a hospital chair — and announced that the first of two chemotherapy drugs was about to be administered. “Can’t a girl get some rest around here?” I muttered to myself. I grabbed my German ear plugs, donned a funny (and appropriate!) sleeping mask (“leave me alone”), and closed my eyes for the third time.

Two hours later, the social worker dropped by.

“I am sorry I did not get to visit last time, Barb,” she said.

“You missed all the excitement before Christmas,” I replied. Then, I told her about my hospital stay, stretcher and all.

“How are you doing today?” she asked.

“I have been quite anxious,” I admitted to her.

“The monthly [gynecological cancer support group](#) Zoom meeting is on Monday, Barb — did you want me to give you the information?” she asked.

“Yes, please,” I answered. The timing was likely not a coincidence.

Then, I closed my eyes again, intent to make myself invisible for the next few hours.

“It’s time to wake up, Barb, and take your diuretic pills,” a nurse said, gently touching my arm. “Does that mean that it will soon be time to ring the bell?” I asked, eyes wide open.

I had been pondering this closing ritual in detail for weeks now. Did it really mean that I was beating cancer, as others in the same situation had proudly announced online? I concluded that maybe trying to scare it away is more accurate in my case. After all, I had been cancer-free ever since the surgery in August of last year, and the CT scan from early January proved it.

At the same time, I had wondered whether these chemotherapy treatments would really help prolong my life. Would I ever have to repeat them, especially over the course of the next five years when the oncologist would keep a close eye on me because the recurrence rate was comparatively high?

“Give your head a shake,” my guide with the top hat had said to me whenever I conjured up another new episode of my award-winning series *Gone with the Chemo Wind* in the middle of the night. “Focus on the now, not the future.”

For whom the (chemo) bell tolls

I followed his instruction when the nurses declared, “We are ready for you, when you are, Barb.” Handing one of them my cell phone, I put a big smile on my face.



Ding Dong

“Okay, let’s do this!” I announced boldly.

Then, I rang the heck out of the medium-sized bell I had been given. The nurses cheered and clapped loudly, drawing the attention of patients nearby, who either joined in or looked annoyed. After about 20 seconds, I bowed for the nurses. “Way to go, Barb,” they shouted happily and turned their attention back to the other patients.

After grabbing my stuff, I stopped at the nurses’ station on my way out. “Thank you from the bottom of my heart for taking care of me since late September, ladies,” I said. “And I hope to never see any of you again!” I declared jokingly. Everyone chuckled and waved goodbye as I exited the clinic in tears.

“Congratulations, Barb — you did it!” my trusted neighbour said when I got into her car.

“I don’t feel like celebrating at all,” I replied.

“Did you ring the bell?”

“Yes — in fact, I had great hopes that marking this milestone with a closing ritual of sorts would help me officially cross the finish line of the ‘chemo tunnel’,” I explained. “But all I feel like now is numb and exhausted.” My inner child was nodding vigorously.

“Give it some time, Barb,” my neighbour recommended and pulled into the parking garage a short time later.

My relatives and close friends were thrilled with the “Barb ringing the chemo bell” video I had sent them. “We are so proud of you for making it this far,” they said. I thanked them for their vote of confidence. “I aim to please” was my standard, somewhat cheeky, answer.

Nevertheless, I felt a huge sense of disappointment on the evening of my last treatment. Why? Because I blamed myself for not having been more present during — that is, enjoyed, if not savoured — the act of ringing the bell and acknowledged its significance as a closing ritual. As far as I was concerned, I had wasted an important opportunity to move on to a new reality that did not involve receiving chemotherapy drugs.

In hindsight, I realized that I had desperately wanted this ritual to be more meaningful and memorable than those “Life is good” moments I had been experiencing. “Oh well,” I thought in the evening. “Expectations are there to be crushed.”

Maybe a good night’s sleep would do the trick.

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12: “Hot flashes from hell” — Recovery, Part 2

It did not. Instead of feeling relieved and cheerful to have finished a chemotherapy marathon of sorts, physical and mental exhaustion and a foul mood greeted me the next morning, much to my dismay.

So long, caffeine

My spirits were lifted, at least initially, when the phone rang. It was the same radiology nurse who had shown me the vaginal stretching devices in mid-December. She carefully walked me through what to expect before, during, and after the radiation treatments which were to start in mid-February. “Finally, I’ve got some answers,” I said to her.

“There is one more thing, Barb,” she replied. Effective immediately, I was to cease consuming any caffeinated beverages, fizzy drinks, and fruit juices and substantially increase my water intake. I was stunned; nowhere in the paperwork I had been given had it said anything about turning myself into a cold-blooded animal with fins and scales that breathes with gills.

“Not even herbal tea?”

“You can have one cup today, Barb, but until further notice it is water only.”

“But I just finished my six rounds of chemo yesterday.”

As a devoted black tea aficionado, I would now have to add [caffeine withdrawal symptoms](#) to my already long list of side effects.

“We need the chemo drugs to be flushed out of your system before radiation treatments begin, Barb,” the nurse explained. “Caffeine is hard on the bladder, which is one of the organs that will be affected by the radiation.”

“Fair enough.”

Giving up caffeine for a few weeks was a temporary sacrifice for which my body would likely thank me in the long run. I was also hopeful that my discipline as a musician and academic would make a difference. After all, I had spent many hours practising my instrument and countless more to excel as a university professor.

A statement made by Angela Merkel, Germany’s long-time chancellor, came to mind as well. Following a visit to a refugee camp in Dresden in 2015, she said, “*Wir schaffen das*,” which in English means something along the lines of “We will get through this,” or “We will get this done.” In other words, I was going to adopt a “can-do” attitude, and if I did have to cheat once or twice, the world would surely not come to a screeching halt.

Questions and Answers

Ten minutes later, the phone rang again.

“Your CT simulation appointment will take place on February 5 at 9 am and will last 90 minutes,” another radiation nurse informed me. “We are located directly below the cancer clinic, in the basement; please arrive 10 minutes early,” she requested.

“Could I please see a dietician that morning as well — I have a few questions,” I said.

“You bet,” she answered.

Her cheerful reply made me wonder whether I was not the first one to make such a request. “Dr. Barb” had found the printed information provided by the radiologist to be much less detailed compared to what health authorities in other provinces posted online.

“We will also be able to tell you more about your radiation therapy appointment schedule on that day as well,” the nurse continued. “Please keep all 28 treatment days wide open as starting times could change at short notice.”

“Not a problem,” I replied.

As per my Greek colleague’s suggestion (“This worked for me,” she had said), I had planned to ask the nurse for a favour. Could she kindly schedule all my appointments at the same time or time of day (morning or afternoon)? The nurse’s authoritative tone of voice — which sounded remarkably like “Dr. Barb,” come to think of it — signalled, however, that she would not entertain such a request.

I dreaded having to share this piece of news with my trusted neighbour. She had kindly volunteered to take me to all appointments in February and March. Would she withdraw her offer? When she assured me that she would not (“You won’t know how you will feel every day,” she said), I was grateful. Hopefully, the weather would cooperate as well.

Fifteen minutes later, the phone rang again. It was the radiologist who called about the CT scan and patiently answered all my questions.

“What about the [bladder cyst](#)?”

“I expect it to go away by itself, but the PET scan will tell us more.”

“I have been experiencing belly pains; could that be because of the cyst?”

“That’s hard to say. Call immediately if you run a fever because that could be a sign of infection.”

“I will.”

Then, I put the kettle on for some — herbal — tea to enjoy for the last time in a while.

My GP contacted me in the afternoon and asked me about the treatment. Trying to sound upbeat, I told him about the ringing the bell ritual. He was pleased but it quickly became clear that he had called for several different reasons.

“Given that the pandemic numbers keep going up, you must continue to isolate, Barb — and try to get lots of rest over the next few weeks,” he said.

“You bet,” I replied.

“I also read the oncologist’s report from Tuesday — how are you feeling today, Barb?” my GP wondered.

“I had a bad day. But I am doing much better, now that the radiologist and the nurses have told me more about the upcoming radiation treatments,” I stated.

“That’s wonderful,” he replied.

“I also plan to attend a gynecological cancer support meeting on Monday,” I informed him. According to my main guide, I would not regret it.

“That’s an excellent idea — is there anything else I can help you with today?” he wondered.

“Yes — do you have a sense yet when will I be vaccinated?” I asked.

My MD friend had encouraged me to get answers, because each Canadian province managed its [vaccination rollout schedule](#) on its own. My GP’s reply was a huge relief. I would likely be fast tracked because I was receiving cancer treatments. It was, however, anyone’s guess as to when I would receive my first shot.

“Before I let you go, I want you to know that you have been my hero ever since I came to see you last year,” I said, and thanked him profusely for everything he had done for me. “You are awesome!”

I could hear him chuckle.

“Take good care of yourself, Barb, and call me if you need anything.”

To everyone’s delight, especially my own, I experienced no vertigo whatsoever after the last chemotherapy treatment. I was, however, still fatigued, my feet and hands were tingling, and my right thumb was often numb which made it tough to write and type. Ringing noises in both ears also bugged me at times.

Those side effects were, however, not the problem.

Some Like It Hot

If you have never had hormonally-induced [hot flashes](#), I would like you to pretend right now that you have a powerful oven in your upper body. It only knows two settings — broil and off — and turns itself on randomly at the most inopportune times. Episodes can range from 30 seconds to what feels like an eternity. They will also haunt you for years, if not until the end of your days (or so I am told).

The good news is that wearing layers helps, including at night. Nevertheless, I have always hated having my glasses fog up, turning beet red, and engaging in what I have dubbed “The Barb striptease.” It typically involves taking off as many layers of clothing as fast as possible without embarrassing myself in front of others who might be watching while standing, for instance, in university lecture halls or church sanctuaries.

For a couple of nights after the last chemotherapy treatments, night sweats involving wildly fluctuating temperature changes (“where did my duvet go?”) kept me up but did not make me lose my cool. After all, I was a seasoned hot flash professional with many years of experience who had successfully dealt with, for instance, melting makeup and slippery piano keyboards. But I had never ever encountered a soaking-wet pillow.

“This was a disgusting first in the long and horrid history of hot flashes at my house,” I said to my twin sister the next morning. I was also annoyed that the second pillowcase I had put on the night before had made no difference. “But where else is body sweat supposed to go if you have no hair on your head?” we both wondered.

“I’ve got a plan,” I said to her. I would put on the cute little pink, very stylish sleeping cap my Greek colleague had loaned me and wrap a big towel around my pillow at bedtime.

Sadly, I had forgotten all about it when I went to sleep.

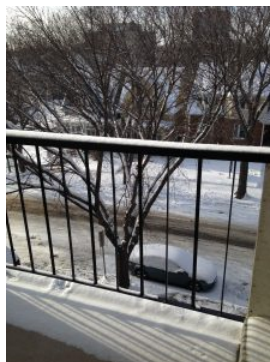
“My entire body is on fire,” I thought when I woke up at 1:45 am. Seconds later, I jumped out of bed and ripped off my night clothes in record time, only to feel horrified. My pyjamas, panties, and one-of-a-kind German wool socks were completely drenched, and my sleeping bra was so heavy with sweat that I could wring it out over the sink. There was also no question that the bedding needed to be changed — again. After toweling myself off from top to bottom in the bathroom, I went to the linen closet to grab new pillowcases and sheets when another hot flash hit.

“Oh, my Lord, this one feels like a volcano is erupting inside me!” I blurted out. I concluded that the devil himself had sent it straight from (chemo?) hell. When I could feel more hot flashes coming on that were increasing in both strength and length, I knew I needed to do something drastic to cool myself off.

However, instead of drinking cold water and/or jumping into the shower, I ran from my bedroom to my living room, opened the balcony door, and stepped outside into the snow in my bare feet.

“Ahhh,” I groaned. I was thrilled to bits about the -35 degrees Celsius winds that suddenly caressed my body. “This is heaven,” I said and let out a huge sigh of relief. It was a super-cold, but beautiful starry night, and I remember admiring the moon when I suddenly realized that I had been standing out there in my birthday suit for several minutes, if not longer.

“Well, I hope the devil and whoever else was watching enjoyed the sight,” I declared and came back inside, feeling much more like myself. Incidentally, the risk of severe frostbite never entered my mind. All I cared about was for my body temperature to return to normal, so I could get a few more hours of sleep. To that end, I changed my bedding, put on a new set of night clothes, fell into bed, and dreamed of — wait for it — polar bears on fire!



Looking for Romeo

The next morning, I realized two things. Those annoying night sweats had culminated in my very own “balcony scene” — but where was Romeo when you really needed him? “I am right here,” my top-hatted guide promptly replied with a smile. More importantly, the other side effects had been much less challenging the week after my last treatment. For obvious reasons, I had been prepared for — and imagined in gory detail — the worst post-treatment period ever. What had been different this time around,

besides having the medication changed back to what it had been before? Drinking water only, meditating and exercising daily, and getting a lot of rest.

“I wish I had known that at the beginning of this journey,” I said to my relatives and close friends, who were grateful that I was doing well.

Reality check

“Are you still planning to show up for the gynecological cancer support group Zoom meeting tonight?” my twin sister asked during our stretching routine the next day.

“Yes,” I answered. “But I have no clue what to expect.”

The social worker had said that the meeting was geared at gynecological cancer survivors. It turned out, however, that the other 22 ladies in attendance had either received an ovarian cancer diagnosis themselves in the past five years or had relatives who did.

“These are not my people,” I thought, ready to make up a phony excuse and sign off.

“Stay put,” my main guide said with a stern voice.

I sobered right up when most of the attendees shared, in a matter-of-fact tone, that they had faced at least one recurrence. “Where is my tissue box?” I wondered. I realized that my own journey, while equally valid, suddenly seemed insignificant in comparison.

As the only participant wearing a turban, I laughed out loud when one of the attendees began complaining about not being able to visit her hairdresser anytime soon because of Covid-19 restrictions. In fact, seeing every single participant with hair on her head — and lots of it — cheered me up no end. I also appreciated the suggestions made regarding follow-up appointments.

“You don’t want to fall off the doctor’s radar,” several ladies said.

“I’ll definitely keep that in mind,” I replied.

I was also wondering whether that was the real reason I needed to stay until the end of the meeting. In the past, I had successfully steered clear of medical doctors and hospitals. Now, I could not imagine my life without the ongoing advice of trusted health professionals. “I am going to be in touch with my medical team and/or GP for the rest of my life,” I promised my relatives and friends and, most importantly, my little girl and main guide — that is, myself.

A week later I told one of my favourite new colleagues, a former student, about the support group meeting. She replied that her mother had been diagnosed with the exact same type of cancer many years ago.

“Do you think your mom would be willing to chat with me?” I asked.

“You bet — here is her number,” she replied.

Our talk took about an hour and was most enlightening.

“Like you, I gained weight during chemo, but I lost it all eventually.”

“What do you remember about radiation treatments?”

“Not much because it was a long time ago; but I did drive myself there, an hour each way, on most days.”

“Did you struggle with side effects?”

“There were a few days of discomfort, but for the most part, I felt good.”

That was the answer I had secretly been hoping for. Then, my colleague’s mom shared a most unexpected piece of information: she and her own mother had been diagnosed with cancer at the same time. As a result, they had gone through chemotherapy and radiation together. Thankfully, both did well after their treatments.

“I am so happy for both of you,” I said.

“You will be just fine, Barb,” she emphasized.

I very much wanted to believe her.

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13: “Moving up” — Recovery, Part 3

Today, I embrace my potential to be, do, and have whatever I can dream.

I had meditated on that inspiring statement, courtesy of Deepak Chopra, the morning after the cancer support meeting and pondered it for the remainder of the day.

As if by magic, the term “[downsizing](#)” kept flashing through my mind each time I did. Even though I had been very happy in my two-bedroom apartment for the past 16 years, I had contemplated moving into a smaller place for a while, and for good reason. My second bedroom had morphed into a glorified multipurpose room in which to exercise, store books, and welcome my two annual overnight guests, my twin sister and the friend who took care of me after my surgery.

According to my financial advisor, if I — as a person who has always been good at stashing away money because I prefer to live below my means — invested most of what I would save each month over the course of the next decade, it would make a huge difference to my long-term retirement plans.

Specifically, I would be able to quit my university professor job before I turned 65 and could afford to live in the same, but more expensive, town as my twin sister. Encouraged by that prospect, I had even viewed an available one-bedroom flat in my building in February 2020. When the pandemic hit shortly after and I needed to teach from home, I ditched the idea. I never let it go entirely, however, and in the early summer and fall I intentionally decluttered every single closet and cupboard in my apartment.

Maybe I was ready now?

My trusted neighbour promptly argued that moving was strenuous at the best of times. Moreover, I had only just finished my last chemo treatment and needed to rest up for the upcoming radiation therapy appointments. “True,” I said.

Most importantly, my neighbour emphasized that even if I moved only within my building, I would still have to hire outside help during a pandemic. “Very true,” I admitted.

She advised, therefore, to wait unless I was unhappy with my place or could no longer afford it, neither of which was the case in late January 2021.

When I woke up the next morning, I asked the universe to give me a sign as to whether to stay put or move was the right thing to do. “It will require a minor miracle,” I concluded.

Two hours later, the caretaker knocked on my door to let me know about some boiler issue of sorts. “It’s now or never,” I said to myself and asked him about the rental situation in the building.

Less is more

“There’s a one-bedroom apartment available on the seventh floor, but it does not have a balcony,” he said. “There go my naked balcony cool downs,” I thought, but kept my mouth shut. I would also have to be prepared to surrender my underground parking spot. It was only guaranteed with a two-bedroom unit.

“Would you like me to show it to you?” the caretaker asked.



Facing east

“This is perfect,” I exclaimed upon entering.

Not only was the apartment move-in ready, but the east-facing views were outstanding. The bedroom was also big enough to double as an office of sorts.

Most importantly, the energetic vibrations of the new place were right; I felt at home immediately. My inner child agreed.

At the same time, there was a whole lot less storage space. “I’ll just get rid of more stuff I don’t need anymore,” I decided right there and then — and I meant it. I was not going to turn into my frugal mother who loved canning goods (sweet pears or sour cherries, anyone?) as much as she loved buying laundry soap on sale, only to spend hours organizing them in our dark and creepy cellar.

Many years after her death, I realized that my mom’s slight hoarding tendencies were likely related to her having lived through [World War II](#) as a teenager where [food was often scarce](#) and detergents that masked odors and musty smells considered a luxury. “When I was your age, we had milk soup for breakfast, lettuce and potatoes for lunch, and potatoes and lettuce for supper,” she would remind us, especially when my twin sister and I dared to complain about a new recipe that had caught her attention (like [Sauerkraut](#) with white beans and pineapple).

“There’s no balcony,” the caretaker said, bringing me back into the present. I knew I would do just fine without one. The cold and snowy prairie weather made it unusable for at least five months of the year, if not longer. Would I be okay with parking my car behind the building year-round? “Yes,” I thought. An underground garage had been one of the main reasons that I chose this building many years ago. However, I had always considered it a luxury, not a necessity.

“I got in touch with the rental agent, who was quite thrilled,” I said to the caretaker the next day. “I am not surprised,” he replied, a cancer survivor himself with a dad-like, if not [Dalai Lama](#), vibe. He had been wonderfully supportive over the years and even more caring since my surgery.

“You have been an excellent long-term tenant, and we would hate to lose you,” he said.

“But I will have to be out of this apartment in late March at the very latest or risk losing the new place to someone else who has also expressed interest,” I replied. That scared me because I needed to isolate while undergoing radiation therapy. Moreover, I had been warned that severe fatigue and diarrhea would be my constant companions between mid-February and mid-April.

“Since the apartment is currently empty, would you be willing to move in over the next couple of weeks before you start radiation?” the caretaker asked via text a short time later. “My team and I would move your furniture; you don’t have to even be around that day,” he offered. We agreed on Tuesday, February 9, 2021.

“What about the pieces of furniture I don’t want to take with me?” I asked him. “And I will not have the energy to clean my place from top to bottom,” I added.

“Leave that all to me, sunshine,” the caretaker said with a big smile.

“Wow,” I thought, filled with infinite gratitude.

The universe had not only sent me an unmistakable sign, but also removed all obstacles from my dream of downsizing in record time. In hindsight, this was the biggest of the small miracles that occurred during my journey back to health, as it signalled a new beginning, inside and out.

One box at a time

My sisters, while pleased to hear my news, took turns reminding me not to rush the process of going through my belongings. They were wasting their time, as far as I was concerned: my body made sure I would not forget that I still needed a considerable amount of downtime.

My trusted neighbour was surprised as well, but happy that the caretaker had come through on my behalf. “Do you want me to clean the kitchen cupboards on the weekend so you can start putting stuff in them?” she asked. “Yes, that would be wonderful; thank you so much,” I replied. Her partner, as well as the friends who had helped me with setting up my new TV, immediately volunteered to look after anything I wished to donate.

What I was not prepared for, or perhaps tried to ignore, was the fact that moving, especially in record time, would be emotionally draining. *Keep — donate — or toss?* was my daily mantra, as I filled boxes and bags with seemingly random “stuff”. I tagged them so as not to lose sight of what was to go where, with whom, and when.

“How can an individual living by herself have that many saggy pillows, old towels, and useless kitchen utensils?” I asked myself more than once during my sorting efforts. It made me think of my beloved university office, parts of which were overflowing with

books I had been given but never read, CDs I had bought but never listened to, and way too many binders with paperwork that could have been shredded years ago. Had I perhaps inherited some of my mother's hoarding tendencies after all? If so, I was more determined than ever not to let them get the better of me.

When a large box suddenly showed up at the back of my pantry, I admitted to myself that going through my earthly possessions was long overdue. I had never opened it since my move from Vancouver Island to Saskatchewan many years ago. To my genuine surprise, it contained books in German from my parents' library as well as several different types of yarn. Since I had not missed reading German translations of [Readers' Digest Condensed Books](#) from the 1970s and given up knitting in the late 1980s to focus on my academic studies, I promptly added the box to the "donate" pile. I laughed out loud when my main guide and inner child spontaneously offered a round of applause.

"My overall goal is to keep my environmental footprint as small as possible," I said to my MD friend when I told her about my plans to move within the building.

To that end, five carloads of stuff ranging from countless romance novels to a multitude of plastic clothes hangers were delivered to the local thrift store. Five big garbage bags filled with used but perfectly fine bedding, towels, clothing, and various household items went to a local organization that supported the most marginalized members of the community. Finally, five duffle bags with paper files found their way to my office at the university.

I concluded that it felt good to let go of stuff – and not just because there would be less of it to clean. Surrounding myself with fewer material possessions would allow more healing energy to flow throughout my apartment. In other words, it would help me get better faster. "I agree wholeheartedly," said my top-hatted guide, much to my delight.

I also shed many tears of gratitude when the caretaker told me that he had found a good home for the furniture I did want not to take with me.

"Your generous heart will make a huge difference in that person's life," he assured me.

"I am only paying it forward," I replied.

Three little pokes

"I have an appointment at 9 am," I said to the security officer at the hospital on Friday, February 5, 2021. His job was to screen patients receiving radiation therapy in the basement of the cancer clinic.

"Were you contacted yesterday and asked the standard Covid-19 questions?"

"Yes."

"Any changes since then?"

"No."

Then, he checked the time on his phone. "You are here too early; please leave this area and return in five minutes."

"But I was told by the nurse who called me to arrive 10 minutes before my appointment."

The look on his face clearly indicated that resistance on my part was futile. A lady who had arrived after me piped up to tell him that she had been given the exact same instructions. "I will have to have a word with the staff around here," he muttered, dead-serious but with a smile in his eyes, and waved us both through.

"Welcome to your [CT simulation appointment](#)," the nurse said when I checked in at the reception 30 seconds later. "You will have to drink 500 ml of water and then wait here for 45 minutes before the actual scan is taken," she explained.

Then, she provided more information on what my 28-day treatment schedule was going to look like. "Your first appointment will be on Wednesday, February 17," she said, handing me a printout with more information; I was to bring it with me that day. "Most importantly, don't make any firm plans between now and March 26," the nurse emphasized, and for good reason.

Approximately 100 patients were receiving radiation therapy each week ("that many?"). Consequently, only four consecutive appointments would be scheduled at a time, and appointment times would only be confirmed the day before. I would also have to stay longer once a week, when a nurse would assess me, especially once side effects had begun to occur. The vaginal sizing appointment and the three internal treatments would take place closer to the end of March.

“What about the PET scan — has it been scheduled yet?” I wondered.

“We have no control over that,” the nurse responded.

She promised to check with her colleagues at the other hospital. Then, she asked me to sit down in the waiting room — physically distanced from others, of course.

“I had asked to see a [dietician](#) this morning,” I said to the nurse who brought me a big paper cup filled with water a few minutes later.

“I am going to call her right now,” she replied.

“The nurse told you to drink water only?” the dietician asked, after sitting down across from me and listening to my questions.

“Yes — it’s been tough,” I moaned, which made her chuckle.

“You can definitely have some herbal tea,” she said, much to my delight.

Next, I asked her about foods to avoid, as the printed material I had been given was not detailed enough for my taste. “The most important thing to remember is to adjust your diet once the symptoms begin; there is nothing you need to do now,” she clarified.

“Have you ever come across patients who had no symptoms whatsoever during radiation?” I asked. The dietician laughed.

“Never,” she said.

Then, she predicted that my bowels, bladder, and vagina would begin to complain about two weeks into the treatments and continue to be unhappy for two weeks afterwards. “Yes, I read that and have stocked up on over-the-counter medication and soft toilet paper already,” I stated.

The dietician also wanted to know more about my current eating habits and provided helpful tips about food substitutions as well. “Bland is best,” she advised, and smiled when I let out a big, frustrated sigh. My mom was forced to serve flavourless meals when my dad was receiving dialysis treatments; flavour enhancers like salt and pepper were strictly off limits. Was that perhaps the real reason why I disliked boring hospital food so much?

The dietician was optimistic. “You’ll be fine,” she said and waved goodbye when yet another nurse came to get me for the CT simulation scan.

“The most exciting thing that we are going to do today is to take X-rays of your pelvic region and mark the area that is going to be radiated externally,” I was told on my way to the treatment room. A hot flash hit when I made myself comfortable on the static exam table.

“May I take off my turban?” I asked. It made me feel pretty every time I wore it.

“Of course,” the nurses said. One of them turned out to be a fellow cancer survivor which made this appointment even more special to me.

“These will be my first tattoos,” I said to her, unexpectedly excited; my oldest nephew had once told me that he always marked special occasions with body art. I was not quite sure whether the three quick pokes qualified as such — I had only felt the one that was closest to my belly scar.

“Has someone talked to you about hygiene and skin irritations during radiation therapy treatments?” the nurse wondered.

“Not yet,” I said.

“Use unscented products only and keep the area well moisturized,” she instructed.

“I will,” I replied and made a mental note to add bar soap and lotion to this week’s shopping list.

Are you ready for Tuesday?

On the weekend that preceded my move into the new apartment, I got tired of assuring my relatives and close friends that everything would go just fine. My wonderful caretaker had already taken most of the smaller pieces of furniture upstairs, including two empty bookshelves, a small L-shaped desk, and a comfy office chair which my inner academic had insisted on being taken up first.

Disassembling the desk and putting it together again turned out to be an unexpectedly complicated and time-consuming affair, much to the caretaker's surprise. Moreover, I had to admit to him after he was finished that I (or "Dr. Barb"?) had messed up: the corner part needed to come off again so as not to compete with my queen-sized bed.

"I bet you rushed through measuring it," said my trusted neighbour, making me blush.

"No, I blame my 'chemo brain'!" I quipped.

Since the part of the desk that needed to come off had shelves, I decided that it was going to double as a storage unit inside my bedroom closet. Similarly, my former bedroom dresser was now going to adorn my kitchen and house my huge tea collection plus two junk drawers with stuff I figured would come in handy sometime soon.

In hindsight, I credit my late mother — rather than the many home-renovation shows I had been watching on TV lately — for encouraging me to repurpose items in a creative and sustainable way. Her favourite way to spruce up a room was to apply a fresh coat of paint ("eggshell or off-white?") and/or hang a new, fashionable wallpaper. "You need to know how to do this kind of stuff yourselves, girls," my mother would insist, "even if you can afford to hire someone to do it for you." As a result, I also know my way around a hammer and a screwdriver.

Otherwise, I had found the past ten days of sorting and packing to be exhausting and exhilarating at the same time. Why? Because even though it was physically tiring (I quickly realized that for every hour I went through my "stuff," I would have to rest for several hours), it made me feel once again back in charge of what had been a highly purpose-driven life until my medical team took control in early August 2020.

Moving from the fourth floor up to the seventh floor six months later — that is, after finishing chemotherapy and before beginning radiation treatments — was so much more than a slight modification of my home address. My new apartment number captured, and would continue to reflect, the extent to which I had changed permanently on a physical, mental, and spiritual level ever since my cancer diagnosis.

"I am not surprised, Barb," my best friend replied when I shared this moving-related epiphany of sorts with her. "You are not the same person anymore — you are a different, better version of yourself now."

Hallelujah!

On the actual moving day, February 9, 2021, I woke up early and proceeded to check off items from my list of last-minute things to do. I continued to make good use of what had become my favourite transportation device, a big plastic shopping cart that weighed next to nothing, as well as the building elevator. At 9:30 am I informed the caretaker that I was going to rest until he and his crew were ready to get started.



Rolling out the "green carpet"

Then, I laughed — and cried — my way through 16 years of selected memories that flashed through my head as I lay on my big blue sofa with my eyes closed. They included hosting many friends and family, including sisters, nieces, and two adorable nephews from Vancouver Island, over the years; I had rolled out the red (or "green"?) carpet every time they visited.

I had also splurged occasionally on fancy appliances, including a Dyson vacuum cleaner and a Vitamix kitchen machine. And I congratulated myself on purchasing a new, ultra-comfortable mattress in mid-2019 and replacing two aging couches in early 2020 because I had spent a lot of time on them since my diagnosis.

Overall, I had been very happy in this apartment, but was ready to move on or, more precisely, up.

Invoking my favourite post-surgery mantra, *I release the past; I accept the present; I welcome the future*, was next on my agenda. It prepared me not only perfectly for expressing my deep and sincere gratitude to the universe, but also turned out to be an unexpectedly healing activity. “If only I could give every single person who has made a difference in my life to date a big thank-you hug right now,” I thought. Then, there was a knock on the door.

“Let’s get this show on the road,” I said confidently and yelled “coming,” as I got up from my couch.

“How are you enjoying your new place?” my relatives and close friends asked me after expressing their collective relief about how smoothly the move had gone.

“I love it!” I replied with a big smile on my face.

Then, I proudly showed off the fantastic views I now enjoyed from my living room and office/bedroom.

My dream of downsizing had not only become a reality, but I had also done it in record time. How? By fully embracing what Deepak Chopra had emphasized in his [daily abundance meditations](#): to expect and accept abundance in my life while remembering that attention energizes and intention transforms.

Scheduling is everything

“Hello, I am calling to tell you about your upcoming PET scan,” a female voice said on the phone the very next day. I had been wondering for a while about this medical procedure my oncology radiologist had requested in mid-December.

“Your appointment is scheduled two weeks from today, on February 24, in the afternoon.”

“Really? But I am supposed to receive radiation therapy in town that day.”

The person on the other end ignored that piece of information in favour of providing me with detailed pre-arrival instructions. Specifically, neither food nor drink except for plain water were allowed for six hours prior to the appointment. I was also to rest for two full days beforehand, which included avoiding prolonged exposure to cold temperatures and exercise.

“No problem,” I said. Minus 40 degrees Celsius temperatures did not make me want to venture outside. I was, however, sad to have to cancel my daily workouts as they gave me much-needed energy.

“You must also avoid contact with pregnant women and children for at least two hours afterwards,” she reminded me.

“No worries,” I said.

Finally, she told me to contact them if I had to cancel because I was sick or because it was unsafe to travel.

My main guide then nudged me hard to ask her whether my radiation therapy appointment in town would be rescheduled to accommodate the PET scan appointment out of town.

“When do you start treatments?” she asked.

“A week from today.”

“What? But the PET scan needs to happen first!” she stated, clearly frustrated.

“I would not know anything about that,” I said. Had there been a serious miscommunication between hospitals, and would I somehow be blamed for it?

“I am going to contact your doctor about this right now,” she announced, sounding dead-serious. “And I will only call you back if the date and times I just gave you need to be changed.”

“Fine,” I replied and hung up shortly after.

Five minutes later the phone rang again.

“Can you make next Tuesday at 8 am work?” the same PET scan scheduler asked.

“I am not sure,” I said.

I was somewhat in shock about the sudden turn of events. My main guide promptly insisted I request an appointment later in the day.

“It will take us at least two and a half hours to get there, and the weather is still supposed to be bad next week,” I argued. I also made it crystal clear to her that I would not travel the day before and stay at a hotel overnight during a pandemic.

“Understood,” the scheduler said. “How about 11 am? You would need to arrive no later than 10:30 am to check in.”

“You got it,” I answered. After hanging up, I asked the universe for a small miracle in the transportation department.

First on my list of people with flexible schedules and cars that would start on chilly mornings was the retired couple who had helped me with setting up my TV. I had already tentatively checked with them in December, but would either one be available six days from now? If nothing else, the weather app on my phone suggested favourable driving conditions on that day.

“I can take you, Barb, no problem,” said the wife and laughed when she heard me breathe a heavy sigh of relief. “In fact, I will get in touch with my daughter today about going on a socially distanced walk with her while you are busy at the hospital.”

“Do you think we can meet up with my friend who looked after me when I was recovering from surgery?” I asked; I had introduced them to each other at the time.

“It will be lovely to see her again,” was my chauffeur’s reply.

“Thank you from the bottom of my heart,” I said, close to tears.

How would I ever be able to repay her for her kindness?

It's genetic

Interestingly, my hair decided to come “back with a vengeance” (to quote my trusted neighbour) while I was resting up for the PET scan appointment. My hairdresser’s expert advice had been to keep massaging my head to stimulate hair growth. The stubbles that appeared after the second chemotherapy treatment had soon begun to itch, and I could not help but scratch them.

Thankfully, the line of organic hair products I had been using since early 2019 not only brought the desired relief, they also had been made specifically for the creator’s elderly mother, a fellow cancer survivor. Moreover, the shampoo and conditioner had made a huge difference to my hormonally challenged hair — and that of my twin sister who I talked into trying this line as well — especially during menopause.

However, neither of the products could reproduce my pre-chemotherapy hair colour of the past two decades, a subtle combination of dark blond, light brown, and copper red. “As a young professional, you do not want to look like our Oma, do you?” my sister in Germany had asked me in 2001 when I visited her prior to presenting at an international conference. She dragged me to a nearby hair salon, and after seeing the results I had to agree with her: I looked great, and there was no shame in hiding, or at least slowing down my genetic predisposition of going grey prematurely until I was ready to embrace it — whenever that would be, if ever, I had thought at the time.



My mom: not a white hair in sight!

Unlike my bad teeth and horrible eyesight, I could not blame my natural hair colour on my mother’s DNA. She had encountered so few white hairs during her 58 years of life that you could count them on both hands (see picture).

Now, to my surprise and secret delight, “Barb’s new sexy salt and pepper look” was a great hit with my relatives and close friends, and it required little to no effort on my part. Given that a polar vortex — or “[deep freeze](#)” as prairie folk like to call it — had been keeping the Canadian prairies in check for a while, I also very much welcomed an albeit short layer of hair for additional warmth.

On the long weekend before the upcoming appointment, I sorted through an old suitcase filled to the brim with old photo albums that I had retrieved from the back of my storage unit in the basement. Reliving three decades worth of memories reminded me not only that time flies, but also that it is a precious commodity. To that end, I promised myself — and my inner child — not to waste any more of our time together from now on.

Then, we figured out together which pictures were worth keeping and/or making digital copies of, and which ones should be shredded. In addition to choosing a few funny snapshots with relatives (“Were the boys really this little once?”), I opted in favour of important milestones such as graduating with a doctoral degree and becoming a Canadian citizen.

My most cherished pictures, though, showed non-relatives who had made a huge impact on my life. They ranged from supportive mentors and colleagues to special roommates, fellow students, and trusted neighbours. Several of them had passed away in the last five years, and I realized that I still missed them, some of them every day.

The night before the PET scan I also arrived at the conclusion that I felt so much lighter overall, and for good reason. I had already dropped a few pounds because my food cravings had decreased considerably since the last chemotherapy. Now I was shedding emotional weight that I had carried around with me for many years.

“Why didn’t I do this a long time ago?” I wondered. I set the alarm and went to sleep.

The bra-less life

My chauffeur-friend had brought extra pillows and a blanket, as I had requested, when she picked me up on February 16, 2021, at 7:30 am. Since I had been isolating since mid-December, we were going to be extra-careful while travelling together.

“Do you know how excited I am about today, even though it involves a hospital trip?” I mumbled into the two masks I had donned for added protection. “Thanks to multiple rounds of chemotherapy and a raging pandemic, I have not left the city since you, your hubby, and I went for a drive and walk on the Thanksgiving weekend in mid-October,” I reminded her.

Three seconds later, my phone dinged.

“I saw you leave; how are the roads?” my trusted neighbour asked.

“Not a problem; let’s hope there won’t be any highway closures to deal with,” I texted back.

After two and a half hours of solid driving at the posted 110 km/h speed limit, we arrived at the hospital. Complicated signage and a lack of knowledgeable people to ask for directions made it difficult to locate the entrance. The efficient hospital receptionist who checked me in at 10:25 am made up for it, however. She handed me a piece of paper that outlined how to find the PET scan department.

“Go past the Starbucks, turn left at the gift shop, and look for the elevator with the blue doors...,” it began. I concluded that a woman must have written these brilliant instructions and chuckled all the way as I found my way to the hospital’s basement.

“Please follow me,” a nurse said to me after she had introduced herself. “We are going to take your vitals first, then weigh you, and finally check your blood sugar,” she announced. My stomach promptly growled in response, as I had not had anything to eat for 15 hours. Then, my mind’s eye conjured up a huge stack of pancakes with maple syrup, whipped cream, and strawberries to remind me that it was, in fact, [Shrove Tuesday](#) of all days.

“Are you wearing anything with metal?” the nurse asked, rudely interrupting my food dreams as we walked to a different treatment room.

“I don’t think so,” I answered. “A bra without underwire is fine, right?”

“Yes, but if it has eyes and hooks, it needs to come off,” she insisted. It did before I took a seat in a reclining hospital chair a short while later. The nurse eventually reappeared and expertly inserted an IV needle to administer the dye.

“I’ll be back in three quarters of an hour when it’s time for your scan, which will take approximately 20 minutes — just relax until then,” she told me.

“Fine,” I thought.

I was going to have a long, nice chat with my body. “Thank you for keeping me alive all this time and cooperating so brilliantly, especially since the diagnosis,” I said to every single cell that was listening. Then, I apologized for mistakes of the past, which

pleased my heart, mind, and soul.

Finally, I made a vow of sorts — from now on, I would carefully listen to my body and respond to its needs to the best of my abilities. I concluded that I was more ready than ever for whatever the universe had in store for me and felt amazingly energetic at the end of this meaningful exercise.

A different nurse, who turned out to be the PET scan technician, opened the door.

“You are not claustrophobic, are you?”

“No.”

“It will be over before you know it.”

I got on the movable exam table and was told to put my arms above my head and hold still the entire time. “I’ll put some pillows on either side of your face to help with that,” the technician said and did just that. I closed my eyes and started reciting my favourite German prayers. My clergy friend from Alberta had suggested I give it a try, and the repetition of familiar words helped me and my inner child to relax.

“All done,” the technician said 20 minutes later. I thanked him, went back to the other treatment room to put on my bra, and left the PET scan department. Curiously, the instructions I had been given when I checked in did not work in reverse: the elevator door refused to open. “This is my life in a nutshell,” I said and took the stairs instead.

My chauffeur was already waiting for me in front of the main outside entrance.

“I am officially starving,” I announced.

“Then let’s head to the burger drive-through right away,” she replied.

While she carefully exited the hospital parking lot via a steep downhill ramp, I texted my friend on our way there. I was thrilled that she was able to meet us. She arrived right after we had finished lunch in the car — I, for one, had savoured every single bite, knowing full well that fried foods and pelvic radiation treatments did not mix well.



Smile for the camera!

“It’s so good to see you,” I said. I was devastated that it was unsafe for me to embrace her because of my compromised immune system and pandemic restrictions. We chatted for a bit about how much we had missed each other while trying to keep warm at the same time. “How about a picture in which I hug you from behind to mark the occasion?” she suggested, and we both smiled for the camera.

Then, it was time to say goodbye. “We’ve got to get home before it gets dark, Barb!” said my chauffeur. I promptly got quite emotional. When would we see each other again in person?

An hour and a half later, we stopped because the gas tank needed filling. “You have been a real trooper today and deserve a sweet treat,” I was told and rewarded with a yummy ice cream sundae. That choice of dessert may seem counterintuitive in the middle of a Canadian prairie winter. But I felt toasty warm on the inside after what had turned out to be a lovely road trip.

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14: “It’s Glow Time” — 28 Days of Radiation

Have you ever underestimated how much something would affect you because others who have been through it before you have found it to be easy?

The day after my PET scan appointment I said to my twin sister, “I cannot believe it’s [Ash Wednesday](#) today and I get to go for my first radiation therapy treatment.” If I had been a Catholic monk in the distant past, I would have spent the 40 days of [Lent](#) fasting and drinking copious amounts of beer to compensate. Instead, I continued to isolate to avoid contracting a virus that could potentially kill me and arranged my life around 28 hospital visits. Two important reminders to that end — “drink water now” and “don’t pee” — became second nature within a few days.

“How long do these treatments take?” many of my relatives and friends had asked.

“I was told 10 to 15 minutes each,” I answered.

During a recent meditation, my main guide had suggested that while receiving radiation therapy in the next five and a half weeks I reinforce my [personal energy shields](#). I had first learned about how to apply these invisible — and in my experience highly effective — protective measures when taking basic training in energy healing 15 years before. They function like cocoons of light, prevent accidental drainage, and filter out any unwanted energies as well. “That will be my waiting room activity Monday through Friday for the next five and a half weeks,” I decided.

Pull down your pants

On February 17, 2021, my reminder alarm went off at 10 am, and I guzzled down half a liter of water as instructed. At 10:30 am, I put on heavy winter gear, a mask, and my favourite blue hat and took the stairs to the second floor to meet my trusted neighbour.

“How are you feeling, Barb?” she asked as we pulled out of the parking garage a few minutes later.

“I am really tired from the trip yesterday but otherwise okay,” I said truthfully. We arrived at the hospital in plenty of time; I was to text her when I was done, as she would park nearby and wait for me.

“Thank you so much for giving me a ride!” I told her and headed to the main entrance. The Covid-19 security officer upstairs reminded me that I needed to put on a fresh mask before I would be allowed to proceed. Her counterpart downstairs waved me through because I had arrived at exactly five minutes prior to my 11 am appointment. “Well done, Barb,” my guide said.

I walked up to the desk and handed the nurse the printout I had been given at my CT simulation appointment. She told me to check in with her on my way out, as she would confirm tomorrow’s appointment and give me a schedule with the tentative times for Friday, Monday, and Tuesday. Then, I went down the hall to the waiting room, grabbed a chair, and reinforced my energy shields by envisioning a protective cocoon of golden light. “Looking good, Barb,” my guide commented. He was unexpectedly chatty, and for good reason: my inner child was nervous, and his sonorous bass voice helped relax her.

Looking around the waiting room, I noticed that there were a lot of posters on the bulletin board and information about a recent, big [cancer equipment fundraiser](#). I made a mental note to study them in detail in the coming weeks.

“Barbara?” A female introduced herself as the [radiation technician](#) who was going to look after me today. “Don’t forget to turn your chair around, so the cleaning staff knows that it needs to be disinfected,” she reminded me when I got up. I followed her and stated my full name and birthdate to verify my identity. The treatment room was just around the corner and housed a huge external [radiation therapy machine](#).

“Put your stuff on the chair, including your hat and purse; you can keep your boots on,” she said. Then, I got onto the static exam table which was covered with a flat sheet that hid head and knee rests.

Looking straight at me, the technician said, “Pull down your pants and pretend you are a sack of potatoes.” I laughed out loud and did as I was told. Clearly, I was in the best of hands.

“My colleague and I will now adjust your position until the radiation beam lines up with your tattoos,” I was told; my hands and arms were to rest on my chest.

“Hold as still as possible while the machine is doing its thing,” the technician said. “A siren-like noise will go off to signal that the radiation beam is in use. The noise is kind of annoying, but you will get used to it.”

“No worries,” I replied. I had already wondered what the weird *ding-ding* sounds and lit up *Beam on* sign were all about while waiting for my appointment outside; now I knew.

“We are going to be on the other side of the window and can see and hear you during the treatment,” the technician announced. “If there is anything that bugs you, let us know.”

“You bet,” I replied and watched them leave the room.

Then, I closed my eyes and focused on my breathing to calm myself down. As soon as the machine started to move around the table, I began praying to distract myself. When the rather ominous-sounding, siren-like noise came on, I kept reinforcing the energy shield around my body, specifically around my fully exposed pelvic region. Soon after, the technician came back in. “That went by fast,” I remember thinking.

“Good job, Barb; you are all done for today,” she announced. “Remember to put unscented lotion on your belly at least once a day.”

“Consider it done,” I replied. I got dressed and filled out a patient survey upon her request. “I will see you tomorrow,” I said and headed back to the reception desk to retrieve my updated schedule.

“It was not too scary,” I said to my trusted neighbour, who pulled up at the front entrance of the hospital a few minutes later. I texted the new appointment times to her from the backseat. “Let’s hope there won’t be too many last-minute changes,” I said, after pressing the send button. She told me not to worry and suggested I take it easy.

And I did, still exhausted both physically and emotionally from travelling the day before. In fact, I rested until it was time to get up and attend the first of six online Lenten [Bible Studies](#), led by my wonderfully bubbly pastor, a fellow extrovert whose insightful sermons I looked forward to every Sunday. Under normal circumstances, I would not have been able to participate in evening activities because the six weeks before Easter were often the busiest of the semester. But this year I could and would get involved. The topic to be explored in 2021 was “hope,” which I took as an additional sign from the universe to sign up. Specifically, we were going to “[sense](#)”, “smell”, “hear”, and “taste” hope through prayer and discussion.

“That was really fun,” I said to my pastor during the chat time afterwards. I apologized for not participating in the discussion, as my energy was still low.

“No talking is required,” she replied perceptively.

“It will be wonderful to be a part of meaningful conversations in the next five weeks that will not focus on my current health challenges,” I told her before I said goodbye and turned off the computer.

Have you ever been shot?

“Oh no — we have to go back to the hospital,” I said to my trusted neighbour the next day on our way home after my second radiation treatment. Much to my embarrassment, I had forgotten to pick up my updated appointment schedule on my way out. She smiled, turned around, and dropped me off at the main entrance. Five minutes later I apologized to the nurse at the reception.

“I am glad you came back, Barb — the radiology nurse wants to see you.”

“Now? What about?”

“I don’t know, but please go to treatment room 1 and wait for her.”

I texted my trusted neighbour about the change in plans. “I’ll wait for you, no worries,” she replied. “I hope it won’t take too long,” I said.

Five minutes later, I smiled when the nurse who had shown me the vaginal stretcher devices in mid-December came in.

“Hello Barb — we have got your PET scan results,” she informed me.

“Is it bad?” I asked, suddenly worried.

“Not at all,” she replied. “But the radiologist has ordered a couple more tests.”

A short while later, the radiologist himself arrived. “The good news is that there is no cancer anywhere,” he told me.

“That’s great,” I said and let out a big sigh of relief. I could feel my inner child jumping up and down for joy, yelling “we are going to be fine – we are going to be fine – we are going to be fine...!” My main guide nodded in agreement. “Told you so, my dear.”

“I want an [MRI scan](#) to investigate that cyst further,” the radiologist continued.

“Is that really necessary?” I wondered, feeling all “scanned out” after the PET scan adventure two days ago. Thankfully, the MRI did not require travel. It would be done at the other hospital in town sometime before mid-March.

That was not all, however.

“You also have the beginning stages of a [tonsillitis](#),” the radiologist informed me. Was my throat sore?

“No,” I said, somewhat in shock. Did it hurt when I swallowed? “No,” I replied.

“An [ear-nose-throat oncology surgeon](#) will see you next week, either right before or after a scheduled radiation treatment,” the radiologist announced.

“Wow — he’s serious,” I thought and mentally updated my list of procedures.

“My nurse will now walk you through an MRI safety questionnaire.” He wished me a good day and left.

The next ten minutes were spent on learning about what could be harmful to or interfere with the Magnetic Resonance Imaging scan I would soon be able to add to my growing list of medical procedures. It made me wonder how many my father had undergone while being treated for cancer in the 1970s. Could my dislike of hospitals — I could never be a physician or nurse — might somehow (or directly?) be related to being dragged along on what I perceived to be boring visits? Some had involved lengthy train rides as my mom did not have a driver’s licence.

The oncology nurse’s questions brought me back into the present and ranged from predictable to bizarre.

Had I ever had an MRI before?

“No.”

Did I have a pacemaker, an IV access port, or any surgical hardware such as pins, rods, screws, plates, or wires?

“No,” I answered and confirmed that I did not have any artificial joints or limbs either.

“Have you ever had an eye injury from a metal object, like shavings or slivers?” she asked. When she saw the puzzled look on my face, the nurse explained that it was quite common amongst certain trade workers.

“No.”

“What about medication patches, breast tissue expanders, and permanent makeup?” she wondered.

“No,” I replied — and laughed out loud when she wanted to know whether I had ever been injured by a bullet. “I am sure I would have remembered that,” I quipped.

“Any history of kidney or liver disease, diabetes, claustrophobia, drug or latex allergy, or dyes related to other scans?” Once again, I answered in the negative.

“Excellent, Barb,” the nurse said and told me I was free to go.

“That was truly weird,” I said to my trusted neighbour on our way home. Before she could offer a response, my little one piped up.

“No,” she said. “That was so much fun!”

Let’s not jump the gun

On the following Monday, my GP wanted to chat with me about the PET scan results. I was happy he did, as I feared I had missed or misunderstood something the radiologist had addressed. After he had walked me through the report, I complained about the MRI scan the radiologist had ordered. My GP sided with his colleague: “I like his approach — very thorough.”

“Any more questions, Barb?” my GP asked.

“Two, in fact,” I responded. “Do you have a sense of when I will be vaccinated against Covid-19?” I was tired of isolating at home and missed my friends terribly.

“Not yet,” my GP answered, but assured me that my turn would come.

Then, I asked him about returning to work. I had successfully applied last year for a research sabbatical from July to December 2021. What if I were not healthy enough by then to make the most of it? My GP immediately wondered whether I could ask for it to be moved to 2022. He was pleased to hear that my academic dean had already suggested I include that scenario in my application letter.

I was also keen to return to work sometime during the second half of 2021; if nothing else, my mental health would benefit from it. “We will talk about it after the radiation treatments are over,” my GP said. He also agreed to deal with my employer and insurance on behalf of my oncologist who would soon go on leave.

“You are awesome — thank you,” I said, grateful as ever for his continued support.

Ten minutes later, the phone rang again. “How are you, Barb?” my oncologist asked.

“Fine, but weren’t you supposed to call me the day after tomorrow?” I replied, frantically checking my calendar. The little girl inside me promptly got anxious and wanted me to hang up. Since my main guide wanted me to stay on the line, he handed her his fancy top hat to play with.

“That is correct — do you have time for me now?” the oncologist asked.

“Yes, of course,” I said and told her about several post-chemotherapy side effects that continued to ail me.

“The fatigue will continue while you are receiving radiation and for some time afterwards,” she told me. I should not get hyped up about the continuing numbness in my left foot and right thumb. “Just give it time,” the oncologist advised. She also commended me on having attended a cancer survivor support group meeting a month before.

This, in turn, reminded me to ask her what would happen after I finished treatments. “My colleague will perform a pelvic exam in late May — you will get a letter in the mail about it,” was her answer. Then she told me that she would be back at work on October 1.

“That’s fantastic news,” I exclaimed. “One last thing: my GP has suggested I move my six-month sabbatical to next year,” I said. “How do you feel about that?”

“It’s a good idea,” she said. “We have pushed you very hard ever since your surgery last August,” she admitted and insisted I take some time off: “Your body will thank you for it in the long run.”

I promised that I would delay my return to work, wished her all the best (she was going to be a wonderful mom!), and hung up.

Kopfkino

A mental image of me sitting on my favourite beach 2000 km to the west and watching the sunset surrounded by my relatives then appeared in my mind’s eye. If this had been 2019 instead of 2021, I would have booked a flight in late April and stayed with each of my sisters, who live in two of the most beautiful regions of Vancouver Island.



View of Mount Baker (USA) from Willows Beach, Victoria, B.C.

“I miss spending time with you as well as the beaches, forests, and mountain views,” I would tell them, then engage in a fair amount of [Kopfkino](#) (literally “head cinema”) to compensate. What else was I supposed to do, given that all travel of the non-essential kind was on hold due to the pandemic?

My oncologist's request to go easy on myself in the coming months was not lost on others. "Does your medical insurance in Canada cover a three-week stay at an approved medical facility after you are done with treatments?" my older sister had asked early on. This was standard procedure in Germany, and I had seen the difference it had made to her own recovery from two back surgeries and a stroke; a mutual German friend had even met her future partner while recuperating away from home after chemotherapy.

"I wish it were an option here, too," I answered. "But it could be so much worse." To date, I had spent only \$100 out of pocket at my pharmacy down the street. If I had received my cancer diagnosis in the United States, I would likely have had to take out a loan to finance my way back to health, according to my friends down south. Thank you, [Tommy Douglas](#), premier of Saskatchewan in the 1950s (and maternal grandfather to actor [Kiefer Sutherland](#)), for introducing [universal healthcare in Canada](#). I feel so lucky and privileged to have called this country home for nearly 35 years.

A special task

"I wish these hot flashes would stop," I declared at 2:30 am the next morning, February 23, 2021. After drinking some cold water and doing a bit of light reading while my body temperature returned to normal, I went back to bed and tried to fall back asleep.

"Get back up and turn your computer on right now," said my main guide, a serious tone in his voice.

"Leave me alone," I mumbled and adjusted my ear plugs.

Ten minutes later, I gave in to his request, got up, fetched my computer, and turned it on, somewhat begrudgingly.

"All right, what exactly do you want me to do?"

"Write down your cancer story, Barb." All of a sudden, I felt wide awake.

"You want me to do what?"

"Write down your cancer story, Barb," he repeated, knowing full well that I had heard and understood him perfectly the first time.

And before I could begin to ask "Why? Why me? Why now?" he started dictating a preliminary book title — "Patience required" — and 15 chapter titles to me. Forty-five minutes later I stared, somewhat incredulously, at a Table of Contents that was virtually complete. I was no stranger to what is known as [automatic or channeled writing](#). In fact, I benefitted from it as a university professor, especially when I carried out research and prepared scholarly publications. However, in this case, the time of the day (or night), speed, and content clarity had been second-to-none as far as I was concerned.

"The universe has special plans for you, Barb," my guide announced — I was to share my recollections with the explicit goal of raising funds for cancer research.

I was astonished, to say the least. "You want *me* to write a non-academic book?"

The idea was not new. For years I had cracked jokes about writing my memoirs, tentatively titled *The Accidental Academic*. When I found out that the title was already taken, and my oldest sister announced she would publish an autobiography, I dismissed the notion. "So much for making millions and retiring early!" I had jokingly concluded at the time.

What scared me now was not the act of writing itself; writing was second nature to me, a seasoned academic. I was also accustomed to seeing my name in print ever since I had entered graduate school.

So, what was the problem?

The idea that I would grant people I had never met access to my innermost thoughts — instead of directing them to intriguing archival documents and scholarly publications that examine them — made me feel decidedly uncomfortable. I had always been careful about protecting my privacy and that of my loved ones: this was the main reason I posted very little on social media platforms (and why there are no names of people given in this book, in case you had been wondering).

My main guide was quick to remind me that I would, of course, have complete creative control and could share as much or as little as I wanted. But did the world really need another, albeit inspiring, cancer survivor story?

"What do you usually say to your students when they are hesitant to give something new a try which you know will help them reach new heights?" my guide asked.

“Um — do it, or Dr. Reul will find you...?”

“No, the other one.”

“That they might just enjoy it.”

Suddenly, I felt all my fears of “going public” melt away.

It also began to dawn on me why the fundraising campaign story boards on the walls of the radiation therapy waiting room had touched me so deeply. Several community leaders — including a local financial institution as well as a rock band that involved the town mayor and a fellow academic — had raised a whopping [1.5 million Canadian dollars in 2019/20](#) to replace an aging CT scan simulator machine. Thanks to their impressive efforts, my medical team had been able to plan my radiation therapy treatments with much greater precision.

“I get it,” I thought. “The universe wants me to give back, right?” My guide smiled, took off his top hat, and bowed to me. “I will do my very best,” I vowed, turning off the computer and going back to bed.

When I woke up several hours later, I checked to make sure I had not just imagined the whole thing. It was all still there — so, why was it so hard to write the introduction?

“Don’t overdo it, Barb; you still need a lot of downtime,” my trusted neighbour, relatives, and close friends reminded me when I told them about my intentions.

“I know, but my journey of recovery has now taken on an entirely different meaning, if not a higher purpose,” I declared confidently.

My best friend also reminded me that it would be very therapeutic to tell my story my way. This, in turn, jogged my memory about something I had learned early on in my academic career: it is what you do *after* you have finished a project that counts the most.

My brother-in-law, a published author himself (and poet laureate of the [Comox Valley](#) in 2020/21), promptly offered to help with proofreading and editing my manuscript. So did a dear colleague who felt that my recollections and reflections would make a welcome and valuable addition to his English literature class that examines narratives of illness.

When I began writing my story down in earnest, however, I realized that I had no idea when my manuscript would be finished — my energy continued to be very low — and how long it might be (“keep it short and sweet”). What had become increasingly clear to me, however, was that I had been given an unexpected gift at an unexpected, but perfect, time.

Open wide

Two days later I checked in at the cancer clinic upstairs for my appointment at 2:10 pm with the ear-nose-throat doctor. Watching the clock like a hawk, I became increasingly anxious when everyone else’s name was called but mine. I had told the receptionist that the radiation therapy technologists were expecting me at 3:05 pm for an appointment at 3:10 pm.

“Oh well,” I thought. “They are probably just running late.” My inner child was not impressed, however. She promptly voiced her anger about me forgetting to bring a chocolate bar for her (“just in case”).

At 2:55 pm, I got up and talked to the receptionist: “Should I go downstairs for my treatment and come back afterwards? Or ask for either one to be rescheduled?”

“I cannot find you in the computer,” she declared, slightly frustrated. Her colleague who had taken down my name had left for the day and could not verify my story.

As if by magic, my favourite radiology cancer nurse appeared. “What are you still doing here, girl?” she asked, making me laugh. “Get yourself down there; I’ll handle things up here.”

“Bless you,” I said and ran off.

When I rushed back up there 20 minutes later, another nurse apologized profusely for making me wait.

“It’s fine,” I said. Shortly after, the ENT oncologist wanted me to know how very sorry he was for the long delay.

“It happens,” I said. Hopefully, the resident and medical student he had brought with him were paying close attention to his excellent bedside manner.

"The examination will only take five minutes," he said. Then the nurse apologized to me on behalf of the radiologist who was running late.

"No worries," I replied. With a bit of luck, it would not take him as long as it had in mid-December to join us, I figured. He appeared a few minutes later, albeit virtually, on an iPad that had been positioned on the windowsill across from my treatment chair so he could watch everything. "Don't forget to check her tonsils," said the radiologist to the ENT doctor, who tried his best to ignore him.

"You are not going to like some of the things I will do now," I was told.

"It's fine." I replied. I was not going to make a fuss: after all, I had survived six rounds of chemotherapy. To the radiologist's surprise and everyone else's relief, including my own, everything looked normal.

"It's always best to check right away, just in case symptoms return three months from now," the ENT doctor added, with the radiologist nodding in agreement.

"Thank you," I said and watched everyone leave the room except for the nurse. "Where is the closest washroom?" I asked her. My full bladder had been killing me for the past two hours!

Mount Vesuvius

Exactly 14 days after my first radiation therapy, the side effects I had been warned about began.

"Take your diarrhea medication only if you have more than three watery stools per day," a radiation therapy nurse had instructed me during the weekly assessment.

"I will," I promised.

"Have you stocked up on low fibre foods?"

"Yes, my fridge and freezer are full." Even though cooking had become a favourite activity of mine during the pandemic, it was hard to get excited about preparing what I considered boring meals. It felt odd to buy white rice that would cook in mere minutes ("I don't think I have had any in decades," I remember telling my trusted neighbour) and use high-protein bone broth powder which my pastor friend's daughter who works for a dietary supplement company had sent in the mail.

"Any fatigue yet?" the nurse asked.

"You mean *still*?" I quipped. "Exercising daily with my favourite online personal trainer has kept it at bay."

"That's wonderful, Barb," the nurse replied.

Then I proceeded to tell the nurse that, to my delight, my twin sister had decided to take additional training to assist individuals on the cancer continuum with regaining their physical strength.

"I want to be an agent of change," my twin had told me on the weekend.

"Just like me," I replied, referring to my book project. "Let me pay for the course," I offered. "It is but a small token of my continued appreciation for how much of a difference your expertise as a fitness professional has made in my life." She agreed — and then put me through my paces.

The next morning, I asked myself what I was going to eat that would not upset my bowels. Berries, nuts, and lettuce, and anything that would give me gas, such as beans, Brussels sprouts (my favourite), and coleslaw, were taboo.

"Those three yogurt containers full of home-made chili will have to wait until later," the dietician had said when she called me to find out how I was doing.

I was to peel my potatoes and apples, and swap everything that contained wheat for something that did not. "There goes my spelt and wild rice bread," I thought, with much regret. I had been completely unaware of the high amount of fibre I had added to my diet during my weight loss journey several years before.

"It's all temporary, Barb," the dietician reminded me. She wanted to know how much quality time I spent on the toilet these days.



Better safe than sorry

"I am fine in the mornings," I told her.

"That's because you have fasted at night," she noted.

"It gets worse after lunch and is terrible in the evening and at night," I reported. "And I had a really close call yesterday."

The weather had been gorgeous, which compelled me to accept an invitation to join the couple who had helped me with setting up my TV for a physically distanced walk.

After fifteen minutes of enjoying each other's company, I suddenly leaned on my walking poles and bent over. "What's going on, Barb?" they asked. Sudden belly cramps made me gasp for air.

"I need to go home — now!" I announced.

In hindsight, I should probably not have joked to them earlier on about my restricted diet ("I feel like a bunny") and frequent visits to the washroom ("I bought 60 rolls of toilet paper on sale the other day"). Walking as fast as I could, I was terrified I would soil my jeans on my way home. My friends, in turn, were scared that I would faint from the pain.

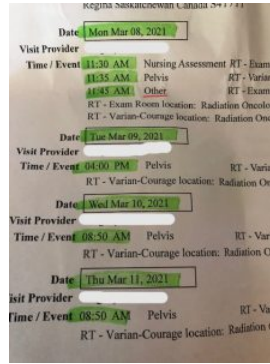
After what seemed like an eternity, I finally stood in front of the building elevator, waiting for the door to open. I concluded that I should never have moved to the seventh floor, while pressing my butt cheeks together as much as I could on the way up. After entering my apartment and pushing the door shut, I pulled down my pants and sat down on the toilet with literally two seconds to spare. "Let the volcano erupt," I winced and closed my eyes.

"Barb," said the dietician, "from now on, take one pill a day whether you need it or not, okay?" This was not a normal case of diarrhea, she argued, but caused by the radiation.

"No joke," I thought.

I checked with the nurses at the hospital, who agreed with her instructions. A dear friend who suffered from [irritable bowel syndrome](#) also drew my attention to the quick-dissolve variety of the over-the-counter diarrhea medication I was using. "Put a couple of pills in every purse you own," she advised.

"Good idea." I put some on my nightstand and inside jacket pockets as well. "Better safe than sorry," I concluded, praying daily that my bowels would soon begin to quiet down.



Radiation itinerary

“If you are free, could you come in right now instead of in an hour?” my favourite radiology nurse asked on the phone the next morning. It was March 8, 2021, the day of my vaginal sizing appointment (see “Other” on the radiation itinerary). I had not been looking forward to it even though I was told it would feel like a pelvic exam and should not hurt.

“This is not going to work for me, folks,” I said when the sterile, decidedly unsexy stretching device chosen by the radiologist caused definite discomfort. He motioned the nurse to hand him a different one.

“Better?” he asked. I nodded.

After the radiologist left the room, the nurse handed me a [dilator](#) made from heavy plastic and a tube of lubrication jelly, wrapped in a brown paper bag for privacy. I am glad she could not read my mind when I put it in my purse — my inner stand-up comedian was having a field day.

Did the device come in other colours than stark white, and was there a luxury version available that vibrated? Were there other adult gadgets kept under lock and key somewhere in this hospital that warranted close inspection? Looking down at my tent-like hospital gown, I could guess the answers.

“Some ladies like to insert the vaginal stretcher prior to intercourse,” the nurse said, looking me straight in the eye.

“Good for them,” was my sassy reply.

When I got home and read the instruction sheet, I was no longer amused. The thought of getting to know a piece of hard plastic intimately over the course of the next year did not excite me at all. There was also no way I was going to “close my eyes and think of England” several times a week, as had been suggested by a certain “Dr. Google.” The odds of my favourite movie star offering to warm my sheets anytime soon were also slim to none. Would my bedroom soon qualify as a monastic outpost?

If nothing else, my huge belly scar was healing nicely, the radiation therapists and nurses had assured me. When I mentioned it to friends in Australia and New Zealand who had both had hysterectomies as well, they jokingly suggested a “I’ll show you mine, if you show me yours” Zoom party. I cringed, wondering when I was going to be comfortable with my post-surgery abdominal “look”, if ever. I was not interested in hiding my scar with fancy lingerie or in covering it up permanently with a [tattoo](#) (as suggested by my trusted massage therapist).

The scar was also surrounded by muscles that seized up if I did (too many) twists, crunches, or planks. In other words, I could kiss a flat stomach goodbye forever. “Give it time,” my twin sister advised. “If you work *with* your body, it will surprise you.” I decided to give her the benefit of the doubt.

It was snowing hard the next morning when I left to walk to the other hospital in town at which my cancer journey had begun seven months earlier. As instructed, I checked in at 11:45 am for my MRI scan at 12:30 pm, not having had anything to eat or drink since 8:30 am. Then I waited for almost two hours, spending most of it in a freezing change room clad in a flimsy hospital gown and robe. Leaving on my hat, winter jacket, wool socks, and heavy boots helped, as did a warm blanket offered by a kind nurse who apologized for the delay. However, by the time I was taken to the MRI treatment room, I was a frozen pretzel.

“I’ve got a radiation therapy appointment at the other hospital at 3:30 pm and need to drink 500 ml one hour beforehand,” I informed the nurse when I got onto the moveable bed that was attached to a giant tube.

"I'll have it ready for you when you are finished," she promised, adding, "It's going to be loud in there." The ear plugs she had handed me next promptly fell out, only for another nurse to grab a set of over-ear headphones and put them on me while I was being pushed into the tube. Having reinforced my personal energy shields, I closed my eyes and then alternated Christian prayers in German and in English with repeating my favourite Sanskrit meditation sounds and phrases to distract myself.

"You are all done, Barb," the nurse said 45 minutes later. "I called the radiation therapy department at the other hospital and told them that you might be a couple of minutes late today. I did not want you to have to rush there," she explained.

"Thank you," I said and got dressed, relieved for it to be over. Then I gulped down a large paper cup filled with ice cold water on my way out, walked home as fast as I could through the snow, ate a quick, intentionally bland lunch, and made it to my next appointment on time.

Please cheer me up

On the following day, a Wednesday, I could not figure out what was wrong with me. I had very little energy and wept before and during the car ride to the hospital. My trusted neighbour managed to calm me (and my inner child) down, but I was still emotional when I walked into the hospital.

"I've had an awful morning," I said to the radiation therapists.

"Didn't you tell us you were a music professor?" one of them said.

"You've got a great memory." I positioned myself on the table. A few second later, rock music blasted through the speakers. "Oh no," I moaned.

"Are you more of a country girl?" the technician wondered.

"No, I like classical music."

"I will see what I can find," she said — and then the cheerful first movement of Mozart's [Eine kleine Nachtmusik](#) began playing. My mood immediately improved, and when the opening of Vivaldi's [Spring](#) conjured up birds and blue skies in my mind, I was positively beaming.

"Thank you so much for the [music therapy](#)," I said after the treatment. It was going to be an awesome day after all!

I was wrong.

Bad fatigue hit shortly after I returned home. I cancelled my daily exercise session online as a result, the first time since I had finished chemotherapy. "I hate feeling so weak," I said to my twin sister. Fluctuating body temperature issues and persistent ear noises as well as a numb left foot and right thumb did little to improve my mood the next day. "I am so sorry to hear that, but at least the colour of your face no longer matches that of your bedroom walls," my twin sister commented.

Neither of us expected me to "travel back to chemo land" — that is, experience crippling fatigue — again on the following Saturday. I spent most of it in bed, exhausted, weepy, and feeling sick to my stomach. My trusted neighbour, an excellent cook, brought me supper, which she had carefully prepared according to the list of foods allowed. It tasted great, but a short while later an awful diarrhea attack and painful cramps made me curse in German. How much longer would these side effects continue? I, for one, was ready to quit treatments ("I am cancer-free, remember?"), but my main guide would not hear of it.

"I am most concerned about your nausea," another radiation therapy nurse said to me on the following Monday during my weekly assessment.

"I did not have any while undergoing chemo," I told her, hoping for answers.

"I am not sure what's going on," she admitted. "After all, your stomach is not being radiated, only your pelvis. Are you watching your diet carefully?"

"Yes, like a hawk, but I'm not really hungry these days."

"You have to eat, Barb!" the nurse reminded me. "Take the nausea pills left over from your last round of chemotherapy."

"You bet," I replied, congratulating myself on not throwing them out or, worse, losing them during my move in early February. I had been absolutely mortified when a big folder with important documents had disappeared. I had been so careful (or so I thought).

“Do you have any idea how much I have fretted over this?”, I asked my clergy friend, frustrated to the core.

“It happens,” she said.

“Not to me,” I replied. “Dr. Barb” was nodding vigorously.

“Surely, you would have made virtual copies of these files?”

“Yes, as a matter of fact.”

My clergy friend laughed out loud. “So, what has the universe been trying to teach you?”

“Um — that being human means we mess up occasionally?”

“You got it, Barb.”

In hindsight, I realized that the missing documents were all related to my long-term retirement plans. They also represented my distant future — no wonder I had been so distressed.

When my GP called the next day, he had excellent news for me. “You will receive a letter from the provincial health authority in the mail shortly that allows you to jump the vaccination cue,” he said. I promised to make a vaccine appointment as soon as it arrived or call him if it did not arrive within three days.

“Since I’ve got you on the phone — nobody has talked to me about the MRI scan,” I said to him. “Would you mind taking a look?”

“A report has not been submitted yet,” he said after checking online.

Then he became very serious. “Barb, given that the [Covid-19 numbers have skyrocketed](#) in town and will likely continue to climb, I want you to walk by yourself and not enter a supermarket for the next two to three weeks.” I was not surprised, and neither were my trusted neighbour and the couple who had helped me with setting up my TV — they immediately offered their assistance with getting groceries.

“Just tell us what you need, and we will get it for you,” they said, much to my delight and relief.

That evening I threw up.

“That’s not good,” said my GP when he called the next morning. “Keep taking the medication,” he advised.

“What about the MRI scan?”

“There is nothing new to see.”

“Not even the bladder cyst?”

“It is now described as a cystic area at the front of your pelvis,” he clarified and told me not to worry about it.

“Fair enough,” I said. Then, I thanked him for getting back to me so quickly and hung up. Fifteen minutes later, I had convinced myself that my medical team had probably overlooked something after all. My main guide promptly threatened to slap me with his top hat. This, in turn, amused my little girl and even made “Dr. Barb” laugh. Yup, never a dull moment in my head!

Glowing on the inside

A couple of hours later, I went through the two Covid-19 screening points at the hospital. Both security officers recognized me by now, even with a mask on. The female liked my blue hat: “It really suits you,” she said more than once. The male appreciated my punctuality: “Go right ahead,” he said every day.

Neither of them was aware that on that day, March 18, 2021, was my third and last internal — that is, vaginal — radiation appointment. It involved a different set of nurses, who wondered whether the two previous treatments had caused any side effects. Still reeling from my recent vomiting episode, I entertained them with a hilarious “maybe I am allergic to meatballs?” routine. Then I changed into a hospital gown and robe to hide the fact that I was naked from the waist down, walked across the hall, and used the washroom to empty my bladder.

“You know the drill by now, Barb,” one of the nurses said. I got on the table, opened my legs, and took a deep breath when a vaginal stretcher, identical in size and shape to the one I had been given, was inserted. Unlike mine, it was attached to a radiation monitor via a long cord.

"Pardon me, is that a [Geiger counter](#) you are using?" I asked the [medical radiation physicist](#) on duty who had joined the nurses that day.

"Yes," she said, explaining that it detects the presence of radiation inside my body. "It's on," she declared, and then everyone left the room for four minutes.

I was trying to get through my usual prayer routine but was distracted by horrible rock music that someone had forgotten to turn off. "Oh well," I thought. Was that my pelvis glowing on the inside?

"Here's some lube for you," I was told when I hopped off the table. When the three nurses detailed the vaginal stretching routine that I was to adopt for the next year, I felt like a student in class whose professor was worried that they would forget or, worse, refuse to follow explicit instructions. It was critical to start exactly one week after I had finished treatments, the nurses emphasized.

"That's on [Good Friday](#), ladies," I said. "But I am not sure how good of a Friday it will be," I quipped, making them chuckle.

"You can always substitute intercourse," one of the nurses suggested.

"Your wish is my command," I deadpanned, likely making the physicist wonder what the loud laughter around the corner was all about.

"By the way, I only have six external treatments left. What happens after that?" I asked.

"For at least two weeks afterwards, be prepared for the side effects to get a whole lot worse." The nurses smiled when I let out a frustrated moan and rolled my eyes.

"Will the radiologist want to see me again?" I wondered.

"The computer says you have an appointment on May 26. Watch out for a letter in the mail," one of the nurses said.

"I will." I thanked them and went across the hall to change back into my regular clothes.

The end is in sight



For "mom" from Barb!

"I have two questions for you," I said to my trusted neighbour during the drive home. "Can you think of something I could give you as a thank you for all your support lately, especially for chauffeuring me back and forth to the hospital five times a week?"

"I'll let you know," she answered. Having her choose her own presents had worked well at Christmas time. She had also appreciated the small gifts and self-made cards I had brought with me to the car or dropped off at her place to express my continued gratitude. Ultimately, she chose a beautiful picture of a vase with poppies by an artist she admired.

"You have excellent taste," I said.

"I know," she replied, and we both smiled.

I had also decided that I would drive myself to the hospital from now on. There were only six appointments left, and my neighbour's first Covid-19 vaccination was scheduled the week after I finished treatments. My daily hospital visits posed an additional, avoidable risk, I argued. She immediately agreed, visibly relieved.

"Let's hope all of the appointments will be in the morning, when I have the most energy," I said to her. Nearly half had been changed at short notice, some of them to the opposite time of day.

"If you are not feeling up to taking your car, I will drive you," my trusted neighbour decided.

"Thank you," I said, praying for good weather as well.

The promised vaccination letter was waiting for me when I got home. The local drive-through, for which many of my older friends had lined up as soon as they were eligible, was not an option for patients in the critically vulnerable category like me and my best friend, who had received her letter as well. Twenty minutes later, I received a call-back, and five minutes later I entered "vaccination on April 8, 2:40 pm, University" into my calendars. "What a relief," I thought; our city had been a Canadian hotspot for weeks. Receiving my first shot almost two weeks after having finished radiation therapy would also give my immune system time to recuperate.

"Do you have a date for your second dose yet?" my German friends and relatives wondered when I told them about my upcoming vaccination.

"No, but it will happen this year sometime before August 8," I responded. The Canadian government had implemented a much longer [waiting period between vaccinations](#) than European countries.

"I wish it would stop snowing," I said, while driving myself to the hospital during the last week of treatments. On Monday, I ended up spending an extra ten minutes in the car because I had left too early but did not want to wait in the hospital, for obvious reasons. On Tuesday, I started crying in the car because I was missing the pre- and post-appointment chats with my trusted neighbour, who still texted twice a day to make sure I was doing okay. On Wednesday, I compensated for feeling lonely at home by carefully sucking the chocolate off a pound of almonds before throwing them into the trash.

My bowels were not amused and reminded me repeatedly that, besides chicken, fish, and turkey, my safest food choices were bananas, canned peaches, oatmeal, white rice, butternut squash — and boiled carrots. "Will I turn into the Easter bunny soon?" I wondered on my drive home from the hospital on Thursday.

To my great surprise, my trusted neighbour showed up at my door that evening. She handed me not only homemade pumpkin muffins but also a lovely "Congrats! You're done" card. When I read that she had been happy to support me on my journey to even better days, I promptly burst into tears of gratitude. Then, I admitted how much I missed talking to her every day. "Why didn't you say something, girl?" she asked and immediately agreed to drive me to the hospital the following morning.

One last time

"Hi there," I said to the nurse at the reception desk on Friday, March 26, 2021. "I have a thank you card for you and your colleagues, as today is my last treatment — would you please pass it around?"

"How kind of you," she replied and put it on her desk.

Then I waited to be picked up by the radiation therapist down the hall. I handed her the other card I had prepared before taking my jacket off. Her astonishment made me suspect that she and her colleagues had not expected to be thanked for what they did every day. Earlier that week I had asked one of her colleagues about the type of training that was required. To my surprise, I found out that they were not nurses, but a similarly close-knit professional community.

"We are ready for you, Barb," the other radiation therapist said. I got on the table and pulled down my underwear and faux fur lined leggings. "These are really cool pants," I was told.

"Thank you; they are my favourites." I watched the therapists leave the room. Then I psyched myself up for what was to come, one last time.

Unlike during the previous 24 external radiation therapy treatments, I had decided to leave my eyes open today and watch the machine move around the table. It turned out to be unexpectedly boring — until every single guide and angel who had been with me through six rounds of chemotherapy literally lined up at the end of the table for a photo opportunity of sorts. When my main guide took off his top hat in reverence and motioned for my late parents and several other close friends who had passed away to join them, including my friend from Montreal holding her clarinet, I began to sob. "Smile and wave goodbye to Barb, everybody," he said when the noisy *Beam on* siren went on.

"Thank you for everything," I thought, and then crossed the finish line of my cancer treatment journey, albeit only in my head.

Wiping the tears off my face, I put my clothes and hat back on.

“Here’s a letter to confirm that you have finished treatments,” said the radiation therapist. “Continue everything you are doing for two weeks and call your radiologist or the social worker if you need help.”

I put the piece of paper in my purse and said an emotional goodbye.

On my way out, another therapist stopped me. “I loved your card,” she said, “especially the part about wanting to tell your story and raise funds for cancer research.”

Her heartfelt words promptly made me cry again. Having lost a child, she had written a book herself and found it to be very healing. Could she tell how frustrated I was that Covid-19 restrictions prevented me from giving her and all the other hospital staff thank-you hugs to express my gratitude for welcoming me into their family for the past five and a half weeks? I hope so.

Help me celebrate today

“Will you do something special to mark this occasion, Barb?” my trusted neighbour asked me on our way home.

“She is right — you should put something on Facebook,” my main guide whispered into my ear an hour later during my meditative walk at a nearby park.

“You want me to go public already?” I thought.

“It’s time,” he said, smiling.

Before turning on the computer, I called my twin sister and told her about my plans. “Is there anyone among your Facebook friends who would scold me for not telling them what’s been going on with you?” she asked.

“I don’t think so,” I replied. I had not considered that scenario but understood her worries perfectly, given how fervently I had insisted on keeping my health journey private. “If that should indeed happen, let me apologize to you on their behalf right now,” I said.

At 1:07 pm, I posted the following message:

“Barbara Reul is feeling thankful on 26.03.2021: *Help me celebrate today — I finished a 9-months-long cancer journey that included a sudden diagnosis in late July, followed by major surgery, six bumpy rounds of chemo, and 28 taxing radiation treatments. Thank you to everyone who I invited to help me navigate uncharted waters. I will pay forward all your acts of kindness by writing down my story and then share it widely to help raise funds for cancer research!*”

To my genuine surprise, close to 150 friends clicked on the “like”, “love”, and “care” emojis within 48 hours. “I had no idea there were that many because I am super careful with accepting requests and hardly post anything,” I said to my clergy friend in Alberta, who was thrilled about my post.

Over 90 people had taken the time to offer wonderfully uplifting, heartfelt comments. One of my favourites had been penned by Winston’s mom, one of my walking-buddy colleagues: “What a road it has been, and you’ve tackled each and every step with grace, poise, dignity, strength and of course — one hell of a sense of humour!” I laughed out loud when a couple of days after the post she also called me “Barbara Reul 2.0.” Later that evening, I rejoiced with my Greek friend and colleague, who was celebrating her five-year anniversary of having been cancer-free. There was hope for me!

Not everyone was impressed with my post, however. Several people shared their disappointment about having to find out through social media what had been going on, while others judged me openly for not allowing them to be more involved in my health journey. “Not classy,” I thought. It took me weeks to admit that my own, unfair reaction at the time deserved that label and months to view their comments for what they were, namely expressions of genuine concern for me. If only they had used e-mail to contact me, like a former professor and his wife did, as well as two German fellow scholars who identified themselves as fellow cancer survivors as well. “Very classy,” I concluded.

To my great relief, very few individuals asked for details. “Do you feel comfortable telling me what kind of cancer it was?” the daughter of my late mother’s best friend asked gently via Facebook Messenger. As a trained nurse who had spent her early years on the oncology ward, she knew exactly how to ask me to disclose information without violating my privacy.



Celebrating life

Much to my surprise, a beautiful bouquet of flowers arrived shortly after I had pressed the post button on Facebook. I could not wait to open the card that accompanied it. “Happy last radiation treatment day! In awe of your bravery, resilience, and toughness,” it read and was signed by our wonderful library coordinator at work. I was crying when

I texted her back to thank her from the bottom of my heart. Life was good!

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15: “Patience required” — Recovery, Part 4

Have you ever felt like a heavy weight was lifted off your shoulder, only to realize that the weight is — and will be there — for the rest of your life?

That is what I felt like three days after finishing my radiation therapy treatments. I was attending my second ovarian cancer support group meeting online. I recognized some of the faces but was not familiar with the lady who shared the saddest news of the evening. Her cancer had returned within one year of going into remission, and she was now undergoing her second round of chemotherapy. “Where is my tissue box?” I said out loud. Then, I wiped the tears from my face and rejoiced when she said that her new nausea medication was working perfectly.

When it was my turn to talk, I made everyone laugh by telling them about my complicated relationship with boiled carrots. Another participant then admitted that she was still struggling with diarrhea years after finishing treatments. “Could this be me?” I wondered anxiously.

Others expressed their concern about a local oncologist who was not into ordering tests such as CT scans and blood work, especially after the two-year mark had passed. “I have talked to my GP about this, and he will order them for me now,” one lady said. Since I had a CT scan done in January, a PET scan in February, and an MRI scan in March, I hoped that blood work would be the only test the oncologist would order for me in late May, if that.

The following day, March 30, 2021, would have been my mom’s 95th birthday, which she shared with my oldest friend from university, who was turning 60. I celebrated their accomplishments with a long walk, having spent much of the weekend at home, either resting in bed because of fatigue or praying to the diarrhea gods for mercy. Working on the book manuscript had been impossible, much to my regret.

When I got home, the phone rang.

“Just a heads-up,” said my best friend. “I have to go back to the hospital because my lymphoma wound is not healing properly.”

“Wow! I am so sorry to hear that,” I said. How long was her cancer journey going to continue? She had been through so much already. “I will call you every day,” I promised her, and for good reason. A “no visitors allowed” Covid-19 rule took effect the very next day. Since case numbers in the city had begun to skyrocket once again, I felt more grateful than ever to be finished with treatments.

Holy Week sacrifices

On [Maundy Thursday](#), I had not expected to become so emotional while watching the Lutheran Bishop of Saskatchewan reverently washing his daughter’s and grandchildren’s feet in the sanctuary. Nor had I been prepared to cry myself through countless hymns on Holy Wednesday and Holy Saturday. The sacred poetry focused on healing and drew attention to sacrifices that had been asked of me during my journey back to health. And I was not out of the woods yet, as fatigue, headaches, and, of course, diarrhea, as well as belly pains and vaginal discharge continued to upset my body.

On Good Friday morning, I took a deep breath before I opened the drawer of my bathroom vanity, took out the vaginal stretcher, and followed the instructions it had come with. Then, I pressed the start button on my phone’s timer and meditated together with Deepak Chopra. I concluded that my “pelvic homework” had been neither long nor scary four minutes later when I removed the device and cleaned it before putting it back into a boring, see-through plastic bag. Maybe a more exciting storage container would help me accept the fact that I would have to engage in this less than glamorous behaviour four times a week for an entire year. It certainly felt like a chore if not a sacrifice, albeit a small one.

As a frequent traveller prior to the pandemic, I also wondered how I would incorporate this routine if I ever were to leave town for extended periods of time again. Visions of locking myself into a bathroom and lying on the floor or in an empty bathtub, device in hand, made me cringe and crave Nutella on the spot. At least my new travel companion would not be as cumbersome as the [sleep apnea](#) equipment my oldest sister had been required to pack whenever she wanted to see her children and grandchild.

On Easter Sunday morning, I spontaneously decided to go on an early-morning walk. Enjoying the sunshine and blue sky, I felt like someone (Jesus?) had literally pulled the stone from the tomb in which I had found myself during treatments. After a hearty

breakfast and listening to the joyful organ prelude that I had pre-recorded, I sang countless “Hallelujahs” while worshipping online, albeit by myself. For the first time in a long time, the words “untroubled” and “carefree” popped into my head, much to my delight.

In the afternoon, I got in touch with many relatives and friends to wish them a Happy Easter. My intention was to make them and myself laugh by emphasizing that I did not resemble a rabbit after all, despite sporting white hair and eating boiled carrots almost daily. A delicious, diet-friendly supper, courtesy of my trusted neighbour, made it official: this had been an excellent Easter Sunday!

“When do you think you will be able to eat real food again?” my colleague, the fellow German who had visited me at the hospital when I had surgery, joked during our usual Sunday evening chat.

“When I run out of chocolate Easter eggs!” I quipped. My answer signalled to both of us that I was on the mend, both physically and mentally.

Two days later, my favourite radiation oncology nurse wanted to know how I was doing. She was pleased to hear that I was managing the side effects to the best of my abilities. “Be gentle with yourself,” she reminded me before confirming the time and date of my follow-up appointment in late May. The radiologist did not need to see me, only the medical oncologist, the nurse informed me. “And you won’t need to do any blood work either.” Thank the Lord! I never once minded being poked by a lab technician or nurse since I have good veins, but I always hated waiting for the results afterwards, for obvious reasons.

I also wondered when it would be safe for me to go to the pool again. “Give it four to six weeks,” the nurse advised. Her answer pleased me, even though I knew I was not going to rush back to my favourite physical activity anytime soon. Under normal circumstances, I would have considered booking myself into the “spa,” a [resort-style hotel with a geothermal pool](#) in a small prairie town 45 minutes west of the city.



Taking the waters

“Darn pandemic,” I said to my twin sister that evening. We had such a great time the last time we stayed there in August 2019 to celebrate our birthday. “It would have been the ideal place to ‘take the waters’,” my older sister in Germany commented, and I could not agree more — except that a “no travel” advisory had been in effect for weeks now, which some people blatantly chose to ignore, arguing that they just “needed to get out.” Others continued to demonstrate in front of the legislative building and, to my horror, also outside the hospital at which I had gotten my CT and MRI scans.

“It is all a hoax,” they claimed, referring to the provincial health authority as “serial killers.” Why did these individuals not realize that they put us all at risk, especially folks like myself and my best friend whose immune systems were compromised?

A shot in the arm

On the afternoon of Friday, April 8, 2021, I made sure to wear a short-sleeved shirt and put my medical insurance card into my wallet before driving myself to the university. Seven minutes later I pulled into the same lot in which I had parked prior to all my classes being moved online over a year ago. A yellow-vested individual greeted me, while another directed me to a spot closer to the building which I needed to enter to get my first shot.

After showing my ID to a security officer and answering a few Covid-19-related questions, I felt weird lining up in the same gym in which I used to play badminton years ago. I had quit after the fellow professor who loved to chase me around the court decided to retire and moved away. To everyone’s surprise, including my own, I began taking [Zumba](#) and [Jazzercise](#) classes at a [YWCA](#) fitness centre near my house and stuck with it for several years. “I love the music and the dance routines,” I remember telling my twin sister. “The best part is that I never see any of my students there who assume their university professors live in their offices 24/7.”

When it was my turn to check in to be vaccinated, I showed my medical card and then walked around the corner to receive my shot. The nurse first went through a detailed questionnaire; after finding out that I had finished radiation therapy less than two weeks ago she left her station to pick up a form.

“Please sign this to indicate that we cannot guarantee full efficacy due to your compromised immune system,” she said when she returned. Then she administered the [Pfizer-BioNTech vaccine](#) and asked me to wait for 15 minutes at the back of the gym.

Feeling unexpectedly underwhelmed, I thanked her, found a chair, and checked my watch to figure out when it would be safe for me to leave. I was not impressed when I noticed that several people who had taken a seat well after me, got up early and exited the building. “Well, at least they got their shots today, which is a good thing,” I concluded on my way back to the parking lot.

“My upper left arm is killing me,” I announced to my twin sister later that afternoon when it was time for our daily workout. I was not surprised as it had also hurt when I received the flu vaccine in early November of last year.

“We did a bunch of biceps and triceps lifts yesterday, so you might be sore from that as well,” she noted.

When I woke up in the middle of the night with even more pain, however, I decided it was time to pop a pill. “Can you adjust today’s exercise routine, so it won’t involve me lifting my arm?” I asked her. My twin sister did, and by Saturday morning, my muscle soreness and arm pain were gone.

Many firsts

That same morning, I also enjoyed my first cup of black tea since my last chemotherapy treatment on January 20, 2021. The hot, dark brown liquid had tasted better than chocolate — at least initially. Then, I started to cough, my throat began to itch, and my irritated bladder sent me a message as well. “I’d better go easy on the caffeine for the next while,” I decided, as not to stress my body unnecessarily.

A [fascial release massage](#) that afternoon helped me relax, as did a lovely conversation with my massage therapist. We had kept in close touch ever since my diagnosis. I was ever so grateful that she never once complained about me moving appointments and happily triple-disinfected the treatment room prior to my arrival. To my delight, she commented on how much my hair had grown and its colour but refrained from identifying body parts that desperately needed shaving.

Three days after the massage, I finally tried it, noticing how soft the hair under my armpits had grown back. While I did not dare to pluck my unexpectedly bushy eyebrows, I did put mascara on my baby eye lashes. Life was good!



Fasch Memorial Stone, Zerbst/Anhalt, Germany

Exactly one week after being vaccinated, I tried out a new, bright red lipstick and wore a nice pair of earrings in honour of the 333rd birthday of Johann Friedrich Fasch, my “composer husband,” as I have been known to call him. Under normal circumstances, I would have attended the 16th International Fasch Festival in the small town of [Zerbst](#) in Central Germany, where Fasch had spent 36 years as court [Kapellmeister](#) or director of music.

In fact, I had been scheduled to present two public lectures and a conference paper on archival materials related to his life and works over the course of five days. Instead, I was waiting for the festival organizers, who had already moved the festival from mid-April to mid-June 2021 due to Covid-19 restrictions, to announce its cancellation — which they did on April 20, 2021, a first in the 30-year history of my favourite baroque music festival.

“How do you feel about that?” my best friend asked me on the phone. I had called her with the news while taking my car for a spin down a secondary highway, passing two huge *Travel not recommended* signs on the way.

“I think I would have been more disappointed if it had run as usual and I could not have been there,” I told her. In the past, relatives and friends had teased me mercilessly for planning my life around this biennial event, a highlight in my work life ever since I had attended my first festival as a graduate student in 1993. Now, I could not imagine what the next one would look like — if there would be one — and did not care when my scholarly book on Fasch would be finished, if ever.

“Wow,” I thought, after acknowledging how little I cared about my career trajectory these days. Clearly, I had changed since last July; in fact, I hardly recognized that person. “I do, and I have missed her terribly,” said the little girl inside me, and hugged me for the first time in a long time.

Living with the fear of dying

“I have excellent news,” my clergy friend from Alberta said on the phone the next day. She had been cancer-free for five years! I was so relieved and grateful to be able to celebrate this milestone with her, albeit virtually.

Unlike me, she had almost died during her first chemotherapy session due to a severe allergic reaction and decided to discontinue them in favour of taking medication. Every six months she had to go in for a check-up, and every single one had been a nail-biter, to say the least. I wondered whether it was going to be like that for me as well, just at shorter intervals because the recurrence rate of endometrial cancer was much higher than breast cancer.

“What’s on your agenda today, Barb?” she asked, bringing me back into the present.

“I am going to attend a [webinar hosted by the Ovarian Cancer Society](#) tonight,” I told her. According to the support group participants, the presenter was a most engaging and knowledgeable individual. Moreover, the topic was of great interest to me as I was still grieving my friend’s untimely death in mid-October 2020.

“Hello everyone,” said the moderator to 48 people (if not more) who had logged on from all over Canada. About five minutes into the presentation, I realized that I had misunderstood the title, “Living with the fear of dying.” It was not about the possibility of one’s cancer reoccurring. Instead, the speaker addressed the taboo of talking about death and dying in Western Culture and what to do about it.

“Should I leave?” I asked my main guide, somewhat frustrated with my “chemo brain.” “Stay put and pay attention,” he answered. “You’ll learn something important.”

He was right, of course.

To my surprise, several of the fears associated with a terminal diagnosis that were identified turned out to apply to me, even though my prognosis was excellent. I worried less about my academic legacy — what else were students, colleagues, and publications for? — and more about being remembered as a kind relative and a supportive friend, especially since I had never given birth or been partnered. As someone who had watched her mother look after her ill husband, my father, for many years, I also did not wish to be a burden to anyone.

Missing out on living life to the fullest resonated the most with me. “Your body cannot tell the difference between good stress and bad stress,” my oldest sister would remind me when I had wrestled with life-work balance issues in the past. What, if anything, would I change if I had only a few more months or years to live? “This is heavy stuff,” I concluded, unsure as to whether I should watch the rest of the presentation or not.

The speaker went on to give examples of what dying people talked about. I immediately identified with feeling isolated, not just during a pandemic, and having been told by others, including my main guide, to adjust my attitude. Like terminally ill patients, I had struggled with loss of control and experienced feelings of unfairness during my cancer journey, especially at the beginning.

“In truth, we have little and sometimes no control over the things that really count,” the presenter emphasized. Dying was “not a punishment for not trying hard enough and being good enough,” she stressed. As an eternal optimist and woman of strong faith, I had never viewed the end of my life through that lens. Would I blame myself if the cancer returned or would I take it in stride?

The question of whether oncologists should pose a “[Patient Dignity Question](#)” intrigued me. I would certainly have appreciated being asked by my medical team at the very beginning of my journey what they needed to know about me as a person to provide

the best care.

A quotation from Stephen Jenkinson's film [The Meaning of Death](#) touched me deeply as well. It read: "All we need of you is that you be human, now. Our work is to sort out what being human should be in such a time." Was this a map of sorts for how I was to approach the future? If nothing else, I viewed it as a challenge that appealed to my competitive nature.

Two comments from the questions and answers session were particularly memorable. The first one — "Is it wrong not to want to engage with the cancer world after finishing treatments?" — gave me pause. I certainly did not want to become an attraction of sorts as an author, with "university prof shares cancer journey" as the tagline. Nor did I want to be pitied, judged, or criticized for what I had been through.

Would receiving negative feedback perhaps feel like when others introduced me as "the German lady"? People's reactions to meeting me — "You don't have much of an accent," I would be told — had ranged from bizarre ("Do you know my third cousin from Düsseldorf?") to plain rude ("I like your red top — they only wear black or grey in Nazi movies"). I determined that my best course of action was to focus on the overall goal of this book, i.e., to educate and entertain others and raise funds for cancer research. The rest — whatever that meant — I would surrender to the universe to handle.

The other comment made me laugh even though it was not meant as a joke. A 20-year cancer survivor had questioned her fear of "dying of something stupid." My inner stand-up comedian promptly offered a myriad of possible scenarios, including death by poisoning from boiled carrots or by choking from chocolate-covered almonds.

After signing off, I realized that this webinar represented an important step in my own healing journey: it had given me a glimpse of what my personal space in the world of cancer survivors might look like.

The final question

My GP called me the next day, Thursday, April 22, 2021.

"How are you, Barb?" he asked.

"I did not expect the side effects from radiation to be this awful," I replied. Then, I listed the most memorable ones. "I am also still dealing with low energy and fatigue — I had another bad episode last Saturday that came out of nowhere," I told him. The numbness in my left foot and right thumb were another daily reminder of having finished chemotherapy only (or already?) three months ago, as were the occasional headache, sleepless night, and hot flash.

"Give it time," my GP advised and wondered whether I had any questions for him. "I have an entire list," I said, dead serious. "Dr. Barb" had been working on them diligently over the last couple of days.

Since I had received my first shot two weeks ago, could I start going on walks again with others who had been vaccinated as well? And would it be okay to begin playing at church again as of mid-May, at which time the pastor and church musician would be vaccinated as well? "Yes" was my GP's response to both queries.

Little did he know that my pastor would successfully talk me out of the latter idea the very next day. "It's not safe yet," she argued convincingly, citing viral load concerns and the fact that services would not be held in the big sanctuary until Covid-19 restrictions were lifted. "Fair enough," I told her and volunteered to record a prelude for [Pentecost](#) Sunday in late May.

My next question focused on when I could join my relatives out west.

"It's been really tough," I said with a big sigh. I had not seen them in the flesh since late 2019 and, despite being able to use technology to communicate with them regularly, missed them more every day.

"I understand that, but let me explain something to you," my GP replied. Then, he launched into an eloquent mini-lecture on how dangerous the new variants of Covid-19 were. Referencing medical journals ("What, no footnotes?" I thought, but kept my mouth shut), he described the seriousness of the situation, and insisted that extreme caution was the best course of action. The bottom line was that my GP advised me not to travel until I had received my second shot and then wait at least two weeks before I said goodbye to my friends in town.

"That's not really the answer I was hoping for," I said. Since the province I lived in was leading the country as far as vaccinations were concerned, I had talked my twin sister, who was scheduled to receive her first shot in late April, into booking a cheap flight in

late May. “Come and get me,” I had begged her. A miracle of sorts would now be required for that to happen; travelling out west sometime in June was more realistic.

“Thanks to Covid, this health marathon is turning into an [Iron Woman competition](#),” I moaned. My GP’s response — “patience is required” — was an effective reminder that making plans of any kind during a pandemic was futile.

Then we discussed when I might be well enough to return to my day job in the second half of 2021.

There was no denying it: the radiation therapy treatments had taken an unexpected toll on me, specifically regarding my energy household. With that backdrop in mind, I told my GP that “pre-cancer Barb” had been able to generate seemingly limitless energy and committed much of it to her work year-round. The medical teams’ ambitious treatment plans for “cancer patient Barb” had depleted both her energetic savings account and line of credit and, as a result, seriously damaged her energy credit history. Excelling at minimum wage jobs in the physical fitness and attitude sectors had not been enough to prevent declaring energy bankruptcy after only a few short months. “Post-cancer Barb” was enrolled in an energy management programme but continued to struggle to make monthly rent payments. How long would it take for her to inspire confidence in her energy bank again?

In other words, there was no way to tell when I would recover from the mental fatigue that had quietly replaced my “chemo brain” symptoms after radiation therapy treatments had begun, if ever. Despite engaging in daily exercise — and looking perfectly fine on the outside because of it — good energy days now felt completely different and were the exception, not the rule.

This new reality also had an impact on breathing life into my cancer story on paper. How difficult could it be to write a book that involved no actual research besides scrutinizing my diary to ensure chronological and contextual accuracy?

Prior to the surgery, I would have given myself a month and then, as a seasoned and fast writer, would have tried to get it done in half the time. Instead, I had woken up feeling sluggish, listless, and lethargic, if not uninspired on most days in March and many in April and, to my horror, turned into “Zombie Barb.” The daily editing process — which has always been my favourite part of the writing process — was turning into a terrible chore.

I had even complained to my main guide about it on Easter Monday.

“Therapeutic or not — this is hard,” I said.

“Who said it was going to be easy?” he replied.

That comment made me laugh because he had just quoted a response that I would often give to students expressing their dissatisfaction with an assignment to me. To cheer myself up, I followed my main guide’s advice to add clever subtitles to what I had written so far for a couple of days, only to admit to myself that I had run out of steam and would need to finish the manuscript later.

When my inner child promptly nodded in agreement, I realized that the universe had never set a specific deadline or indicated when I should or could stop keeping a cancer diary.

The cut-off date as far as my health journey was concerned dawned on me when I arrived at the last question to pose to my GP.

“What is my current patient status, now that I have finished radiation therapy?” I asked him.

“You are in [remission](#), Barb,” he announced, “because none of the scans have shown any evidence of cancer being present.”

“That was the answer I was hoping for,” I said, literally jumping up and down for joy. I had not realized how much I had craved hearing these words.

“What a relief!” my relatives and close friends said. “You have no idea,” I replied and smiled from ear to ear. Life was really good!

And this marks the end of my recollections of how I coped with cancer during a pandemic. It is, however, not the conclusion of this book.

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16: “Perfect Timing” — Epilogue

If you got here by skipping to the end, I forgive you. It is exactly what I have been known to do! If you did not, I am proud of you. Either way, it is time to take stock.

Have I changed because of what happened to me? Yes.

Will I live life differently from now on? Yes, because I am no longer the same person.

How? To find out, read on.

Supergirl’s creed

Many years ago, my older sister in Germany had framed a cute little poem and hung it on her office wall. Here is my translation and the original text:

The impossible we attend to immediately. Unmögliches erledigen wir sofort.

Miracles take a little longer. Wunder dauern etwas länger.

Upon request, witchcraft will be used. Auf Wunsch wird gehext

These three lines arguably captured my approach to life in general before my cancer surgery in August 2020. It touched on the what (“the impossible,” “miracles”); the when (“immediately,” “a little longer,” “upon request”); and the how (“attend to,” “witchcraft”). Listening carefully to the universe for karmic advice, I had chosen to which external and internal life projects I would apply these vows of sorts. Taking control of my destiny in Germany and Canada or “the where” — which the poem does not touch on, incidentally — had not been without its ups and downs.

Overall, it had been a stable ride, however, for which I had always been very grateful.

In hindsight, it occurred to me that I had viewed my cancer diagnosis through the same “karmic assignment lens,” for the lack of a better label, communicated in this poem. What did I need to learn and/or teach others from this specific challenge? Was there a tangible reward or added incentive to get it right so as not to have to repeat the lesson?

I had already figured out after radiation treatments had finished that my late mother’s favourite prayer contained the answer to the latter question. A higher power would reward me with serenity, courage, and wisdom after having successfully faced life-altering experiences.

“That sounds remarkably like the story line of Mozart’s [Magic Flute](#) to me,” I had concluded at the time. I also chuckled because it was one of my favourite musical works to teach at university. The year 2020 and the first quarter of 2021 had been anything but a fairy tale, however, even though there had been a snake (the cancer?); a prince (my medical team?) who was charged with freeing a princess (me?) who had been kidnapped; and various trials and tribulations the couple needed to overcome (the treatment plan?).

Thankfully, Mozart’s librettist had opted against the backdrop of a pandemic and instead invented hilarious characters such as a bird catcher who made the audience laugh while reminding them of how difficult following rules can be as a human being.

So, what do you think my lesson(s) were, dear reader?

Permanence in change (to quote Goethe)

It was now the perfect time to let go of attending to “the impossible,” i.e., excelling in everything I did and being everything to everyone, for once and all. Instead, I would follow “Dr. Barb’s two golden rules,” which she had already adopted for herself several years ago but not embraced fully until being faced with cancer. From now on, I would view “asking for help” and “doing less” as signs of strength, not weakness, as I strove to lead a more balanced, comparatively uneventful life.

Affecting “miracles” of a different kind — first and foremost being cancer-free — now deserved 100% of my energy and willpower. There was no doubt that I had a long way to go until the side effects of chemotherapy and radiation would no longer rule my daily life as was the case right now.

Eyelids that weighed 50 pounds each greeted me almost every morning in April, and I got scared every time I experienced belly pains or threw out multi-coloured panty liners. And who was this white-haired woman staring back at me when I looked in the mirror or into my phone or computer camera? She had a huge scar on her belly and engaged in a vaginal stretching routine every other day. It was my sincere hope that when her time to leave this earth had come, she would die from old age rather than “something stupid.”

There was no question that I would continue to use “witchcraft” — that is, listen to messages from my inner guidance system and the universe “upon request.” [Dietrich Bonhoeffer](#), a famous Lutheran theologian who died in a concentration camp near the small German village in which I grew up, described perfectly what my heart, mind, body, and soul feel like whenever that happens. He wrote the following in 1945 which I have translated for you and continue to pray on a daily basis: “Sheltered wondrously by good forces, we expect with confidence what may come. God is with us in the evening and in the morning and, certainly, on every new day.” (*“Von guten Mächten wunderbar geborgen, erwarten wir getrost, was kommen mag. Gott ist mit uns am Abend und am Morgen und ganz gewiss an jedem neuen Tag.”*)



Selfie from late April 2021

“I cannot believe it has been nine months since you told me to go see my doctor,” I said to my trusted massage therapist on April 30, 2021. In hindsight, the timing of my illness had been perfect: instead of educating others during a pandemic, I had been taught what living with cancer was all about.

“What do you expect the universe will do now that you are cancer-free?” my massage therapist asked.

“Pleasantly surprise me,” I said and smiled.

And then she lived happily ever after (or so she thought)

On June 22, I flew to Victoria, B. C., having ditched the “road trip with twin” idea in favour of selling my car back to the dealership. When I stepped into what was going to be my new home for the next twelve months or so, I knew that this change of scenery was exactly what I needed. On July 1, I went back to work (albeit remotely and part-time), and on August 11, 2021, my twin sister and I celebrated a very “chilled” 54th birthday, eating cake on my fabulous balcony which I, the non-drinker, had promptly dubbed “Barb’s beer garden.”

“I should write a sequel about my year of healing,” I said to my twin sister, inner child, and main guide. “You go, girl,” they replied. In other words, stay tuned for more, dear reader!

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Appendix

About this Book

This is an **educational, entertaining, and highly personal memoir** written during a global pandemic. It provides an insightful snapshot of the occasionally bumpy yet spiritually transformative cancer journey of a middle-aged, immigrant, and non-partnered academic living in a sunny Canadian prairie province.

It will be of **interest** to anyone who: 1) is or has been on the **cancer continuum** as a patient, caregiver, family member, or friend; 2) is or strives to be a **health professional** (oncologist, GP, nurse, social worker, pharmacist, physio- or exercise therapist, etc.); 3) is an administrator, instructor, teaching assistant, or student at a **post-secondary institution** interested in health sciences, English literature (memoir writing, creative non-fiction, and narratives of illness), Women's and Gender Studies, Spirituality Studies, Religious Studies, and the Fine Arts; 4) **fellow authors and/or readers** who like to give writers from the Canadian prairies a chance.

The **Appendix** includes “**Leading Reading Questions**” meant to increase everyone's reading experience (and lighten the load of fellow university professors who wish to adopt this book, or part of this book, for a class).

Perfect Timing – Recollections of coping with cancer during a pandemic was **shortlisted for two 2023 Saskatchewan Book Awards**, the Ministry of Parks, Culture, and Sport First Book Award and the City of Regina Award.

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About the Author

Barbara Reul is a **cancer survivor** from the Canadian prairies. As a university professor, she never thought she would be brave enough to write about herself.

She is making this – often unintentionally hilarious – illness narrative **freely accessible** to interested readers, hoping to **raise funds for cancer research** in the process.

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Other Publications By and About This Author

Reul, Barbara. “(Open) Access is the Key: From *Perfect Timing* (2021) to *Right on Time* (2023)”.

- **Recording:** This panel talk presented on 16 May 2024 as part of the University of Regina 2024 Spring OEP Bootcamp. Click [here](#) to access all recordings from that event, and then scroll down to 16 May to find the session I was part of; my talk begins at **time stamp 28’30”**.

Reul, Barbara. “Writing from the Inside Out – From *Perfect Timing* (2021) to *Right on Time* (2023).” University of Regina [OEP Program Newsletter | Spring 2024](#): 4-7.

- **Article:** This article focuses on why I chose to go with **open access over traditional publishing** for my **two self-referential narratives** – and how the universe (?) threw me a **medical curveball** in July 2022 that caused me to add another, final chapter to [Right on Time](#) (spoiler alert!).

Thibeault, James. “[Being Open About Cancer](#): Interview with **Barbara Reul**, author of the open-licensed memoirs *Perfect Timing* and *Right on Time*.” *Scholars@Bentley Library Publications* (2024).

- **Article:** This article is based on an **in-depth interview** conducted in **January 2024** by James Thibeault, Senior Librarian for Open Educational Resources at Bentley University, USA, as part of their *Scholars@Bentley* series.

Reul, Barbara. “‘Miracles are no longer required’ – Life Writing as a Healing Tool,” [Intersections 54 \(March 2023\), article 8](#).

- **Article:** This brief article summarizes my health journey as a cancer patient. It ends with an update on what happened after I finished active care in late March 2021, going beyond the time frame of the sequel, [Right on Time](#).

Reul, Barbara. [RIGHT ON TIME – Healing from cancer during a pandemic](#) (December 2023).

- **Book:** This (much longer) sequel to [Perfect Timing](#) chronicles my time of healing and recovery from **May 2021 to July 2022**.
- In it, I also discuss in more detail the actual **writing and OER publishing experience**, summarized and reflected upon first on May 11, 2022 as part of the University of Regina’s OER Spring 2022 boot camp (“It’s all about open access – and impact: From memoir to textbook”). This online presentation was recorded; it can be accessed via an article related to my talk that was published in the University of Regina [OER spring 2022 newsletter](#), pp. 4-5.

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